DM = Demographics

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Subject DM_GL_901
Generated On: 13 Jul 2020 10:14:40

Subject	SUBJID	SUBJID; Entered; Num
Site ID	[NOT SUBMITTED]	SITEID; Entered; Char

FA = Findings About Events or Interventions

SV = Subject Visits

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Date of Visit SV_GL_900

Generated On: 13 Jul 2020 10:14:40

FACAT = SUBJECT STATUS

FAOBJ = <VISIT NAME>

Did this visit occur?	FATEST = Occurrence	FAORRES when FATESTCD = OCCUR	Yes ☐ No ☐ TECHVISIT; Coded; Cha
Subject's Status	FATEST = Subject Status	FAORRES when FATESTCD = SCMPLSTS	TECHSTATUS; Entered
FADTC Visit Dat	SVSTDTC [IEDTC, DMDTC, SUDTC, DSDTC, MHDTC, FADTC, QSDTC, XZDTC, PCD		VISDAT; Time; Date
Type of Contact	FAORRES when FATESTCD=TYPVCON		Site Visit ☐ Electronic Contact ☐ Home Visit ☐ Remote, Telephone Call ☐ Remote, Video Call ☐ TYPVCON_GL3; Coded

SV = Subject Visits

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Unscheduled Visit ***SV_GL_901***
Generated On: 13 Jul 2020 10:14:40

Visit Date	***SVSTDTC [IEDTC, DMDTC, SUDTC, DSDTC, MHDTC, FADTC, QSDTC, XZDTC, PCDTC, ISDTC, VSDTC]***	***VISDAT; Time; Date***

Collection of biopsies for RNA, histology, and single cell isolation samples		***PC_GL_900_R; Checke***
Genetic (DNA) evaluations		***PC_GL_900_D; Checke***
Endoscopy Information		***ENDO_INFO_001; Chec***

DM = Demographics

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Demographics DM_GL_900

Generated On: 13 Jul 2020 10:14:40 DSCAT = PROTOCOL MILESTONE

INFORMED CONSENT			
What was the date of the signature on informed consent?		DSSTDTC when DSTERM = INFORMED CO	RFICDAT; Time; Date)
PROTOCOL DATE			
What was the issue/report date of the protocol the subject consented to at study enrollment?		SPRTDTC in SUPPDS when IDVAR = DSSEQ	SPRTDAT; Time; Date
DEMOGRAPHICS			
What is the subject's age?		AGE	AGE; Entered; Num
Age unit	AGEU	Hours ☐ Days ☐ Weeks ☐ Months ☐ Years ■	AGEU; Coded; Char
What is the sex of the subject?	SEX	Female ☐ Male ☐ Unknown ☐ Undifferentiated ☐	SEX; Coded; Char
What is the ethnicity of the subject?	ETHNIC	Hispanic or Latino ☐ Not Hispanic or Latino ☐ Not Reported ☐	ETHNIC; Coded; Char
What is the race of the subject *(Check all that apply)*			
American Indian or Alaska Native	RACE	RACEAIAN in SUPPDM when IDVAR = NULL	RACEAIAN; Checked; N
Asian		RACEA in SUPPDM when IDVAR = NULL	RACEA; Checked; Num
Black or African American		RACEBA in SUPPDM when IDVAR = NULL	RACEBA; Checked; Nu
Native Hawaiian or other Pacific Islander		RACENHOP in SUPPDM when IDVAR = NULL	RACENHOP; Checked;
White		RACEW in SUPPDM when IDVAR = NULL	RACEW; Checked; Num
Not reported		[NOT SUBMITTED]	RACENR; Checked; Nu
If subject was re-screened, specify below:			
Previous Subject ID		PSUBJID in SUPPDM when IDVAR = NULL [NOT SUBMITTED]	PSUBJID; Entered; Cha

NOTE:
When more than one selected, RACE = MULTIPLE and individual responses are RACEAIAN, RACEA, RACEBA, RACENHOP, RACEW in SUPPDM

VS = Vital Signs

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Weight and Height ***WTHT***
Generated On: 13 Jul 2020 10:14:40

VSTEST	Height	*VSORRES, VSORRESU when VSTESTCD = HEIGHT*	**cm** ☐ in ☐	*HEIGHT; Entered; Num*
VSTEST	Weight	*VSORRES, VSORRESU when VSTESTCD = WEIGHT*	**kg** ☐ lb ☐	*WEIGHT; Entered; Num*

VS = Vital Signs

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Weight ***WEIGHT***

Generated On: 13 Jul 2020 10:14:40

VSTEST	Weight	*VSORRES, VSORRESU when VSTESTCD = WEIGHT*	**kg** ☐ lb ☐	*WEIGHT; Entered; Num*

DS = Disposition

IE = Inclusion/Exclusion Criteria Not Met

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Inclusion/Exclusion Criteria *IE_GL_900*
Generated On: 13 Jul 2020 10:14:40 *DSCAT = PROTOCOL MILESTONE*

Did the subject meet all eligibility criteria? *DSTERM, DSDECOD = ELIGIBILITY CRITERIA MET* *DSSTDTC = <IEDTC>*	☐ Yes *[NOT SUBMITTED]* ☐ No	*IEYN; Coded; Char*
If No, enter below information about criteria not satisfied/met (click Add New Log Line to enter more than one criterion)	☐ Exclusion *IECAT* ☐ Inclusion	*IECAT; Coded; Char*
What was the category of the criterion?		
Criterion ID	*IETESTCD*	*IETESTCD; Entered; Ch*
Now	*[NOT SUBMITTED]*	*IEGL900NOW; Time; Da*

DM = Demographics

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Consent for Participation Week 4 Endoscopy and Biopsy Substudy DS_GL_905

Generated On: 13 Jul 2020 10:14:40

Category	[NOT SUBMITTED] OTHER EVENT	DSCAT; Entered; Char
Substudy Name	Week 4 Endoscopy and Biopsy Substudy DSCAT = PROTOCOL MILESTONE	DSSCAT; Coded; Char
Did the subject agree to participate? DMSBSTD1 in SUPPDM when IDVAR = NULL DSTERM, DSDECOD = INFORMED CONSENT FOR SUB-STUDY PARTICIPATION	Yes ☐ [NOT SUBMITTED] No ☐	DSSTUDYN; Coded; Ch
Consent Signature Date	DSSTDTC	DSSTDAT; Time; Date

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Consent for Optional Samples for Genetic Only Research BE_GL_902
Generated On: 13 Jul 2020 10:14:40 DSCAT = PROTOCOL MILESTONE

Category	[NOT SUBMITTED]	OPTIONAL SAMPLES FOR GENETIC ONLY RESEARCH ☐ OPTIONAL SAMPLES FOR SCIENTIFIC RESEARCH ☐ STORAGE OF SAMPLES FOR FUTURE RESEARCH ☐	BECAT; Coded; Char
Was consent obtained to provide (an) optional sample(s) for genetic research? DSTERM, DSDECOD = INFORMED CONSENT FOR OPTIONAL SAMPLES FOR GENETIC ONLY RESEARCH	[NOT SUBMITTED]	Yes ☐ No ☐	BEDNAYN; Coded; Cha
If 'Yes', date of signature	DSSTDTC		BESTDAT; Time; Date

MH = Medical History

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Medical History of Interest MH
Generated On: 13 Jul 2020 10:14:40

Category — MHCAT

- Arthritis — MHCAT1; Coded; Char
- Cardiovascular and metabolic diseases
- Malignancy
- Psychiatric conditions
- Other

Medical History Term — MHTERM

- Ankylosing spondylitis — MHTERM1; Coded; Cha
- Osteoarthritis
- Psoriatic Arthritis
- Rheumatoid arthritis
- Angina pectoris
- Coronary Artery Disease
- Diabetes Mellitus
- Family history of Early Coronary Artery Disease (less than 55 years of age)
- Hyperlipidemia
- Hypertension
- Myocardial infarction
- Peripheral Vascular Disease
- Stroke
- Transient Ischemic Attack (TIA)
- Skin Squamous Cell Carcinoma
- Skin Basal Cell Carcinoma
- Other Malignancy
- Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas)
- Anxiety
- Depression
- Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis)
- Gout
- Hidradenitis suppurativa

Psoriasis ☐

Does the subject have or has the subject had any of the following: *MHOCCUR* *MHPRESP = Y*

Yes ☐ *MHYN1; Coded; Char*
No ☐

Category

Arthritis ■
Cardiovascular and metabolic diseases ☐
Malignancy ☐
Psychiatric conditions ☐
Other ☐

Medical History Term

Ankylosing spondylitis ☐
Osteoarthritis ■
Psoriatic Arthritis ☐
Rheumatoid arthritis ☐
Angina pectoris ☐
Coronary Artery Disease ☐
Diabetes Mellitus ☐
Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
Hyperlipidemia ☐
Hypertension ☐
Myocardial infarction ☐
Peripheral Vascular Disease ☐
Stroke ☐
Transient Ischemic Attack (TIA) ☐
Skin Squamous Cell Carcinoma ☐
Skin Basal Cell Carcinoma ☐
Other Malignancy ☐
Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
Anxiety ☐
Depression ☐

For annotations, refer to CRF page 11

	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ■
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ■
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐

For annotations, refer to CRF page 11

Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
Anxiety ☐
Depression ☐
Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
Gout ☐
Hidradenitis suppurativa ☐
Psoriasis ☐

Does the subject have or has the subject had any of the following:
Yes ☐
No ☐

Category
Arthritis ■
Cardiovascular and metabolic diseases ☐
Malignancy ☐
Psychiatric conditions ☐
Other ☐

Medical History Term
Ankylosing spondylitis ☐
Osteoarthritis ☐
Psoriatic Arthritis ☐
Rheumatoid arthritis ■
Angina pectoris ☐
Coronary Artery Disease ☐
Diabetes Mellitus ☐
Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
Hyperlipidemia ☐
Hypertension ☐
Myocardial infarction ☐
Peripheral Vascular Disease ☐
Stroke ☐

For annotations, refer to CRF page 11

	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ■
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐

For annotations, refer to CRF page 11

	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ■
	Diabetes Mellitus ☐

For annotations, refer to CRF page 11

- Family history of Early Coronary Artery Disease (less than 55 years of age)
- Hyperlipidemia
- Hypertension
- Myocardial infarction
- Peripheral Vascular Disease
- Stroke
- Transient Ischemic Attack (TIA)
- Skin Squamous Cell Carcinoma
- Skin Basal Cell Carcinoma
- Other Malignancy
- Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas)
- Anxiety
- Depression
- Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis)
- Gout
- Hidradenitis suppurativa
- Psoriasis

Does the subject have or has the subject had any of the following:

- Yes
- No

Category

- Arthritis
- Cardiovascular and metabolic diseases
- Malignancy
- Psychiatric conditions
- Other

Medical History Term

- Ankylosing spondylitis
- Osteoarthritis
- Psoriatic Arthritis

For annotations, refer to CRF page 11

	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ■
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐

For annotations, refer to CRF page 11

Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ■
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■

For annotations, refer to CRF page 11

	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ■
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐

For annotations, refer to CRF page 11

	No ☐
Category	Arthritis ☐ Cardiovascular and metabolic diseases ■ Malignancy ☐ Psychiatric conditions ☐ Other ☐
Medical History Term	Ankylosing spondylitis ☐ Osteoarthritis ☐ Psoriatic Arthritis ☐ Rheumatoid arthritis ☐ Angina pectoris ☐ Coronary Artery Disease ☐ Diabetes Mellitus ☐ Family history of Early Coronary Artery Disease (less than 55 years of age) ☐ Hyperlipidemia ☐ Hypertension ■ Myocardial infarction ☐ Peripheral Vascular Disease ☐ Stroke ☐ Transient Ischemic Attack (TIA) ☐ Skin Squamous Cell Carcinoma ☐ Skin Basal Cell Carcinoma ☐ Other Malignancy ☐ Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐ Anxiety ☐ Depression ☐ Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐ Gout ☐

For annotations, refer to CRF page 11

	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ■
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐

For annotations, refer to CRF page 11

	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ■
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐

For annotations, refer to CRF page 11

	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ■

For annotations, refer to CRF page 11

	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ■
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐

For annotations, refer to CRF page 11

	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ■
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ■
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐

For annotations, refer to CRF page 11

	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ■
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ■
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐

For annotations, refer to CRF page 11

	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ■
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ■
	Psychiatric conditions ☐
	Other ☐

For annotations, refer to CRF page 11

Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ■
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐

For annotations, refer to CRF page 11

	Malignancy ■
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ■
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐

For annotations, refer to CRF page 11

	No ☐
Category	Arthritis ☐ Cardiovascular and metabolic diseases ☐ Malignancy ☐ Psychiatric conditions ■ Other ☐
Medical History Term	Ankylosing spondylitis ☐ Osteoarthritis ☐ Psoriatic Arthritis ☐ Rheumatoid arthritis ☐ Angina pectoris ☐ Coronary Artery Disease ☐ Diabetes Mellitus ☐ Family history of Early Coronary Artery Disease (less than 55 years of age) ☐ Hyperlipidemia ☐ Hypertension ☐ Myocardial infarction ☐ Peripheral Vascular Disease ☐ Stroke ☐ Transient Ischemic Attack (TIA) ☐ Skin Squamous Cell Carcinoma ☐ Skin Basal Cell Carcinoma ☐ Other Malignancy ☐ Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐ Anxiety ■ Depression ☐ Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐ Gout ☐

For annotations, refer to CRF page 11

	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ■
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ■

For annotations, refer to CRF page 11

	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ☐
	Other ■
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐

For annotations, refer to CRF page 11

	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ☐
	Other ☐
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐

For annotations, refer to CRF page 11

	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ■
	Hidradenitis suppurativa ☐
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ☐
	Other ■
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐
	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐

For annotations, refer to CRF page 11

	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ■
	Psoriasis ☐
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐
Category	Arthritis ☐
	Cardiovascular and metabolic diseases ☐
	Malignancy ☐
	Psychiatric conditions ☐
	Other ■
Medical History Term	Ankylosing spondylitis ☐
	Osteoarthritis ☐
	Psoriatic Arthritis ☐
	Rheumatoid arthritis ☐
	Angina pectoris ☐
	Coronary Artery Disease ☐
	Diabetes Mellitus ☐

For annotations, refer to CRF page 11

	Family history of Early Coronary Artery Disease (less than 55 years of age) ☐
	Hyperlipidemia ☐
	Hypertension ☐
	Myocardial infarction ☐
	Peripheral Vascular Disease ☐
	Stroke ☐
	Transient Ischemic Attack (TIA) ☐
	Skin Squamous Cell Carcinoma ☐
	Skin Basal Cell Carcinoma ☐
	Other Malignancy ☐
	Family history of Cancer in a 1st Degree Relative (excluding skin squamous cell carcinomas and basal cell carcinomas) ☐
	Anxiety ☐
	Depression ☐
	Chronic liver disease (e.g., fatty liver disease, alcohol-induced, primary sclerosing cholangitis) ☐
	Gout ☐
	Hidradenitis suppurativa ☐
	Psoriasis ■
Does the subject have or has the subject had any of the following:	Yes ☐
	No ☐

MH = Medical History

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Past Medical History/Current Diagnoses MH_CD_001
Generated On: 13 Jul 2020 10:14:40 MHCAT = PAST MEDICAL HISTORY/CURRENT DIAGNOSES

Indicate whether any of the following conditions have ever been present or any of the following procedures have ever been performed. If unknown, choose "No".

Condition/Procedure		CHCOND; Coded; Num
MHTERM	Small bowel resection	1
	Colonic resection	2
	Incision and drainage of abscess	3
	Dilation of anal stenosis	4
	Perianal fistula surgery	5
	Seton placement	6
	Stoma placement	7
	Perianal abscess	8
	Intra-abdominal abscess	9
	Anal stenosis	10
	Colonic stricture	11
	Small bowel stricture	12
	Bowel obstruction	13
	Arthritis	14
	Arthralgia	15
	Iritis	16
	Uveitis	17
	Erythema nodosum	18
	Pyoderma gangrenosum	19
	Aphthous stomatitis	20
	Sclerosing cholangitis	21
	Autoimmune hepatitis	22
	Anal fissure	225
	Anal fistula	226
	Ileitis	227
	Dilation of colonic stricture	228
	Strictureplasty	229
	Any other intestinal fistula	230

Yes/No			CHHIST; Coded; Char
	MHPRESP = Y	MHOCCUR	Yes
			No

Condition/Procedure	
	Small bowel resection

 For annotations, refer to CRF page 38

	Colonic resection ■
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ■
	Dilation of anal stenosis ☐

For annotations, refer to CRF page 38

	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ■
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐

For annotations, refer to CRF page 38

	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ■
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐

For annotations, refer to CRF page 38

	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ■
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐

For annotations, refer to CRF page 38

	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ■
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐

For annotations, refer to CRF page 38

	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ■
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐

For annotations, refer to CRF page 38

	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ■
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐

For annotations, refer to CRF page 38

	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ■
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐

For annotations, refer to CRF page 38

	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ■
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

For annotations, refer to CRF page 38

Yes/No	
	Yes ☐
	No ☐

Condition/Procedure	
	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ■
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	
	Yes ☐
	No ☐

For annotations, refer to CRF page 38

Condition/Procedure	
	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ■
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	
	Yes ☐
	No ☐

Condition/Procedure	
	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐

For annotations, refer to CRF page 38

	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ■
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐

For annotations, refer to CRF page 38

	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ■
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐

For annotations, refer to CRF page 38

	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ■
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐

For annotations, refer to CRF page 38

	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ■
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐

For annotations, refer to CRF page 38

	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐

For annotations, refer to CRF page 38

	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐

Yes/No	Yes ☐
	No ☐

Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐

For annotations, refer to CRF page 38

	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ■
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐

For annotations, refer to CRF page 38

	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ■
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐

For annotations, refer to CRF page 38

	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ■
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐

For annotations, refer to CRF page 38

	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ■
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐

For annotations, refer to CRF page 38

	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ■
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐

For annotations, refer to CRF page 38

	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ■
	Strictureplasty ☐
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐

For annotations, refer to CRF page 38

	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐
	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ■
	Any other intestinal fistula ☐
Yes/No	Yes ☐
	No ☐
Condition/Procedure	Small bowel resection ☐
	Colonic resection ☐
	Incision and drainage of abscess ☐
	Dilation of anal stenosis ☐
	Perianal fistula surgery ☐
	Seton placement ☐
	Stoma placement ☐
	Perianal abscess ☐
	Intra-abdominal abscess ☐
	Anal stenosis ☐

For annotations, refer to CRF page 38

	Colonic stricture ☐
	Small bowel stricture ☐
	Bowel obstruction ☐
	Arthritis ☐
	Arthralgia ☐
	Iritis ☐
	Uveitis ☐
	Erythema nodosum ☐
	Pyoderma gangrenosum ☐
	Aphthous stomatitis ☐
	Sclerosing cholangitis ☐
	Autoimmune hepatitis ☐
	Anal fissure ☐
	Anal fistula ☐
	Ileitis ☐
	Dilation of colonic stricture ☐
	Strictureplasty ☐
	Any other intestinal fistula ■
Yes/No	Yes ☐
	No ☐

MH = Medical History

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Ulcerative Colitis Disease History UC_HIS_001
Generated On: 13 Jul 2020 10:14:40

MHCAT = DIAGNOSIS
MHTERM = ULCERATIVE COLITIS

Date of Initial Diagnosis of Ulcerative Colitis Disease (dd- MMM-yyyy)	MHSTDTC DHDXDT; Time; Date
Duration of Symptoms Prior to Diagnosis in Years (##)	MHDURSY in SUPPMH when IDVAR = I DHSYMYR; Entered; Ch
and Months (0 - 11 required)	MHDURSM in SUPPMH when IDVAR = I DHSYMMO; Entered; Ch

FA = Findings About Events or Interventions

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002 CMCAT = OPIOID MEDICATION HISTORY

Form: Opioid Medication History OMH

Generated On: 13 Jul 2020 10:14:40 FACAT = OPIOID MEDICATION HISTORY FAOBJ = OPIOID

Has the subject ever taken Opioids for Ulcerative Colitis Disease? CMTRT = OPIOID CMPRESP = Y CMOCCUR	Yes ☐	OMHYN; Coded; Char
	No ☐	

If Yes:

Link between related FA and RELREC

FATEST: What was the duration of treatment? FAORRES when FATESTCD = DUR	<=1 year ☐	OMHTRE; Coded; Char
	> 1 year and <= 2 years ☐	
	> 2 years ☐	

FATEST: Indicate the Ulcerative Colitis Disease related reason. FAORRES when FATESTCD = REASCDRE	Pain ☐	OMHREAS; Coded; Cha
	Diarrhea ☐	
	Both Pain and Diarrhea ☐	

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Drug History DR_HIS_001
Generated On: 13 Jul 2020 10:14:40

CMCAT = DRUG HISTORY

Indicate the total cumulative time the subject has been exposed to the following drugs. (One response MUST be selected for each specified drug.)

Drug Name	CMTRT	
		Corticosteroids (Excluding Budesonide) RXTERM; Coded; Num
		Budesonide ☐
		6-Mercaptopurine ☐
		Azathioprine ☐
		Methotrexate ☐
		Mycophenolate Mofetil ☐
		Cyclosporine ☐
		Sirolimus ☐
		Tacrolimus ☐
		Olsalazine ☐
		Balsalazide ☐
		Sulfasalazine ☐
		Mesalamine ☐
		Ciprofloxacin ☐
		Metronidazole ☐
		Rifaximin ☐
		Vedolizumab ☐
		Beclomethasone ☐
		Aminosalicylates ☐
		Tofacitinib ☐
Length of time taken	CMDURCH in SUPPCM when IDVAR is CMSEQ	Never Used ☐ RXTIMUSD; Coded; Nur
		Used <= 1 Year ☐
		Used >1 - <= 2 Years ☐
		Used > 2 Years ☐

Drug Name	CMTRT	
		Corticosteroids (Excluding Budesonide) ☐
		Budesonide
		6-Mercaptopurine ☐
		Azathioprine ☐
		Methotrexate ☐
		Mycophenolate Mofetil ☐

For annotations, refer to CRF page 66

	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ■
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐

 For annotations, refer to CRF page 66

	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ■
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐

For annotations, refer to CRF page 66

Used >1 - <= 2 Years ☐
Used > 2 Years ☐

Drug Name — Corticosteroids (Excluding Budesonide) ☐
Budesonide ☐
6-Mercaptopurine ☐
Azathioprine ☐
Methotrexate ■
Mycophenolate Mofetil ☐
Cyclosporine ☐
Sirolimus ☐
Tacrolimus ☐
Olsalazine ☐
Balsalazide ☐
Sulfasalazine ☐
Mesalamine ☐
Ciprofloxacin ☐
Metronidazole ☐
Rifaximin ☐
Vedolizumab ☐
Beclomethasone ☐
Aminosalicylates ☐
Tofacitinib ☐

Length of time taken — Never Used ☐
Used <= 1 Year ☐
Used >1 - <= 2 Years ☐
Used > 2 Years ☐

Drug Name — Corticosteroids (Excluding Budesonide) ☐
Budesonide ☐
6-Mercaptopurine ☐
Azathioprine ☐
Methotrexate ☐

For annotations, refer to CRF page 66

	Mycophenolate Mofetil ■
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ■
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐

For annotations, refer to CRF page 66

	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ■
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐

 For annotations, refer to CRF page 66

Used <= 1 Year ☐
Used >1 - <= 2 Years ☐
Used > 2 Years ☐

Drug Name — Corticosteroids (Excluding Budesonide) ☐
Budesonide ☐
6-Mercaptopurine ☐
Azathioprine ☐
Methotrexate ☐
Mycophenolate Mofetil ☐
Cyclosporine ☐
Sirolimus ☐
Tacrolimus ■
Olsalazine ☐
Balsalazide ☐
Sulfasalazine ☐
Mesalamine ☐
Ciprofloxacin ☐
Metronidazole ☐
Rifaximin ☐
Vedolizumab ☐
Beclomethasone ☐
Aminosalicylates ☐
Tofacitinib ☐

Length of time taken — Never Used ☐
Used <= 1 Year ☐
Used >1 - <= 2 Years ☐
Used > 2 Years ☐

Drug Name — Corticosteroids (Excluding Budesonide) ☐
Budesonide ☐
6-Mercaptopurine ☐
Azathioprine ☐

For annotations, refer to CRF page 66

	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ■
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ■
	Sulfasalazine ☐

For annotations, refer to CRF page 66

	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ■
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐

 For annotations, refer to CRF page 66

Length of time taken

- ☐ Never Used
- ☐ Used <= 1 Year
- ☐ Used >1 - <= 2 Years
- ☐ Used > 2 Years

Drug Name

- ☐ Corticosteroids (Excluding Budesonide)
- ☐ Budesonide
- ☐ 6-Mercaptopurine
- ☐ Azathioprine
- ☐ Methotrexate
- ☐ Mycophenolate Mofetil
- ☐ Cyclosporine
- ☐ Sirolimus
- ☐ Tacrolimus
- ☐ Olsalazine
- ☐ Balsalazide
- ☐ Sulfasalazine
- ■ Mesalamine
- ☐ Ciprofloxacin
- ☐ Metronidazole
- ☐ Rifaximin
- ☐ Vedolizumab
- ☐ Beclomethasone
- ☐ Aminosalicylates
- ☐ Tofacitinib

Length of time taken

- ☐ Never Used
- ☐ Used <= 1 Year
- ☐ Used >1 - <= 2 Years
- ☐ Used > 2 Years

Drug Name

- ☐ Corticosteroids (Excluding Budesonide)
- ☐ Budesonide
- ☐ 6-Mercaptopurine

For annotations, refer to CRF page 66

	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ■
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐

For annotations, refer to CRF page 66

	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ■
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ■
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ☐

 For annotations, refer to CRF page 66

	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ■
	Beclomethasone ☐
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐

For annotations, refer to CRF page 66

	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ■
	Aminosalicylates ☐
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐

For annotations, refer to CRF page 66

	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐
	Aminosalicylates ■
	Tofacitinib ☐
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐
Drug Name	Corticosteroids (Excluding Budesonide) ☐
	Budesonide ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Mycophenolate Mofetil ☐
	Cyclosporine ☐
	Sirolimus ☐
	Tacrolimus ☐
	Olsalazine ☐
	Balsalazide ☐
	Sulfasalazine ☐
	Mesalamine ☐
	Ciprofloxacin ☐
	Metronidazole ☐
	Rifaximin ☐
	Vedolizumab ☐
	Beclomethasone ☐

For annotations, refer to CRF page 66

	Aminosalicylates ☐
	Tofacitinib ■
Length of time taken	Never Used ☐
	Used <= 1 Year ☐
	Used >1 - <= 2 Years ☐
	Used > 2 Years ☐

SU = Substance Use

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Substance Use Tobacco SU_GL_900B
Generated On: 13 Jul 2020 10:14:40

Does the subject have ANY history of Tobacco/Nicotine use?	[NOT SUBMITTED] SUOCCUR=N when SUTRT = TOBACCO/NICOTINE	Yes ☐ No ☐	SUTOBYN; Coded; Cha
Substance Category	SUCAT	TOBACCO	SUCAT; Entered; Char
Type of Substance	SUTRT	Tobacco ☐ Cigarettes ■ Cigars ☐ Pipes ☐ Smokeless tobacco ☐ Patches/Gum ☐ E-cigarettes or equivalent ☐	SUTRT; Coded; Char
Usage	SUPRESP = Y SUNCF in SUPPSU when IDVAR = SUSEQ	SUOCCUR = Y Current ☐ SUOCCUR = Y Former ☐ SUOCCUR = N Never ☐	SUNCF; Coded; Char
Amount	SUDOSTXT, SUDOSE		SUDSTXT; Entered; Nur
Unit	SUDOSU	Pack ☐ Pouch Dosing Unit ☐ Patch Dosing Unit ☐ Gram ☐ Milligram ☐ Gum Dosage Form ☐ Cigarette Dosage Form ☐ Cigar Dosage Form ☐ Pipe ☐ Milligram per Milliliter ☐	SUDOSU; Coded; Char
Frequency	SUDOSFRQ	Per Day ☐ Per Week ☐ Per Month ☐ Occasional ☐	SUDOSFRQ; Coded; Ch
Start Date	SUSTDTC		SUSTDAT; Time; Date
End Date (*if applicable*)	SUENDTC		SUENDAT; Time; Date
Substance Category		TOBACCO	
Type of Substance		Tobacco ☐	

For annotations, refer to CRF page 82

	Cigarettes ☐
	Cigars ■
	Pipes ☐
	Smokeless tobacco ☐
	Patches/Gum ☐
	E-cigarettes or equivalent ☐
Usage	Current ☐
	Former ☐
	Never ☐
Amount	_____
Unit	Pack ☐
	Pouch Dosing Unit ☐
	Patch Dosing Unit ☐
	Gram ☐
	Milligram ☐
	Gum Dosage Form ☐
	Cigarette Dosage Form ☐
	Cigar Dosage Form ☐
	Pipe ☐
	Milligram per Milliliter ☐
Frequency	Per Day ☐
	Per Week ☐
	Per Month ☐
	Occasional ☐
Start Date	_____
End Date (*if applicable*)	_____
Substance Category	TOBACCO
Type of Substance	Tobacco ☐
	Cigarettes ☐
	Cigars ☐
	Pipes ■
	Smokeless tobacco ☐
	Patches/Gum ☐

For annotations, refer to CRF page 82

	E-cigarettes or equivalent ☐
Usage	Current ☐
	Former ☐
	Never ☐
Amount	_____
Unit	Pack ☐
	Pouch Dosing Unit ☐
	Patch Dosing Unit ☐
	Gram ☐
	Milligram ☐
	Gum Dosage Form ☐
	Cigarette Dosage Form ☐
	Cigar Dosage Form ☐
	Pipe ☐
	Milligram per Milliliter ☐
Frequency	Per Day ☐
	Per Week ☐
	Per Month ☐
	Occasional ☐
Start Date	_____
End Date (*if applicable*)	_____
Substance Category	TOBACCO
Type of Substance	Tobacco ☐
	Cigarettes ☐
	Cigars ☐
	Pipes ☐
	Smokeless tobacco ■
	Patches/Gum ☐
	E-cigarettes or equivalent ☐
Usage	Current ☐
	Former ☐
	Never ☐

For annotations, refer to CRF page 82

Amount	_____
Unit	Pack ☐
	Pouch Dosing Unit ☐
	Patch Dosing Unit ☐
	Gram ☐
	Milligram ☐
	Gum Dosage Form ☐
	Cigarette Dosage Form ☐
	Cigar Dosage Form ☐
	Pipe ☐
	Milligram per Milliliter ☐
Frequency	Per Day ☐
	Per Week ☐
	Per Month ☐
	Occasional ☐
Start Date	_____
End Date (*if applicable*)	_____
Substance Category	NICOTINE
Type of Substance	Tobacco ☐
	Cigarettes ☐
	Cigars ☐
	Pipes ☐
	Smokeless tobacco ☐
	Patches/Gum ■
	E-cigarettes or equivalent ☐
Usage	Current ☐
	Former ☐
	Never ☐
Amount	_____
Unit	Pack ☐
	Pouch Dosing Unit ☐
	Patch Dosing Unit ☐
	Gram ☐

For annotations, refer to CRF page 82

	Milligram ☐
	Gum Dosage Form ☐
	Cigarette Dosage Form ☐
	Cigar Dosage Form ☐
	Pipe ☐
	Milligram per Milliliter ☐
Frequency	Per Day ☐
	Per Week ☐
	Per Month ☐
	Occasional ☐
Start Date	_____
End Date (*if applicable*)	_____
Substance Category	NICOTINE
Type of Substance	Tobacco ☐
	Cigarettes ☐
	Cigars ☐
	Pipes ☐
	Smokeless tobacco ☐
	Patches/Gum ☐
	E-cigarettes or equivalent ■
Usage	Current ☐
	Former ☐
	Never ☐
Amount	_____
Unit	Pack ☐
	Pouch Dosing Unit ☐
	Patch Dosing Unit ☐
	Gram ☐
	Milligram ☐
	Gum Dosage Form ☐
	Cigarette Dosage Form ☐
	Cigar Dosage Form ☐
	Pipe ☐

For annotations, refer to CRF page 82

	Milligram per Milliliter ☐
Frequency	Per Day ☐
	Per Week ☐
	Per Month ☐
	Occasional ☐
Start Date	
End Date (*if applicable*)	

SU = Substance Use

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Substance Use Alcohol SU_GL_900A
Generated On: 13 Jul 2020 10:14:40

Substance Category	SUCAT ALCOHOL	SUCAT; Entered; Char
Does the subject have ANY history of Alcohol use?	[NOT SUBMITTED] Yes ☐ SUOCCUR=N when SUTRT = ALCOHOL No ☐	SUALCYN; Coded; Char
Type of Substance	Alcohol ☐ SUTRT Beer ■ Wine ☐ Distilled Spirits ☐	SUTRT; Coded; Char
Usage	SUPRESP = Y SUOCCUR = Y Current ☐ SUNCF in SUPPSU when IDVAR = SUSEQ SUOCCUR = Y Former ☐ SUOCCUR = N Never ☐	SUNCF; Coded; Char
Amount	SUDOSTXT, SUDOSE	SUDSTXT; Entered; Nu
Unit	mL ☐ SUDOSU Glass Dosing Unit ☐ Bottle Dosing Unit ☐ Can Dosing Unit ☐	SUDOSU; Coded; Char
Frequency	Per Day ☐ SUDOSFRQ Per Week ☐ Per Month ☐ Occasional ☐	SUDOSFRQ; Coded; Ch
Start Date	SUSTDTC	SUSTDAT; Time; Date
End Date (*if applicable*)	SUENDTC	SUENDAT; Time; Date
Type of Substance	Alcohol ☐ Beer ☐ Wine ■ Distilled Spirits ☐	
Usage	Current ☐ Former ☐ Never ☐	
Amount		
Unit	mL ☐ Glass Dosing Unit ☐ Bottle Dosing Unit ☐	

 For annotations, refer to CRF page 88

	Can Dosing Unit ☐
Frequency	Per Day ☐ Per Week ☐ Per Month ☐ Occasional ☐
Start Date	
End Date (*if applicable*)	
Type of Substance	Alcohol ☐ Beer ☐ Wine ☐ Distilled Spirits ■
Usage	Current ☐ Former ☐ Never ☐
Amount	
Unit	mL ☐ Glass Dosing Unit ☐ Bottle Dosing Unit ☐ Can Dosing Unit ☐
Frequency	Per Day ☐ Per Week ☐ Per Month ☐ Occasional ☐
Start Date	
End Date (*if applicable*)	

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Corticosteroids and Immunomodulators Failure Status BFSCI
Generated On: 13 Jul 2020 10:14:40 FACAT = CORTICOSTEROIDS/IMMUNOMODULATORS FAILURE

Immunomodulators (6-MP/AZA) FASCAT

Have had evidence of an initial inadequate response, recurrent disease, or a relapse despite receiving:

At least 3 months of therapy with 1 mg/kg/day of 6-MP or 2 mg/kg/day of AZA — FAOBJ — FAORRES when FATESTCD = OCCUR — Yes ☐ No ☐ — IMM1; Coded; Char

A lower dosage of 6-MP or AZA when country or local guidelines specify a different treatment regimer — FAOBJ — FAORRES when FATESTCD = OCCUR — Yes ☐ No ☐ — IMM2; Coded; Char

The dosage of 6-MP or AZA confirmed to be therapeutic for the subject with whole blood thioguanine nucleotide levels >200 pmole/8 x 10^8 RBCs — FAOBJ = DOSAGE OF 6-MP OR AZA CONFIRMED TO BE THERAPEUTIC FOR THE SUBJECT — FAORRES when FATESTCD = OCCUR — Yes ☐ — IMM3; Coded; Char

The highest tolerated dosage due to leukopenia, elevated liver enzymes or nausea — FAOBJ — FAORRES when FATESTCD = OCCUR — Yes ☐ No ☐ — IMM4; Coded; Char

Have developed clinically significant adverse events unresponsive to dose reduction which in the judgment of the investigator, precluded the use of 6-MP or AZA to treat UC within the past 5 years — [NOT SUBMITTED] — Yes ☐ No ☐ — IMM5; Coded; Char

If Yes, check all that apply:

FAOBJ

	FAORRES when FATESTCD = OCCUR	
Pancreatitis		PANCREAT; Checked; I
Arthritis accompanied by high fever and/or rash		ARTHRIT; Checked; Nu
Leukopenia		LEUKOPE; Checked; Nu
Persistently elevated liver enzymes		LIVERENZ; Checked; Nu
Other		OTHER; Checked; Num

Other, Specify — FAPTUCO in SUPPFA when IDVAR = FASEQ — _____ — OTHERSPE; Entered; C

Have a medical condition that precludes the use of 6-MP or AZA — FAOBJ = MEDICAL CONDITION THAT PRECLUDES THE USE OF 6-MP OR AZA — FAORRES when FATESTCD = OCCUR — Yes ☐ No ☐ — IMM6; Coded; Char

Corticosteroids

Have had evidence of an initial inadequate response, recurrent disease, or a relapse despite receiving at least 0.75 mg/kg or ≥ 40mg/day of prednisone (or corticosteroid equivalent, given orally or intravenously) for 2 weeks; or ≥9 mg/day of budesonide or ≥5 mg/day of b orally for at least 4 weeks — FAORRES when FATESTCD = OCCUR — FAOBJ = EVIDENCE OF AN INITIAL INADEQUATE RESPONSE, RECURRENT DISEASE, OR A RELAPSE — Yes ☐ No ☐ — COR1; Coded; Char

Have failed to successfully taper to less than 10 mg per day of prednisone, that is had a flare of disease, within 3 months of starting therapy, or if a relapse occurs within 3 months of stopping corticosteroids — FAOBJ = DISEASE RELAPSE WITHIN 3 MONTHS OF STARTING OR STOPPING CORTICOSTEROIDS — FAORRES when FATESTCD = OCCUR — Yes ☐ No ☐ — COR2; Coded; Char

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Corticosteroids and Immunomodulators Failure Status *BFSCI*
Generated On: 13 Jul 2020 10:14:40

Have developed clinically significant adverse events unresponsive to dose reduction that, in the judgment of the investigator, precluded the use of corticosteroids to treat UC	*[NOT SUBMITTED]*	Yes ☐ No ☐	*COR3; Coded; Char*
If Yes, check all that apply			
Osteonecrosis	*FAORRES when FATESTCD = OCCUR*		*OSTEONECROSIS; Che*
Osteoporosis			*OSTEOPOROSIS; Chec*
Psychosis			*PSYCHOSIS; Checked;*
Uncontrolled Diabetes			*UNCRODIA; Checked; N*
Other			*OTHER1; Checked; Nun*
Other, Specify	*FAPTUCO in SUPPFA when IDVAR = FASEQ*		*OTHSPEC; Entered; Ch*
Have a medical condition that precludes the use of corticosteroids as treatment for UC	*FAORRES when FATESTCD = OCCUR*	Yes ☐ No ☐	*COR4; Coded; Char*

FAOBJ

FAOBJ = MEDICAL CONDITION THAT PRECLUDES THE USE OF CORTICOSTEROIDS AS TREATMENT FOR UC

FATEST = Occurrence

Were any adverse events experienced?	Yes ☐	***AEYN; Coded; Char***
	No ☐	

AE = Adverse Events

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Adverse Events/Serious AEs AE_GL_900
Generated On: 13 Jul 2020 10:14:40

What is the adverse event term?	AETERM	AETERM; Entered; Char
Is this an event of suicidal ideation or behavior? AESIBEYN in SUPPAE when IDVAR = AESEQ	Yes ☐ No ☐	AESIBEYN; Coded; Cha
If yes, specify if the event is suicidal ideation, suicidal behavior excluding completed suicide, or completed suicide AEEVTYP in SUPPAE when IDVAR = AESEQ	Suicidal Ideation ☐ Suicidal Behavior excluding Completed Suicide ☐ Completed Suicide ☐	AESIBE; Coded; Char
Start Date	AESTDTC	AESTDAT; Time; Date
Start Time		AESTTIM; Time; Date
End Date	AEENDTC	AEENDAT; Time; Date
End Time		AEENTIM; Time; Date
Is the adverse event still ongoing?	AEENRF = AFTER	AEONGO; Checked; Nu
Severity AESEV	Mild ☐ Moderate ☐ Severe ☐	AESEV; Coded; Char
Action Taken with Study Treatment AEACN	Dose Not Changed ☐ Drug Interrupted ☐ Drug Withdrawn ☐ Not Applicable ☐	AEACN; Coded; Char
Other Action Taken	AEACNOTH	AEACNOTH; Entered; C
Concomitant or additional therapy given for this adverse event? AECONTRT	Yes ☐ No ☐ Not Applicable ☐ Unknown ☐	AECONTRT; Coded; Ch
Is this an infection? AEINF in SUPPAE when IDVAR = AESEQ	Yes ☐ No ☐	AEINF; Coded; Char
If yes, was infection treated with oral or parenteral antibiotics? AETRTOPA in SUPPAE when IDVAR = AESEQ	Yes ☐ No ☐	AETRTOPA; Coded; Ch
Was this event a study treatment injection site reaction? AEINJSRC in SUPPAE when IDVAR = AESEQ	Yes ☐ No ☐	AEINJSRC; Coded; Cha
If Yes, specify injection site AEINJS in SUPPAE when IDVAR = AESEQ	Upper Right Arm ☐ Upper Left Arm ☐	AEINJS; Coded; Char

	Right Thigh ☐	
	Left Thigh ☐	
	Lower Right Abdominal ☐	
	Lower Left Abdominal ☐	
	Other ☐	
If Other, specify	*AEINJSO in SUPPAE when IDVAR = AESEQ*	*AEINJSO; Entered; Cha*
Relationship to Study Treatment	Not Related ☐	*AEREL; Coded; Char*
	Doubtful ☐	
AEREL	Possible ☐	
	Probable ☐	
	Very Likely ☐	
Outcome	Fatal ☐	*AEOUT; Coded; Char*
	Not Recovered or Not Resolved ☐	
AEOUT	Recovered or Resolved ☐	
	Recovered or Resolved with Sequelae ☐	
	Recovering or Resolving ☐	
	Unknown ☐	
Is the adverse event serious?	Yes ☐	*AESER; Coded; Char*
AESER	No ☐	

The following information should be completed ONLY for Serious Adverse Events

Date Investigator/Investigational Staff became aware of SAE	*[NOT SUBMITTED]*	*AEAWARDT; Time; Date*
Age at onset of SAE	*AEONAGE in SUPPAE when IDVAR = AESEQ*	*AEAGE; Entered; Num*
Age unit	Hours ☐	*AGEU; Coded; Char*
	Days ☐	
AEONAGEU in SUPPAE when IDVAR = AESEQ	Weeks ☐	
	Months ☐	
	Years ■	
Weight	**kg** ☐	*AEWEIGHT; Entered; Nu*
[NOT SUBMITTED]	lb ☐	
Height	**cm** ☐	*AEHEIGHT; Entered; Nu*
[NOT SUBMITTED]	in ☐	
Related to Trial Procedure	Yes ☐	*AETRLPRC; Coded; Ch*
AETRLPRC in SUPPAE when IDVAR = AESEQ		

		No ☐	
Congenital Anomaly or Birth Defect	*AESCONG*	Yes ☐ No ☐	*AESCONG; Coded; Cha*
Significant Disability	*AESDISAB*	Yes ☐ No ☐	*AESDISAB; Coded; Cha*
Death	*AESDTH*	Yes ☐ No ☐	*AESDTH; Coded; Char*
Date of death (if applicable)	*[NOT SUBMITTED]*		*AEDTHDAT; Time; Date*
Cause of Death	*[NOT SUBMITTED]*		*AEDTHRES; Entered; C*
Was autopsy performed? (if applicable)	*[NOT SUBMITTED]*	Yes ☐ No ☐	*AEAUTOP; Coded; Cha*
Prolonged Hospitalization	*AESHOSPP in SUPPAE when IDVAR = AESEQ*	Yes ☐ No ☐	*AESHOSPP; Coded; Ch*
Required Hospitalization	*AESHOSPR in SUPPAE when IDVAR = AESEQ*	Yes ☐ No ☐	*AESHOSPR; Coded; Ch*
If event required or prolonged hospitalization, specify admission date	*[NOT SUBMITTED]*		*AEADMDAT; Time; Date*
Hospital discharge date	*[NOT SUBMITTED]*		*AEDISDAT; Time; Date*
Life Threatening	*AESLIFE*	Yes ☐ No ☐	*AESLIFE; Coded; Char*
Other Medically Important Event	*AESMIE*	Yes ☐ No ☐	*AESMIE; Coded; Char*
Other Medically Important Event Specification	*AESOSP in SUPPAE when IDVAR = AESEQ*		*AESOSP; Entered; Cha*
Did the device cause or contribute to the SAE?	*AERELDEV in SUPPAE when IDVAR = AESEQ*	Yes ☐ No ☐	*AERELDEV; Coded; Ch*
Trial Design	*[NOT SUBMITTED]*	Open-label only ☐ Blinded only ☐ Open-label phase of multi-phased trial ☐ Blinded phase of multi-phased trial ☐	*AETRIDES; Coded; Cha*

Was blind broken?	Yes ☐ No ☐	*AEBLIDBK; Coded; Cha*
Study Agent and Dosing Information		
Did the subject receive any study agent?	Yes ☐ No ☐	*AEAGENT1; Coded; Ch*
Name of study agent	GUSELKUMAB OR PLACEBO ULTRASAFE PLUS/ GOLIMUMAB OR PLACEBO	*AEAGNNM1; Entered; C*
Date of first dose of study agent		*AEASDAT1; Time; Date*
Time of first dose of study agent		*AEASTIM1; Time; Date*
Date of most recent dose of study agent		*AEAEDAT1; Time; Date*
Time of most recent dose of study agent		*AAENTIM1; Time; Date*
Indication	ULCERATIVE COLITIS	*AEAGIND1; Entered; Ch*
Batch/lot no.		*AEAGNUM1; Entered; C*
Dose		*AEAGDSE1; Entered; N*
Dose Unit	mL	*AEAGDSU1; Entered; C*
Dose Frequency	Weekly ☐ Every 2 Weeks ☐ Every 4 Weeks ☐ Every 8 Weeks ☐	*AEAGFRQ1; Coded; Ch*
Route	Intravenous ☐ Subcutaneous ☐	*AEAGRTE1; Coded; Ch*

Were any medications taken?	Yes ☐	***CMYN; Coded; Char***
	No ☐	

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Concomitant Therapy CM_GL_900
Generated On: 13 Jul 2020 10:14:40

Medication/Therapy Category	CMCAT	GENERAL CONMED	CMCAT; Entered; Char
Medication or Therapy	CMTRT		CMTRT; Entered; Char
Indication		Adverse Event ☐	CMINDC; Coded; Char
	CMINDC	Trial Indication ☐	
		Other ☐	
If indication is 'Adverse Event,' choose the primary corresponding AE log line(s), start date(s), and term(s)			AEDSL1; Entered; Char
AE log line, start date, and term	Linked to related AE record via RELREC		
AE log line, start date, and term			AEDSL2; Entered; Char
AE log line, start date, and term			AEDSL3; Entered; Char
AE log line, start date, and term			AEDSL4; Entered; Char
AE log line, start date, and term			AEDSL5; Entered; Char
Indication Specification 'Other', please specify	CMINDDSC in SUPPCM when IDVAR = CMSEQ		CMINDDSC; Entered; Ch
Dose	CMDOSE, CMDOSTXT		CMDSTXT; Entered; Ch
Dose Unit		Capsule ☐	CMDOSU; Coded; Char
		Drop ☐	
		Gram ☐	
		Liter ☐	
		Milliequivalent ☐	
	CMDOSU	Microgram ☐	
		Milligram ☐	
		Patch ☐	
		Puff ☐	
		Tablespoon ☐	
		Tablet ☐	
		Teaspoon ☐	
		Unit ☐	
		Microliter ☐	
		Other ☐	
Dose Form	CMDOSFRM	Aerosol ☐	CMDOSFRM; Coded; Ch
		Capsule ☐	
		Cream ☐	
		Delayed Release Capsule ☐	
		Delayed Release Tablet ☐	

	Extended Release Capsule	☐
	Extended Release Tablet	☐
	For Solution	☐
	For Suspension	☐
	Gel	☐
	Implant	☐
	Inhalant	☐
	Injectable	☐
	Liquid	☐
	Lotion	☐
	Mouthwash	☐
	Ointment	☐
	Patch	☐
	Powder	☐
	Shampoo	☐
	Solution	☐
	Soap	☐
	Spray	☐
	Suppository	☐
	Suspension	☐
	Syrup	☐
	Tablet	☐
	Tape	☐
	Unassigned	☐
	Not Applicable	☐
Frequency	Once	☐ *CMDOSFRQ; Coded; Ch*
	Infrequent	☐
	As Necessary	☐
	Continue	☐
	Daily	☐
CMDOSFRQ	Twice Daily	☐
	Three Times Daily	☐
	Four Times Daily	☐

		Every Other Day ☐	
		Weekly ☐	
		Twice Weekly ☐	
		Three Times Weekly ☐	
		Four Times Weekly ☐	
		Every Two Weeks ☐	
		Every Four Weeks ☐	
		Monthly ☐	
		Twice Per Month ☐	
		Every Three Months ☐	
		Unknown ☐	
		Other ☐	
Route		Auricular ☐	*CMROUTE; Coded; Cha*
		Inhalation ☐	
		Intramuscular ☐	
		Intravenous ☐	
		Nasal ☐	
		Ophthalmic ☐	
	CMROUTE	Oral ☐	
		Parenteral ☐	
		Rectal ☐	
		Subcutaneous ☐	
		Sublingual ☐	
		Topical ☐	
		Intralesional ☐	
		Nasogastric ☐	
		Transdermal ☐	
		Not Applicable ☐	
		Unknown ☐	
		Other ☐	
Start Date	*CMSTDTC*		*CMSTDAT; Time; Date*
If Start Date is entirely or partially unknown: Was the medication/therapy taken prior to the study? *CMSTTPT = <INFORMED CONSENT DATE>*	*CMSTRTPT = BEFORE*	Yes ☐	*CMPRIOR; Coded; Char*
	CMSTRTPT = AFTER	No ☐	

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Concomitant Therapy CM_GL_900
Generated On: 13 Jul 2020 10:14:40

End Date	CMENDTC	CMENDAT; Time; Date
Is the medication/therapy still ongoing?	CMENRF = AFTER	CMONGO; Checked; Nu
Derived Indication	[NOT SUBMITTED]	INDICATN; Entered; Cha

Category	MEDICAL ENCOUNTERS *HOCAT; Entered; Char*	
Did the subject require any medical encounters other than those mandated per trial protocol?	Yes ☐	*HOYN; Coded; Char*
	No ☐	

HO = Healthcare Encounters

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Medical Encounters HO_GL_900 HOCAT = MEDICAL ENCOUNTERS
Generated On: 13 Jul 2020 10:14:40

Collection Date	HODTC	HODAT; Time; Date
Start Date	HOSTDTC	HOSTDAT; Time; Date
Is the medical encounter still ongoing?	HOENRF = AFTER	HOONGO; Checked; Nu
End Date	HOENDTC	HOENDAT; Time; Date
Reason for the medical encounter HOINDC in SUPPHO when IDVAR = HOSEQ	☐ Adverse Event ☐ Other	HOINDC; Coded; Char
If Adverse Event, choose the corresponding AE log line, start date, and term AE log line, start date, and term	Linked to related AE record via RELREC	AEDSL1; Entered; Char
AE log line, start date, and term		AEDSL2; Entered; Char
AE log line, start date, and term		AEDSL3; Entered; Char
AE log line, start date, and term		AEDSL4; Entered; Char
AE log line, start date, and term		AEDSL5; Entered; Char
If Other, Specify	HOINDCO in SUPPHO when IDVAR = HOSEQ	HOINDCO; Entered; Cha
Type of medical encounter HOTERM	☐ ER and Hospitalization ☐ Home Care ☐ Hospital Outpatient Department ☐ Medical Practitioner Office ☐ Outpatient Imaging (MRI, CT, etc.) - Abdominal ☐ Outpatient Imaging (MRI, CT, etc.) - Non-Abdominal ☐ Nutrition and/or Psychological Counselling	HOTERM; Coded; Char
If Outpatient Imaging - Abdominal/Non-abdominal Types HOTERMOI in SUPPHO when IDVAR = HOSEQ	☐ Endoscopy ☐ MRI ☐ CT ☐ X-Ray ☐ Other	HOTERMOI; Coded; Cha
If Other, Specify	HOTMOIO in SUPPHO when IDVAR = HOSEQ	HOTERMOI_OTH; Enter
If Hospital Outpatient Department or Medical Practitioner Office is selected, specify Type of Practitioner HOPRACT in SUPPHO when IDVAR = HOSEQ	☐ Cardiologist ☐ Dermatologist ☐ Gastroenterologist ☐ General Practitioner ☐ Hematologist	HOPRACT; Coded; Cha

Hepatologist	☐
Infectious Disease Specialist	☐
Nephrologist	☐
Nurse	☐
Oncologist	☐
Psychologist	☐
Psychiatrist	☐
Radiologist	☐
Study Physician	☐
Social Worker	☐
Surgeon	☐
Urologist	☐
Other	☐

If Other, Specify	HOPRACTO in SUPPHO when IDVAR = HOSEQ	HOPRACT_OTH; Entere
If ER and Hospitalization is selected, specify	Yes ☐	HOTERMEV; Coded; Ch
Was the ER used?	HOTERMEV in SUPPHO when IDVAR = HOSEQ No ☐	
How many days in ICU	HOTEMICU in SUPPHO when IDVAR = HOSEQ	HOTERMEV_ICU; Enter
How many days inpatient hospital	HOTEMINH in SUPPHO when IDVAR = HOSEQ	HOTERMEV_INH; Enter

Record all UC-related surgeries/procedures using the dropdown list provided. If a surgery/procedure is not present, please specify 'Other' in the dropdown box and put the type of surgery/procedure in "Other Surgery/procedure" space provided. Add a new page for each surgery/procedure. [NOT SUBMITTED]

Surgery/Procedure	PRTRT	PRTRT; Coded; Num
	Apheresis ☐	30
	Barium enema ☐	1
	Biopsy ☐	69 4
	CAT scan ☐	2 21
	Colonoscopy ☐	20 9
	Colostomy - permanent ☐	10 11
	Colostomy - temporary ☐	3 13
	Drainage - open ☐	12 22
	Drainage - percutaneous ☐	19 5
	Drainage - seton placement ☐	23 99
	Flexible sigmoidoscopy ☐	15 14
	Gastrostomy - other ☐	16 65
	Gastrostomy - percutaneous ☐	66 25
	Ileostomy ☐	26 24
	Intestinal anastomosis ☐	8 18
	MRI ☐	28 27
	Other enterostomy ☐	6
[NOT SUBMITTED]	Other surgery ☐	
	Partial gastrectomy w/anast. - duodenum ☐	
	Partial gastrectomy w/anast. - esophagus ☐	
	Partial gastrectomy w/anast. - jejunum ☐	
	Resection of colon - partial ☐	
	Resection of colon - total ☐	
	Resection of rectum - abdominoperineal ☐	
	Resection of rectum - other ☐	
	Resection of rectum - pull-through ☐	
	Small bowel enteroclysis ☐	
	Total gastrectomy ☐	

PR = Procedures

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Current Ulcerative Colitis Surgeries/ Procedures ***UC_SP_001***
Generated On: 13 Jul 2020 10:14:40

		TPN ☐	
		Tube feeding ☐	
		Ultrasonography ☐	
Other Surgery/Procedure (specify)	___ *PRTRT*		*PROTHER; Entered; Ch*
Reason for Surgery/Procedure	___ *PRINDC*		*PRREASON; Entered; C*
Surgery/Procedure performed *PRPINOUT in SUPPPR when IDVAR = PRSEQ*		In-Patient ☐ Out-Patient ☐	*PRPRFM; Coded; Num* *1* *2*
Surgery/Procedure Date (dd- MMM- yyyy)	___ *PRSTDTC*		*PRSTDAT; Time; Date*
If TPN or Tube Feeding: Stop Date (dd- MMM- yyyy)	___ *PRENDTC*		*PRSTOP; Time; Date*
OR Ongoing	*PRENRF*		*PRONGO; Checked; Nu*

NOTE:
If check box selected for 'OR Ongoing' then PRENRF = AFTER,
else PRENRF = NULL (with assumption that end date provided)
else PRENRF = DURING

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Prohibited Medication Review PMR
Generated On: 13 Jul 2020 10:14:40

Participants who initiate the following treatments during study participation will have their study intervention discontinued

Is the subject taking any prohibited medication?		[NOT SUBMITTED] Yes	PMRYN; Coded; Char
		No	
Medication Class	FACAT	PROHIBITED MEDICATION	PMRCAT; Coded; Char 1
Medication FATEST = Occurence FAORRES when FATESTCD = OCCUR	FAOBJ	6-TG	PMRMED; Coded; Char
		Cyclosporine	
		Mycophenolate Mofetil	
		Tacrolimus	
		Sirolimus	
		Infliximab	
		Etanercept	
		Certolizumab pegol	
		Adalimumab	
		Natalizumab	
		Ustekinumab	
		Rituximab	
		Vedolizumab	
		Upadacitinib	
		Filgotinib	
		Ozanimod	
		Etrolizumab	
		Brazikumab	
		Briakinumab	
		Mirikizumab	
		Risankizumab	
		Rhalidomide	
		Thalidomide	
		Other [NOT SUBMITTED]	
		6-Mercaptopurine	
		Azathioprine	
		Methotrexate	
		Tofacitinib	

		Abatacept ☐	
		Anakinra ☐	
		Tildrakizumab ☐	
		Alemtuzumab ☐	
	Other medication, specify	*FAOBJ*	*PMRMEDO; Entered; Ch*
FATEST	Date of Initiation	*FAORRES when FATESTCD = CHDATE*	*PMRDAT; Time; Date*
FATEST	Reason for Initiation *FAORRES when FATESTCD = REAS*	Worsening of Ulcerative Colitis ☐	*PMRREAS; Coded; Cha*
		[NOT SUBMITTED] Other ☐	
	If Other, specify	*FAORRES when FATESTCD = REAS*	*PMRREASO; Entered; C*
	Please select concomitant therapy log line, start date and therapy	*Linked to related CM record via REL*	*CMDSL; Entered; Char*
	Medication Class	PROHIBITED MEDICATION ■	
	Medication	6-TG ☐	
		Cyclosporine ☐	
		Mycophenolate Mofetil ☐	
		Tacrolimus ☐	
		Sirolimus ☐	
		Infliximab ☐	
		Etanercept ☐	
		Certolizumab pegol ☐	
		Adalimumab ☐	
		Natalizumab ☐	
		Ustekinumab ☐	
		Rituximab ☐	
		Vedolizumab ☐	
		Upadacitinib ☐	
		Filgotinib ☐	
		Ozanimod ☐	
		Etrolizumab ☐	
		Brazikumab ☐	
		Briakinumab ☐	
		Mirikizumab ☐	
		Risankizumab ☐	
		Rhalidomide ☐	

For annotations, refer to CRF page 107, 108

	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ■
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐

For annotations, refer to CRF page 107, 108

	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ■
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ■
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐

For annotations, refer to CRF page 107, 108

	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ■
	Mycophenolate Mofetil ☐
	Tacrolimus ☐

For annotations, refer to CRF page 107, 108

	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	_____
Date of Initiation	_____
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	_____

For annotations, refer to CRF page 107, 108

Please select concomitant therapy log line, start date and therapy

Medication Class	PROHIBITED MEDICATION
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ■
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐

For annotations, refer to CRF page 107, 108

	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ■ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ☐ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ☐ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ☐ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ☐ 6-Mercaptopurine ☐

For annotations, refer to CRF page 107, 108

	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ■
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐

For annotations, refer to CRF page 107, 108

	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ■
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐

For annotations, refer to CRF page 107, 108

	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ■

For annotations, refer to CRF page 107, 108

	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐

For annotations, refer to CRF page 107, 108

Cyclosporine ☐
Mycophenolate Mofetil ☐
Tacrolimus ☐
Sirolimus ☐
Infliximab ☐
Etanercept ☐
Certolizumab pegol ■
Adalimumab ☐
Natalizumab ☐
Ustekinumab ☐
Rituximab ☐
Vedolizumab ☐
Upadacitinib ☐
Filgotinib ☐
Ozanimod ☐
Etrolizumab ☐
Brazikumab ☐
Briakinumab ☐
Mirikizumab ☐
Risankizumab ☐
Rhalidomide ☐
Thalidomide ☐
Other ☐
6-Mercaptopurine ☐
Azathioprine ☐
Methotrexate ☐
Tofacitinib ☐
Abatacept ☐
Anakinra ☐
Tildrakizumab ☐
Alemtuzumab ☐

Other medication, specify _____

Date of Initiation _____

For annotations, refer to CRF page 107, 108

Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ☐ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ■ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ☐ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ☐ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ☐ 6-Mercaptopurine ☐ Azathioprine ☐ Methotrexate ☐ Tofacitinib ☐

For annotations, refer to CRF page 107, 108

	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ■
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐

For annotations, refer to CRF page 107, 108

	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ■
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐

For annotations, refer to CRF page 107, 108

	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐

For annotations, refer to CRF page 107, 108

	Ustekinumab ☐
	Rituximab ■
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐

For annotations, refer to CRF page 107, 108

	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ■
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	_____
Date of Initiation	_____
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	_____

For annotations, refer to CRF page 107, 108

Please select concomitant therapy log line, start date and therapy

Medication Class	PROHIBITED MEDICATION
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ■
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐

For annotations, refer to CRF page 107, 108

	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ☐ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ☐ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ■ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ☐ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ☐ 6-Mercaptopurine ☐

For annotations, refer to CRF page 107, 108

	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ■
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐

For annotations, refer to CRF page 107, 108

	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐

For annotations, refer to CRF page 107, 108

	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ■
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐

For annotations, refer to CRF page 107, 108

	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ■
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐

For annotations, refer to CRF page 107, 108

Cyclosporine ☐
Mycophenolate Mofetil ☐
Tacrolimus ☐
Sirolimus ☐
Infliximab ☐
Etanercept ☐
Certolizumab pegol ☐
Adalimumab ☐
Natalizumab ☐
Ustekinumab ☐
Rituximab ☐
Vedolizumab ☐
Upadacitinib ☐
Filgotinib ☐
Ozanimod ☐
Etrolizumab ☐
Brazikumab ☐
Briakinumab ■
Mirikizumab ☐
Risankizumab ☐
Rhalidomide ☐
Thalidomide ☐
Other ☐
6-Mercaptopurine ☐
Azathioprine ☐
Methotrexate ☐
Tofacitinib ☐
Abatacept ☐
Anakinra ☐
Tildrakizumab ☐
Alemtuzumab ☐

Other medication, specify _____

Date of Initiation _____

For annotations, refer to CRF page 107, 108

Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ☐ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ☐ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ☐ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ■ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ☐ 6-Mercaptopurine ☐ Azathioprine ☐ Methotrexate ☐ Tofacitinib ☐

For annotations, refer to CRF page 107, 108

	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ■
	Rhalidomide ☐

For annotations, refer to CRF page 107, 108

	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐

For annotations, refer to CRF page 107, 108

	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ■
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐

For annotations, refer to CRF page 107, 108

	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ■
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐

For annotations, refer to CRF page 107, 108

	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ■
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	_____
Date of Initiation	_____
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	_____

For annotations, refer to CRF page 107, 108

Please select concomitant therapy log line, start date and therapy

Medication Class	PROHIBITED MEDICATION
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ■
	Anakinra ☐
	Tildrakizumab ☐

For annotations, refer to CRF page 107, 108

	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ☐ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ☐ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ☐ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ☐ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ☐ 6-Mercaptopurine ☐

For annotations, refer to CRF page 107, 108

	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ■
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐

For annotations, refer to CRF page 107, 108

	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ■
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐

For annotations, refer to CRF page 107, 108

	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ☐
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ■
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐

For annotations, refer to CRF page 107, 108

	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ■
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐

For annotations, refer to CRF page 107, 108

- [ ] Cyclosporine
- [ ] Mycophenolate Mofetil
- [ ] Tacrolimus
- [ ] Sirolimus
- [ ] Infliximab
- [ ] Etanercept
- [ ] Certolizumab pegol
- [ ] Adalimumab
- [ ] Natalizumab
- [ ] Ustekinumab
- [ ] Rituximab
- [ ] Vedolizumab
- [ ] Upadacitinib
- [ ] Filgotinib
- [ ] Ozanimod
- [ ] Etrolizumab
- [ ] Brazikumab
- [ ] Briakinumab
- [ ] Mirikizumab
- [ ] Risankizumab
- [ ] Rhalidomide
- [ ] Thalidomide
- [x] Other
- [ ] 6-Mercaptopurine
- [ ] Azathioprine
- [ ] Methotrexate
- [ ] Tofacitinib
- [ ] Abatacept
- [ ] Anakinra
- [ ] Tildrakizumab
- [ ] Alemtuzumab

Other medication, specify _____

Date of Initiation _____

For annotations, refer to CRF page 107, 108

Reason for Initiation	Worsening of Ulcerative Colitis ☐ Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐ Cyclosporine ☐ Mycophenolate Mofetil ☐ Tacrolimus ☐ Sirolimus ☐ Infliximab ☐ Etanercept ☐ Certolizumab pegol ☐ Adalimumab ☐ Natalizumab ☐ Ustekinumab ☐ Rituximab ☐ Vedolizumab ☐ Upadacitinib ☐ Filgotinib ☐ Ozanimod ☐ Etrolizumab ☐ Brazikumab ☐ Briakinumab ☐ Mirikizumab ☐ Risankizumab ☐ Rhalidomide ☐ Thalidomide ☐ Other ■ 6-Mercaptopurine ☐ Azathioprine ☐ Methotrexate ☐ Tofacitinib ☐

For annotations, refer to CRF page 107, 108

	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐
	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐

For annotations, refer to CRF page 107, 108

	Thalidomide ☐
	Other ■
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	
Medication Class	PROHIBITED MEDICATION ■
Medication	6-TG ☐
	Cyclosporine ☐
	Mycophenolate Mofetil ☐
	Tacrolimus ☐
	Sirolimus ☐
	Infliximab ☐
	Etanercept ☐
	Certolizumab pegol ☐
	Adalimumab ☐
	Natalizumab ☐
	Ustekinumab ☐
	Rituximab ☐
	Vedolizumab ☐
	Upadacitinib ☐
	Filgotinib ☐
	Ozanimod ☐

For annotations, refer to CRF page 107, 108

	Etrolizumab ☐
	Brazikumab ☐
	Briakinumab ☐
	Mirikizumab ☐
	Risankizumab ☐
	Rhalidomide ☐
	Thalidomide ☐
	Other ■
	6-Mercaptopurine ☐
	Azathioprine ☐
	Methotrexate ☐
	Tofacitinib ☐
	Abatacept ☐
	Anakinra ☐
	Tildrakizumab ☐
	Alemtuzumab ☐
Other medication, specify	
Date of Initiation	
Reason for Initiation	Worsening of Ulcerative Colitis ☐
	Other ☐
If Other, specify	
Please select concomitant therapy log line, start date and therapy	

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Baseline Medication Review - 5-ASA *BMR*

Generated On: 13 Jul 2020 10:14:40 *FACAT = BASELINE MEDICATION REVIEW - 5-ASA*

Please indicate if the subject is taking 5-ASA at the BASELINE VISIT:	*[NOT SUBMITTED]*	Yes ☐ No ☐	*BMRYN; Coded; Char*
If Yes, Please indicate if the following medications are being taken at the BASELINE VISIT.			
Now	*[NOT SUBMITTED]*		*BMRNOW; Time; Date*
Medication	*FAOBJ*	Mesalamine ☐ Balsalazide ☐ Sulfasalazine ☐ Olsalazine ☐ Other aminosalicylates ☐	*BMRMED; Coded; Char*
FATEST = Occurence *FAORRES when FATESTCD = OCCUR*	*[NOT SUBMITTED]*		
Other, specify	*FAOBJ*		*BMRSP; Entered; Char*
Please select concomitant therapy log line, start date and therapy	*Linked to related CM record via RE*		*CMDSL; Entered; Char*

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Baseline Medication Review - Corticosteroid *BMR_COR* *For annotations, refer to CRF page 107, 108*
Generated On: 13 Jul 2020 10:14:40 *FACAT = BASELINE MEDICATION REVIEW - CORTICOSTEROID*

Please indicate if the subject is taking CORTICOSTEROID at the BASELINE VISIT:	Yes ☐ No ☐	*BMRYN; Coded; Char*
If Yes, Please indicate if the following medications are being taken at the BASELINE VISIT.		
NOW		*BMR_CORNOW; Time;*
Medication	Budesonide ☐ Beclomethasone Dipropionate ☐ Hydrocortisone ☐ Methylprednisolone ☐ Prednisolone ☐ Prednisone ☐ Other corticosteroid ☐	*BMRMED; Coded; Char*
Other, specify		*BMRSP; Entered; Char*
Please select concomitant therapy log line, start date and therapy		*CMDSL; Entered; Char*

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Baseline Medication Review - Opioids *FACAT = BASELINE MEDICATION REVIEW - OPIOIDS* *BMR_OPI*
Generated On: 13 Jul 2020 10:14:40 *FAOBJ = OPIOID*

	Please indicate if the subject is taking any OPIOIDS at the BASELINE VISIT: *[NOT SUBMITTED]*	Yes ☐ No ☐	*BMROYN; Coded; Char*
	If Yes:		
FATEST	Are the OPIOIDs being taken for Ulcerative Colitis Disease? *FAORRES when FATESTCD = OCCUR*	Yes ☐ No ☐	*OMHYN; Coded; Char*
FATEST	If yes, indicate the **primary** Ulcerative Colitis Disease related reason. *FAORRES when FATESTCD = REASCDRP*	Pain ☐ Diarrhea ☐	*BMRO_REAS; Coded; C*
	Please select concomitant therapy log line, start date and therapy *Linked to related CM record via REL*		*CMDSL; Entered; Char*
	NOW _ *[NOT SUBMITTED]* _		*BMR_OPINOW; Time; D*

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Medication Review - 5-ASA MR
Generated On: 13 Jul 2020 10:14:40

For annotations, refer to CRF page 107, 108

FACAT = MEDICATION REVIEW - 5-ASA

Since the last scheduled visit, has the subject initiated 5-ASA or have there been any changes to the subject's prescribed dose?	Yes ☐ No ☐	MRYN; Coded; Char
NOW		MRNOW; Time; Date
Medication	Mesalamine ☐ Balsalazide ☐ Sulfasalazine ☐ Olsalazine ☐ Other aminosalicylates ☐	MRMED; Coded; Char
Other, specify		MRMEDSP; Entered; Ch
Date of Initiation or Change		MRDAT; Time; Date
Reason for Initiation or Change	Worsening of Ulcerative Colitis ☐ Other ☐	MRREAS; Coded; Char
If Other, specify		MRREASP; Entered; Ch
Please select concomitant therapy log line, start date and therapy		CMDSL; Entered; Char

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Medication Review - Corticosteroid MR_COR *For annotations, refer to CRF page 150*
Generated On: 13 Jul 2020 10:14:40 *FACAT = MEDICATION REVIEW - CORTICOSTEROID*

Since the last scheduled visit, has the subject initiated CORTICOSTEROID or have there been any changes to the subject's prescribed dose?	Yes ☐ No ☐	*MRYN; Coded; Char*
NOW	_____	*MR_CORNOW; Time; Da*
Medication	Budesonide ☐ Beclomethasone Dipropionate ☐ Hydrocortisone ☐ Methylprednisolone ☐ Prednisolone ☐ Prednisone ☐ Other corticosteroid ☐	*MRMED; Coded; Char*
Other, specify	_____	*MRMEDSP; Entered; Ch*
Date of Initiation or Change	_____	*MRDAT; Time; Date*
Reason for initiation or change	_____	*MRCORREA; Entered; C*
If Other, specify	_____	*MRREASP; Entered; Ch*
Please select concomitant therapy log line, start date and therapy	_____	*CMDSL; Entered; Char*

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Medication Review - Opioids MR_OPI MEDICATION REVIEW - OPIOIDS

Generated On: 13 Jul 2020 10:14:40

Since the last scheduled visit, has the subject initiated a new OPIOID or changed their previously prescribed OPIOID regimen? **Do not record opioids given for the Endoscopy procedure.** **Do not include medication changes initiated at this visit. Do not include any medications that were stopped.**	[NOT SUBMITTED]	Yes ☐ No ☐	MROYN; Coded; Char
FATEST Was the reason for initiation or change due to Ulcerative Colitis Disease? FAOBJ = REASON FOR INITIATION OR CHANGE DUE TO UCO	FAORRES when FATESTCD = OCCUR	Yes ☐ No ☐	OMHYN; Coded; Char
FAOBJ = OPIOID If yes, indicate the **primary** Ulcerative Colitis Disease related reason.	FATEST FAORRES when FATESTCD = REASCDRP	Pain ☐ Diarrhea ☐	BMRO_REAS; Coded; C
Date of Initiation or Change	FATEST FAORRES when FATESTCD = INCHDTC		MRODAT; Time; Date
Please select concomitant therapy log line, start date and therapy	Linked to related CM record via RELREC		CMDSL; Entered; Char
NOW	___ [NOT SUBMITTED] _____		MR_OPINOW; Time; Da

FA = Findings About Events or Interventions

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Un FACAT, CMCAT = BIOLOGIC FAILURE

Project Name: CNTO1959UCO2002

Form: Biologic Failure Status - Vedolizumab BIO_FS_VED

Generated On: 13 Jul 2020 10:14:40

Link between related FA and CM records via RELREC

Vedolizumab (Check all that apply) FAOBJ, CMTRT		
Never Received	FAORRES when FATESTCD = NEVR	INF1; Checked; Num
Primary Non-Responder	FAORRES when FATESTCD = PRIMAR	INF2; Checked; Num
Secondary Non-Responder	FAORRES when FATESTCD = SECOND	INF3; Checked; Num
Intolerant	FAORRES when FATESTCD = INTOL	INF4; Checked; Num
Received, but discontinued for reasons other than non-response or intolerance	FAORRES when FATESTCD = OTHREAS	INF5; Checked; Num

FATEST

If PRIMARY NON-RESPONDER complete the following:

Indicate Date	CMSTDTC		INDDT; Time; Date
Indicate Dose	CMDOSU	CMDOSE Vedolizumab - 300 mg IV ☐	VINDDOSE; Coded; Num
	[NOT SUBMITTED]	Vedolizumab - Other mg IV ☐	
		Vedolizumab - Other mg SC ☐	
Other, Specify	CMDOSE. CMDOSTXT when CMDOSE = OTHER		OTHSPE; Entered; Char

Signs or symptoms that are evidence of persistence of Ulcerative Colitis (select all that apply)

FASCAT = PRIMARY NON-RESPONSE

FAOBJ

Lack of improvement or worsening in stool frequency	FAORRES when FATESTCD = OCCUR	EVIUC1; Checked; Num
Lack of improvement or worsening in rectal bleeding		EVIUC2; Checked; Num
Lack of improvement or worsening in daily abdominal pain		EVIUC3; Checked; Num
Lack of improvement or worsening in urgency		EVIUC4; Checked; Num
Lack of improvement or worsening in the endoscopic appearance of the colonic mucosa		EVIUC5; Checked; Num

If SECONDARY NON-RESPONDER, complete the following:

If Secondary Non-responder, how long did the subject receive maintenance therapy?	4 - 8 weeks ☐	MAITHE; Coded; Num
	9 - 12 weeks ☐	
CMMTDUR in SUPPCM in when IDVAR = CMSEQ	13 weeks - 6 months ☐	
	More than 6 months ☐	

SECONDARY NON-RESPONDER FINAL DOSE

Indicate Date	CMSTDTC		FINDDT; Time; Date
Indicate dose	CMDOSU	CMDOSE Vedolizumab - 300 mg IV ☐	VFINDDOSE; Coded; Nu
	[NOT SUBMITTED]	Vedolizumab - Other mg IV ☐	
		Vedolizumab - Other mg SC ☐	
Other, Specify	CMDOSE, CMDOSTXT when CMDOSE = OTHER		FOTHSPE; Entered; Cha
Indicate interval	CMDOSFRQ	Vedolizumab - Q8w ☐	VFINDINT; Coded; Num
		Vedolizumab - Q4w ☐	
		Vedolizumab - Other ☐	

FA = Findings About Events or Interventions

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Biologic Failure Status - Vedolizumab *BIO_FS_VED*

Generated On: 13 Jul 2020 10:14:40

Other, Specify	*CMDSFRQO in SUPPCM when IDVAR = CMSEQ*	*FOTHSPE2; Entered; Ch*

FASCAT = SECONDARY NON-RESPONSE

Signs or symptoms related to recurrence of Ulcerative Colitis (select all that apply):

FAOBJ

Worsening in stool frequency	*FAORRES when FATESTCD = OCCUR*	*RECUC1; Checked; Nur*
Worsening in rectal bleeding		*RECUC2; Checked; Nur*
Worsening in daily abdominal pain		*RECUC3; Checked; Nur*
Worsening in urgency		*RECUC4; Checked; Nur*
Worsening in the endoscopic appearance of the colonic mucosa		*RECUC5; Checked; Nur*

If INTOLERANT, complete the following:

Final Dose	*CMDOSU*	*CMDOSE*	Vedolizumab - 300 mg IV ☐	*VINTDOSE; Coded; Nun*
		[NOT SUBMITTED]	Vedolizumab - Other mg IV ☐	
			Vedolizumab - Other mg SC ☐	
Other, Specify		*CMDOSE, CMDOSTXT when CMDOSE = OTHER*		*VOTHSPE; Entered; Cha*

FASCAT = INTOLERANCE

Did a significant acute infusion reaction that occurred less than or equal to 24 hours happen after infusion of administration?	Yes ☐ No ☐	*REAOCC; Coded; Char*

FAOBJ = ACUTE INFUSION REACTION *FAORRES when FATESTCD = OCCUR*

If Yes, reaction manifested by (select all that apply):

FAOBJ

Fever greater than 100F (37.8C)	*FAORRES when FATESTCD = OCCUR*	*REAOCC1; Checked; Nu*
Chills or rigors		*REAOCC2; Checked; Nu*
Itching		*REAOCC3; Checked; Nu*
Rash	*FAGRPID = ACUTE ADMINISTRATION REACTION*	*REAOCC4; Checked; Nu*
Flushing		*REAOCC5; Checked; Nu*
Urticaria or angioedema		*REAOCC6; Checked; Nu*
Breathing difficulties		*REAOCC7; Checked; Nu*
Clinical hypotension		*REAOCC8; Checked; Nu*

Did a significant delayed infusion reaction that occurred greater than 24 hours and less than 15 days happen after the infusion and was considered related to the infusion of vedolizumab?	*FAORRES when FATESTCD = OCCUR* Yes ☐ No ☐	*DREAOCC; Coded; Cha*

FAOBJ = DELAYED INFUSION REACTION

If Yes, reaction manifested by (select all that apply):

FAOBJ

Myalgias	*FAORRES when FATESTCD = OCCUR*	*DREAOCC1; Checked; I*
Arthralgias	*FAGRPID = DELAYED ADMINISTRATION REACTION*	*DREAOCC2; Checked; I*
Fever greater than 100F (37.8C)		*DREAOCC3; Checked; I*
Malaise		*DREAOCC4; Checked; I*
Rash		*DREAOCC5; Checked; I*

Did a significant injection site reaction occur within 24 hours of a SC injection of Vedolizumab and was considered related to the injection?	Yes ☐ No ☐	*SREAOCC; Coded; Cha*

FAOBJ = INJECTION SITE REACTION *FAORRES when FATESTCD = OCCUR*

FA = Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Biologic Failure Status - Vedolizumab BIO_FS_VED

Generated On: 13 Jul 2020 10:14:40

	If Yes, reaction manifested by (select all that apply):			
FAOBJ	Significant bruising	FAGRPID = INJECTION SITE REACTION	FAORRES when FATESTCD = OCCUR	SREAOCC1; Checked; I
	Erythema			SREAOCC2; Checked; I
	Hemorrhage			SREAOCC3; Checked; I
	Irritation			SREAOCC4; Checked; I
	Pain			SREAOCC5; Checked; I
	Other			SREAOCC6; Checked; I
	If Other, specify		FAPTUCO in SUPPFA when IDVAR = FASEQ	SREASPE; Entered; Ch

FA = Findings About Events or Interventions

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Tofacitinib Treatment History TO_TH_001

Generated On: 13 Jul 2020 10:14:40

FACAT, CMCAT = TOFACITINIB TREATMENT

FAOBJ **Tofacitinib** (Check all that apply) FAOBJ, CMTRT FAORRES when FATESTCD = OCCUR

Nev FAGRPID = DELAYED ADMINISTRATION REA FAORRES when FATESTCD = NEVRRCD TONR; Checked; Num

Link between related FA and CM records via RELREC

Primary Non-Responder FATEST FAORRES when FATESTCD = PRIMARY TOPNR; Checked; Num

Secondary Non-Responder FAORRES when FATESTCD = SECONDY TOSNR; Checked; Num

Intolerant FAORRES when FATESTCD = INTOLER TOIN; Checked; Num

Received, but discontinued for reasons other than non-response or intolerance FAORRES when FATESTCD = OTHREAS TODIS; Checked; Num

If PRIMARY NON-RESPONDER complete the following

Indicate Date CMSTDTC INDDAT; Time; Date

Indicate Dose Regimen CMDOSE Tofacitinib 10mg PO BID ☐ INDDOSE; Coded; Char

[NOT SUBMITTED] Tofacitinib Other mg ☐ CMDOSU

Other, Specify CMDOSE, CMDOSTXT when CMDOSE = OTHE OTHSPF; Entered; Char

FASCAT = PRIMARY NON-RESPONSE

Signs or symptoms that are evidence of persistence of Ulcerative Colitis (select all that apply)

FAOBJ

Lack of improvement or worsening in stool frequency FAORRES when FATESTCD = OCCUR TOPNR1; Checked; Num

Lack of improvement or worsening in rectal bleeding TOPNR2; Checked; Num

Lack of improvement or worsening in daily abdominal pain TOPNR3; Checked; Num

Lack of improvement or worsening in urgency TOPNR4; Checked; Num

Lack of improvement or worsening in the endoscopic appearance of the colonic mucosa TOPNR5; Checked; Num

If SECONDARY NON-RESPONDER, complete the following:

If Secondary Non-responder, how long did the subject receive maintenance therapy? CMMTDUR in SUPPCM in when IDVAR = CMSEQ

4 - 8 weeks ☐ TOSNRDUR; Coded; Ch

9 - 12 weeks ☐

13 weeks - 6 months ☐

More than 6 months ☐

SECONDARY NON-RESPONDER FINAL DOSE

Indicate Date: CMSTDTC TOSNRDAT; Time; Date

Indicate dose: CMDOSE Tofacitinib 5mg PO ☐ TOSNRDOS; Coded; Ch

Tofacitinib 10mg PO ☐ CMDOSU

[NOT SUBMITTED] Tofacitinib Other mg ☐

Other, Specify CMDOSE, CMDOSTXT when CMDOSE = OTH OTHSPF4; Entered; Cha

Indicate Frequency: CMDOSFRQ Tofacitinib - BID ☐ TOSNRFRE; Coded; Ch

Tofacitinib - Other ☐

Other, Specify CMDSFROO in SUPPCM when IDVAR = CMSEQ OTHSPF3; Entered; Cha

Signs or symptoms related to recurrence of Ulcerative Colitis (select all that apply)**:**

Worsening in stool frequency FAORRES when FATESTCD = OCCU TOUC1; Checked; Num

FA = Findings About Events or Interventions

CM = Concomitant Medications

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Tofacitinib Treatment History *TO_TH_001*
Generated On: 13 Jul 2020 10:14:40

FASCAT = SECONDARY NON-RESPONSE	Worsening in rectal bleeding	*FAORRES when FATESTCD = OCC*	*TOUC2; Checked; Num*
	Worsening in daily abdominal pain		*TOUC3; Checked; Num*
	Worsening in urgency		*TOUC4; Checked; Num*
FAOBJ	Worsening in the endoscopic appearance of the colonic mucosa		*TOUC5; Checked; Num*

If INTOLERANT, complete the following:

Final Dose	*CMDOSE*	Tofacitinib 5mg PO ☐ Tofacitinib 10mg PO ☐ Tofacitinib Other mg ☐	*TOFDOSE; Coded; Char* *CMDOSU*
	[NOT SUBMITTED]		
Other, Specify	*CMDOSE, CMDOSTXT when CMDOSE = OTHER*		*OTHSPF5; Entered; Cha*
Indicate Frequency:	*CMDOSFRQ*	Tofacitinib - BID ☐ Tofacitinib - Other ☐	*TOINFRE; Coded; Char*
FASCAT = INTOLERANCE Other, Specify	*CMDSFRQO in SUPPCM when IDVAR = CMSEQ*		*OTHSPF6; Entered; Cha*
If INTOLERANT, specify the reason:	*FAORRES when FATESTCD = REAS*		*TOINREA; Entered; Cha*

MO = Morphology

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Tuberculosis Testing at Screening TBINFO_1
Generated On: 13 Jul 2020 10:14:40 MOCAT = SCREENING FOR TUBERCULOSIS

Method of Assessment	MOMETHOD	Chest X-Ray ☐	TBMETHOD; Coded; Ch
		Chest CT Scan ☐	
Date of Assessment	MODTC		TBDTC; Time; Date
MOTEST Overall Interpretation	MOORRES when MOTESTCD = INTP	Normal ☐	TBORRES; Coded; Cha
		Abnormal ☐	
		Not Evaluable ☐	
If Abnormal, was there suspicion of tuberculosis?	MOTBSUSP in SUPPMO when IDVAR = MOSEQ	Yes ☐	TBSUSPYN; Coded; Ch
		No ☐	
If Abnormal, please specify	MOCLSIG in SUPPMO when IDVAR = MOSEQ		TBABN; Entered; Char

FA = Findings About Events or Interventions

SR = Skin Response

MO = Morphology

LB = Laboratory Test Results

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002 FACAT = TUBERCULOSIS TESTING FAOBJ = TUBERCULOSIS

Form: Tuberculosis Testing and Results SRCAT = TUBERCULOSIS TESTING TT_TR_001

Generated On: 13 Jul 2020 10:14:40 MOCAT = TUBERCULOSIS TESTING

Link between related FA and SR, MO, LB records via RELREC

Was the subject re-tested for possible TB (at any point after receiving study drug)? FATEST FAORRES when FATESTCD = SUBJRTST FAEVINTX = AFTER RECEIVING STUDY DRUG		Yes / No	SUBJRTST; Coded; Char
Date of Re-Testing Assessment	FADTC		RETDAT; Time; Date
Tuberculin PPD-S	SROBJ		PPDLBL; Checked; Num
Date PPD Administered	SRRFTDTC		SRRFTDAT_ADM; Time
Planned time point when PPD should be read	SRTPT	Fixed Unit: Hours; 48 - 72	SRTPT; Entered; Char
Date PPD Read	SRDTC		SRDAT_RD; Time; Date
SRTEST Induration Diameter	SRORRES when SRTESTCD = INDURDIA		INDURDIA; Entered; Nu
Units	SRORRESU	mm	INDURDIAU; Coded; Ch
SRTEST Overall Interpretation	SRORRES when SRTESTCD = INTP	Negative / Positive	INTP2; Coded; Char
Chest X-ray or Chest CT scan Testing and Results	[NOT SUBMITTED]		XLBL; Checked; Num
Method of Assessment	MOMETHOD	Chest X-Ray / Chest CT Scan	TBMETHOD; Coded; Ch
Date of Assessment	MODTC		MODAT; Time; Date
MOTEST Overall Interpretation	MOORRES when MOTESTCD = INTP	Normal / Abnormal / Not Evaluable	INTP3; Coded; Char
If Abnormal, Was there suspicion of tuberculosis?	MOTBSUSP in SUPPMO when IDVAR = MOSEQ	Yes / No	SUPPMO; Coded; Char
QuantiFERON-TB	[NOT SUBMITTED]		FERONLBL; Checked; N
If QuantiFERON-TB test was done using a local lab, provide the result: QuantiFERON-TB	LBTEST = M. tuberculosis IFN Gamma Response	Negative / Positive	INTP4; Coded; Char LBTESTCD = MYTBGIR
What was the lab specimen collection date?	LBDTC		LBDAT; Time; Date

LBCAT = IMMUNOLOGY

LBMETHOD = QUANTIFERON TB

PC = Pharmacokinetics Concentrations

IS = Immunogenicity Specimen Assessments

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

PCCAT = ANALYTE

ISCAT = ANALYTE

Project Name: CNTO1959UCO2002

Form: PK Serum Concentration PC_GL_900_S PCTESTCD = Pharmacokinetics Tests when PCTESTCD = PCALL

Generated On: 13 Jul 2020 ISTEST = Immunogenicity Specimen Assessments when ISTESTCD = ISALL

Was the sample collected? [NCOMPLT; PCYN; Coded; Char

PCSTAT = NOT DONE, PCREASND = SAMPLE NOT COLLECTED when PCTESTCD = PCALL

ISSTAT = NOT DONE, ISREASND = SAMPLE NOT COLLECTED when ISTESTCD = ISALL

PCSTAT = NOT DONE, PCREASND = NOT APPLICABLE when PCTESTCD = PCALL

ISSTAT = NOT DONE, ISREASND = NOT APPLICABLE when ISTESTCD = ISALL

Yes ☐

No ☐

Not Applicable ☐

Reason Not Collected [PCREASNC; Entered; C

PCREASNC in SUPPPC when IDVAR = PCSEQ

ISREASNC in SUPPIS when IDVAR = ISSEQ

PC = Pharmacokinetics Concentrations

IS = Immunogenicity Specimen Assessments

PCCAT = ANALYTE

ISCAT = ANALYTE

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

PCTEST = Pharmacokinetics Tests when PCTESTCD = PCALL

Form: PK Serum Concentration (IV)

PC_GL_900_S1 = Immunogenicity Specimen Assessments when ISTESTCD = ISALL

Generated On: 13 Jul 2020 10:14:40

Planned Time Point — PCTPT ISTPT — Pre-Dose ☐ / Post-infusion ☐ — PCTPT; Coded; Char

Was the sample collected? — Yes ☐ / No ☐ / Not Applicable ☐ — PCYN; Coded; Char

PCSTAT = NOT DONE, PCREASND = SAMPLE NOT COLLECTED when PCTESTCD = PCALL

ISSTAT = NOT DONE, ISREASND = SAMPLE NOT COLLECTED when ISTESTCD = ISALL

PCSTAT = NOT DONE, PCREASND = NOT APPLICABLE when PCTESTCD = PCALL

ISSTAT = NOT DONE, ISREASND = NOT APPLICABLE when ISTESTCD = ISALL

Reason Not Collected — PCREASNC in SUPPPC when IDVAR = PCSEQ — ISREASNC in SUPPIS when IDVAR = ISSEQ — PCREASNC; Entered; C

Planned Time Point — ose ☐ / Post-infusion ☐

Was the sample collected? — Yes ☐ / No ☐ / Not Applicable ☐

Reason Not Collected

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Collection of biopsies for RNA, histology, and single cell isolation samples PC_GL_900_R

Generated On: 13 Jul 2020 10:14:40 XZCAT = BIOPSY

Was RNA sample collected? XZTEST = Sample Obtained XZTESTCD = SAMPOBT [NOT SUBMITTED] Yes ☐ No ☐ if No, XZSTAT = NOT DONE PCYN_RNA; Coded; Ch

XZSCAT = RNA

Reason Not Collected XZREASND PCREAS_RNA; Entered

Was histology sample collected? XZTEST = Sample Obtained XZTESTCD = SAMPOBT [NOT SUBMITTED] Yes ☐ No ☐ if No, XZSTAT = NOT DONE PCYN_HIS; Coded; Cha

XZSCAT = HISTOLOGY

Reason Not Collected XZREASND PCREAS_HIS; Entered;

Was Single Cell Isolation Sample collected? XZTEST = Sample Obtained XZTESTCD = SAMPOBT [NOT SUBMITTED] Yes ☐ No ☐ if No, XZSTAT = NOT DONE PCYN_SCI; Coded; Cha

XZSCAT = SINGLE CELL ISOLATION

Reason Not Collected XZREASND PCREAS_SCI; Entered;

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: PD Serum Biomarkers *PC_GL_900_SB* *XZCAT = BIOMARKER*
Generated On: 13 Jul 2020 10:14:40 *XZSPEC = SERUM*

Was the sample collected?	*XZTEST = Sample Obtained* *XZTESTCD = SAMPOBT*	*[NOT SUBMITTED]* *if No, XZSTAT = NOT DONE*	Yes ☐ No ☐	*PCYN; Coded; Char*
Reason Not Collected		*XZREASND*		*PCREASNC; Entered; C*

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Whole blood sample collection for RNA analysis *PC_GL_900_B*
Generated On: 13 Jul 2020 10:14:40

XZCAT = BIOMARKER
XZSPEC = BLOOD

Was the sample collected?	*XZTEST = Sample Obtained* *XZTESTCD = SAMPOBT*	*[NOT SUBMITTED]* *if No, XZSTAT = NOT DONE*	Yes ☐ No ☐	*PCYN; Coded; Char*
Reason Not Collected		*XZREASND*		*PCREASNC; Entered; C*

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Fecal Biomarker PC_GL_900_FB XZCAT = BIOMARKER XZSPEC = STOOL
Generated On: 13 Jul 2020 10:14:40

Was the sample collected?	XZTEST = Sample Obtained XZTESTCD = SAMPOBT	[NOT SUBMITTED]	Yes ☐ No ☐	PCYN; Coded; Char
Reason Not Collected		XZREASND		PCREASNC; Entered; C

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002 XZCAT = BIOMARKER
Form: Genetic (DNA) evaluations PC_GL_900_D DNA
Generated On: 13 Jul 2020 10:14:40

Was the sample collected?	[NOT SUBMITTED]	Yes ☐	PCYN; Coded; Char
XZTEST = Sample Obtained XZTESTCD = SAMPOBT	if No or Not Applicable, XZSTAT = NOT DONE	No ☐	
		Not Applicable ☐	
Reason Not Collected	XZREASND		PCREASNC; Entered; C

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Randomization ***DS_GL_902***
Generated On: 13 Jul 2020 10:14:40

Category	***DSCAT*** PROTOCOL MILESTONE	***DSCAT; Coded; Char***
What was the date of randomization?	***DSTERM, DSDECOD = RANDOMIZED*** ***DSSTDTC*** ***[NOT SUBMITTED]***	***DSSTDAT; Time; Date***

DA = Drug Accountability

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Drug Preparation for Intravenous GUS/PBO *DA_GL_901_GUS*
Generated On: 13 Jul 2020 10:14:40

Treatment	*DACAT* Guselkumab/Placebo	*DASCAT_TRT; Coded;*
Was study treatment prepared for this visit?	*[NOT SUBMITTED]* Yes *DASTAT=NOT DONE when DATESTCD = TVPRP* No	*DASAYN; Coded; Char*
Date Prepared	*DADTC*	*DADAT; Time; Date*
Prepared Start Time	*DASTTIM in SUPPDA when IDVAR = DASEQ*	*DASTTIM; Time; Date*
DATEST Volume of study drug added to IV Container	Fixed Unit: mL *DAORRES when DATESTCD = TVTRA* 2 *[NOT SUBMITTED]* Other	*TVTRA_GUS; Coded; C*
Other, specify	Fixed Unit: mL *DAORRES when DATESTCD = TVTRA*	*TVTRA_OTH; Entered;*
Units	*DAORRESU when DATESTCD = TVTRA* mL	*TVTRAU; Coded; Char*
DATEST Total Volume Prepared	Fixed Unit: mL *DAORRES when DATESTCD = TVPRP*	*TVPRP; Entered; Num*
Units	*DAORRESU when DATESTCD = TVPRP* mL	*TVPRPU; Coded; Char*

Specify all Unique Treatment Label Identifiers Used for this Preparation

Drug Container Number	*DAGRPID*	*DAGRPID; Entered; Cha*
Now	*[NOT SUBMITTED]*	*DANOW; Time; Date*

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Subcutaneous Injection - Baseline EX_GL_900A
Generated On: 13 Jul 2020 10:14:40 ECTRT = MASKED

Treatment	ECCAT	Golimumab/Placebo	EXCAT; Coded; Char
Was the syringe administered?	ECOCCUR	Yes / No	EXYN_DOS; Coded; Ch
If No, what was the reason the syringe was not administered? ECREASOC in SUPPEC when IDVAR = ECSEQ		Adverse Event / Dose Skipped / Other	EXREASND; Coded; Ch
If Adverse Event, choose the corresponding AE log line, start date, and term	Linked to related AE record via RELREC when RELID starts with "SYR"		AEDSL2; Entered; Char
If Other, Specify	ECREASOO in SUPPEC when IDVAR = ECSEQ		EXRSNDO; Entered; Ch
What is the treatment label identifier?	ECREFID		EXREFID; Entered; Cha
Treatment kit identifier	[NOT SUBMITTED]	Syringe 1 / Syringe 2	EXKITNO; Coded; Char
Dose	ECDOSE	1 / 2	EXDOSE; Coded; Num
Unit	ECDOSU	mL	EXDOSEU; Coded; Cha
Was the expected volume administered?	ECTOTAMT in SUPPEC when IDVAR = ECSEQ	Yes / No	EXADMYN; Coded; Cha
If No, what was the reason the expected volume was not administered?	ECADJ	Adverse Event / Device Malfunction / Other	EXADMREA; Coded; Ch
If Adverse Event, choose the corresponding AE log line, start date, and term	Linked to related AE record via RELREC when RELID starts with "VOL"		AEDSL; Entered; Char
If Other, specify	ECADJOTH in SUPPEC when IDVAR = ECSEQ		EXADMOSP; Entered; C
Formulation	ECDOSFRM	Aerosol / Capsule / Delayed Release Capsule / Extended Release Capsule / Cream / For Solution / For Suspension / Gel / Implant	EXDOSFRM; Coded; Ch

	Inhalant ☐	
	Injectable ■	
	Liquid ☐	
	Lotion ☐	
	Mouthwash ☐	
	Ointment ☐	
	Patch ☐	
	Powder ☐	
	Shampoo ☐	
	Soap ☐	
	Solution ☐	
	Spray ☐	
	Suppository ☐	
	Suspension ☐	
	Tablet ☐	
	Delayed Release Tablet ☐	
	Extended Release Tablet ☐	
	Tape ☐	
	Minitablets in Capsule ☐	
	Unassigned ☐	
	Not Applicable ☐	
Route	Auricular ☐	*EXROUTE; Coded; Cha*
	Intramuscular ☐	
	Intravenous ☐	
ECROUTE	Nasal ☐	
	Ophthalmic ☐	
	Oral ☐	
	Parenteral ☐	
	Rectal ☐	
	Inhalation ☐	
	Subcutaneous ■	
	Sublingual ☐	
	Topical ☐	

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Subcutaneous Injection - Baseline ***EX_GL_900A***
Generated On: 13 Jul 2020 10:14:40

	Other ☐	
	Unknown ☐	
	Not Applicable ☐	
What was the anatomical location of the administration?	Upper Right Arm ☐	*EXLOC; Coded; Char*
	Upper Left Arm ☐	
	Right Thigh ☐	
ECDIR ECLAT ECLOC	Left Thigh ☐	
	Lower Right Abdominal ☐	
	Lower Left Abdominal ☐	
	Other ☐	
Start Date	*ECSTDTC*	*EXSTDAT; Time; Date*
Actual Start Time		*EXSTTIM; Time; Date*
Treatment	Golimumab/Placebo ■	
Was the syringe administered?	Yes ☐	
	No ☐	
If No, what was the reason the syringe was not administered?	Adverse Event ☐	
	Dose Skipped ☐	
	Other ☐	
If Adverse Event, choose the corresponding AE log line, start date, and term		
If Other, Specify		
What is the treatment label identifier?		
Treatment kit identifier	Syringe 1 ☐	
	Syringe 2 ☐	
Dose	1 ■	
	2 ☐	
Unit	mL ■	
Was the expected volume administered?	Yes ☐	
	No ☐	
If No, what was the reason the expected volume was not administered?	Adverse Event ☐	
	Device Malfunction ☐	
	Other ☐	

 For annotations, refer to CRF page 172

If Adverse Event, choose the corresponding AE log line, start date, and term	
If Other, specify	
Formulation	Aerosol ☐ Capsule ☐ Delayed Release Capsule ☐ Extended Release Capsule ☐ Cream ☐ For Solution ☐ For Suspension ☐ Gel ☐ Implant ☐ Inhalant ☐ Injectable ■ Liquid ☐ Lotion ☐ Mouthwash ☐ Ointment ☐ Patch ☐ Powder ☐ Shampoo ☐ Soap ☐ Solution ☐ Spray ☐ Suppository ☐ Suspension ☐ Tablet ☐ Delayed Release Tablet ☐ Extended Release Tablet ☐ Tape ☐ Minitablets in Capsule ☐ Unassigned ☐ Not Applicable ☐

 For annotations, refer to CRF page 172

Route	Auricular ☐ Intramuscular ☐ Intravenous ☐ Nasal ☐ Ophthalmic ☐ Oral ☐ Parenteral ☐ Rectal ☐ Inhalation ☐ Subcutaneous ■ Sublingual ☐ Topical ☐ Other ☐ Unknown ☐ Not Applicable ☐
What was the anatomical location of the administration?	Upper Right Arm ☐ Upper Left Arm ☐ Right Thigh ☐ Left Thigh ☐ Lower Right Abdominal ☐ Lower Left Abdominal ☐ Other ☐
Start Date	_____
Actual Start Time	_____

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Subcutaneous Injection EX_GL_900A2
Generated On: 13 Jul 2020 10:14:40 ECTRT = MASKED

Treatment	ECCAT		EXCAT2; Entered; Char
Was the syringe administered?	ECOCCUR	Yes ☐	EXYN_DOS; Coded; Ch
		No ☐	
If No, what was the reason the syringe was not administered? ECREASOC in SUPPEC when IDVAR = ECSEQ		Adverse Event ☐	EXREASND; Coded; Ch
		Dose Skipped ☐	
		Other ☐	
If Adverse Event, choose the corresponding AE log line, start date, and term	Linked to related AE record via RELREC when RELID starts with "SYR"		AEDSL2; Entered; Char
If Other, Specify	ECREASOO in SUPPEC when IDVAR = ECSEQ		EXRSNDO; Entered; Ch
What is the treatment label identifier?	ECREFID		EXREFID; Entered; Cha
Dose	ECDOSE	1 ■	EXDOSE; Coded; Num
		2 ☐	
Unit	ECDOSU	mL ■	EXDOSEU; Coded; Cha
Was the expected volume administered?	ECTOTAMT in SUPPEC when IDVAR = ECSEQ	Yes ☐	EXADMYN; Coded; Cha
		No ☐	
If No, what was the reason the expected volume was not administered?	ECADJ	Adverse Event ☐	EXADMREA; Coded; Ch
		Device Malfunction ☐	
		Other ☐	
If Adverse Event, choose the corresponding AE log line, start date, and term	Linked to related AE record via RELREC when RELID starts with "VOL"		AEDSL; Entered; Char
If Other, specify	ECADJOTH in SUPPEC when IDVAR = ECSEQ		EXADMOSP; Entered; C
Formulation	ECDOSFRM	Aerosol ☐	EXDOSFRM; Coded; Ch
		Capsule ☐	
		Delayed Release Capsule ☐	
		Extended Release Capsule ☐	
		Cream ☐	
		For Solution ☐	
		For Suspension ☐	
		Gel ☐	
		Implant ☐	
		Inhalant ☐	
		Injectable ■	
		Liquid ☐	

		Lotion ☐
		Mouthwash ☐
		Ointment ☐
		Patch ☐
		Powder ☐
		Shampoo ☐
		Soap ☐
		Solution ☐
		Spray ☐
		Suppository ☐
		Suspension ☐
		Tablet ☐
		Delayed Release Tablet ☐
		Extended Release Tablet ☐
		Tape ☐
		Minitablets in Capsule ☐
		Unassigned ☐
		Not Applicable ☐
Route	*ECROUTE*	Auricular ☐ *EXROUTE; Coded; Cha*
		Intramuscular ☐
		Intravenous ☐
		Nasal ☐
		Ophthalmic ☐
		Oral ☐
		Parenteral ☐
		Rectal ☐
		Inhalation ☐
		Subcutaneous ■
		Sublingual ☐
		Topical ☐
		Other ☐
		Unknown ☐
		Not Applicable ☐

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Subcutaneous Injection EX_GL_900A2
Generated On: 13 Jul 2020 10:14:40

What was the anatomical location of the administration?				Upper Right Arm ☐	EXLOC; Coded; Char
				Upper Left Arm ☐	
				Right Thigh ☐	
				Left Thigh ☐	
	ECDIR	ECLAT	ECLOC	Lower Right Abdominal ☐	
				Lower Left Abdominal ☐	
				Other ☐	
Start Date			ECSTDTC		EXSTDAT; Time; Date
Actual Start Time					EXSTTIM; Time; Date

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Intravenous EX_GL_900B
Generated On: 13 Jul 2020 10:14:40 ECTRT = MASKED

Treatment — ECCAT
- ☐ Golimumab/Placebo
- ■ Guselkumab/Placebo

EXCAT; Coded; Char

Was IV study agent administered? — ECOCCUR
- ☐ Yes
- ☐ No

EXYN_DOS; Coded; Ch

What was the reason the dose was not administered? — ECREASOC in SUPPEC when IDVAR = ECSEQ
- ☐ Adverse Event
- ☐ Dose Skipped
- ☐ Other

EXREASND; Coded; Ch

If Adverse Event, choose the corresponding AE log line, start date, and term — Linked to related AE record via RELREC

AEDSL2; Entered; Char

If Other, Specify — ECREASOO in SUPPEC when IDVAR = ECSEQ

EXRSNDO; Entered; Ch

Was the total volume of study agent administered? — ECTOTAMT in SUPPEC when IDVAR = ECSEQ
- ☐ Yes
- ☐ No

EXTOTAMT; Coded; Ch

If no, specify the total diluted volume of study agent administered — ECDOSE

Fixed Unit: mL

EXAMONT; Entered; Nu

Units — ECDOSU
- ☐ Milligram
- ■ mL
- ☐ Microgram
- ☐ Gram
- ☐ Capsule
- ☐ Implant
- ☐ Patch
- ☐ Spray
- ☐ Tablespoon
- ☐ Tablet
- ☐ Teaspoon
- ☐ Vial
- ☐ International Unit
- ☐ Puff
- ☐ Milligram per Kilogram
- ☐ Milligram per Square Meter
- ☐ Sachet
- ☐ Medical Drop

EXAMONTU; Coded; Ch

Formulation
- ☐ Aerosol

EXDOSFRM; Coded; Ch

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Intravenous ***EX_GL_900B***
Generated On: 13 Jul 2020 10:14:40

ECDOSFRM

- ☐ Capsule
- ☐ Delayed Release Capsule
- ☐ Extended Release Capsule
- ☐ Cream
- ☐ For Solution
- ☐ For Suspension
- ☐ Gel
- ☐ Implant
- ☐ Inhalant
- ☒ Injectable
- ☐ Liquid
- ☐ Lotion
- ☐ Mouthwash
- ☐ Ointment
- ☐ Patch
- ☐ Powder
- ☐ Shampoo
- ☐ Soap
- ☐ Solution
- ☐ Spray
- ☐ Suppository
- ☐ Suspension
- ☐ Tablet
- ☐ Delayed Release Tablet
- ☐ Extended Release Tablet
- ☐ Tape
- ☐ Minitablets in Capsule
- ☐ Unassigned
- ☐ Not Applicable

Route

ECROUTE

- ☐ Auricular ***EXROUTE; Coded; Cha***
- ☐ Intramuscular
- ☒ IV
- ☐ Nasal

EC = Exposure as Collected

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Study Drug/Placebo Administration for Intravenous ***EX_GL_900B***
Generated On: 13 Jul 2020 10:14:40

	Ophthalmic ☐	
	Oral ☐	
	Parenteral ☐	
	Rectal ☐	
	Inhalation ☐	
	Subcutaneous ☐	
	Sublingual ☐	
	Topical ☐	
	Other ☐	
	Unknown ☐	
	Not Applicable ☐	
Start Date	***ECSTDTC***	***EXSTDAT; Time; Date***
Actual Start Time		***EXSTTIM; Time; Date***
Actual End Time	***ECENDTC***	***EXENTIM; Time; Date***

QS = Questionnaires

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Partial Mayo Score - Screening MAYO_SCORE_001

Generated On: 13 Jul 2020 10:14:40 QSCAT = MAYO

Was Partial Mayo Score performed during Screening? [NOT SUBMITTED] Yes ☐ MYYN; Coded; Char

If no then QSSTAT = NOT DONE when QSTESTCD = QSALL No ☐

If No, specify Reason QSREASND Screen Failure ☐ MYOTHSPE; Coded; Nu 1

[NOT SUBMITTED] Other ☐ 9

If Other, Specify QSREASND _____ OTHSPEC; Entered; Ch

Use the most recent consecutive 3-day period within the 1 week before the visit, excluding the following: the day medications for constipation, diarrhea, or irregularity were taken (For subjects maintained on a stable dose of bulking or stool softening agents throughout the study, the days on which these agents are taken should not be excluded from consideration in calculating the Mayo score.); the day of a procedure or preparation for a procedure (eg, enemas, other laxatives, a clear liquid diet) that would affect stool frequency and/or blood content of the stool; the 48 hours after the use of antimotility agents (ie, diphenoxylate hydrochloride with atropine sulfate, loperamide or other opioids); the 48 hours immediately following a colonoscopy.

Please indicate the number of stools and rectal bleeding scores recorded by the patient for each day. Check the boxes next to the 3 days which were used to calculate the Mayo score.

STOOL FREQUENCY QSSCAT = STOOL FREQUENCY

QSTEST Sum the number of Stools for 3 days checked above = H QSORRES when QSTESTCD = MAYO1H MYSSUM; Entered; Num

QSTEST Divide the sum by three and round to one decimal place [H/3] = I Fixed Unit: Average number stools for three days MYSAVG; Entered; Num

QSORRES when QSTESTCD = MAYO1I

QSTEST What is the number of stools in 24 hours when in remission or prior to UC = J MYREM; Entered; Num

QSORRES when QSTESTCD = MAYO1J

QSTEST Subtract number of stools when in remission or prior to UC from average number of stools for three days [I-J] = K MYSUBT; Entered; Num

QSORRES when QSTESTCD = MAYO1K

QSTEST Round number in 'K' to the nearest whole number. Negative numbers should be zero (up if greater than or equal to 0.5 and down if less than 0.5) L Fixed Unit: No.of stools more than normal MYRND; Entered; Num

QSORRES when QSTESTCD = MAYO1L

QSTEST Stool Frequency Score = M QSORRES when QSTESTCD = MAYO1M MYSSC; Entered; Num

RECTAL BLEEDING QSSCAT = RECTAL BLEEDING

QSTEST Sum the rectal bleeding scores for three days checked above U QSORRES when QSTESTCD = MAYC MYRBSUM; Entered; Nu

QSTEST Divide the number by three and round to one decimal place [U/3] = V QSORRES when QSTESTCD = MYRBAVG; Entered; Nu

Rectal Bleeding Score

QSTEST Round number in 'V' to the nearest whole number (up if greater than or equal to 0.5 and down if less than 0.5) W MYRRND; Entered; Num

QSORRES when QSTESTCD = MAYO1W

PHYSICIAN'S GLOBAL ASSESSMENT SCORE Score of 0 (normal) ☐ MYPGA; Coded; Num

QSSCAT = PHYSICIAN'S GLOBAL ASSESSMENT

Score of 1 (mild disease) ☐
Score of 2 (moderate disease) ☐
Score of 3 (severe disease) ☐

QSTEST Physician's Global Assessment Score X *QSORRES when QSTESTCD = MAYO1X* *MYPGAS; Entered; Num*

*Physician's Global Assessment acknowledges the three other criteria, the patient's recall of abdominal discomfort and general sense of well being, and other observations, such as physical findings and the patient's performance status.

SCREENING PARTIAL MAYO SCORE *QSSCAT = PARTIAL MAYO SCORE*

QSTEST Stool Frequency Score + Rectal Bleeding Score + Physician's Global Assessment Score [M+W+X] = Y *QSORRES when QSTESTCD = MAYO1Y* *MYPMS; Entered; Num*

•A screening endoscopy must be performed within 2 weeks and no later than 4 days prior to the baseline (Week 0) visit.
Only complete Endoscopy if the Screening Partial Mayo Score is >= 3 and all other screening criteria are met.

Day *MYDAY; Coded; Num*

[NOT SUBMITTED]

Day 1 ☐
Day 2 ☐
Day 3 ☐
Day 4 ☐
Day 5 ☐
Day 6 ☐
Day 7 ☐

Date *SAMPDATE in SUPPQS where IDVAR = QSSEQ* *MYDT; Time; Date*

QSTEST Number of Stools *QSSCAT = STOOL FREQUENCY* *QSORRES when QSTESTCD = MAYO1A* *MYSFSCR; Entered; Nu*

Code *MYSCDE; Coded; Char*

[NOT SUBMITTED]

A ☐
B ☐
C ☐
D ☐
E ☐
F ☐
G ☐

Check which day is used to calculate Mayoscore *INCLUDED in SUPPQS where IDVAR = QSSEQ* *MYSLCT; Checked; Nu*

QSTEST Rectal Bleeding Score *QSSCAT = RECTAL BLEEDING* *QSORRES when QSTESTCD = MAYO1N* *MYRBSCR; Coded; Nu*

0 = No blood seen ☐
1 = Streaks of blood with stool less than half the time ☐

	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ■ *MYRCDE; Coded; Char*
	O ☐
	P ☐
[NOT SUBMITTED]	Q ☐
	R ☐
	S ☐
	T ☐
Day	Day 1 ☐
	Day 2 ■
	Day 3 ☐
	Day 4 ☐
	Day 5 ☐
	Day 6 ☐
	Day 7 ☐
Date	
Number of Stools	
Code	A ☐
	B ■
	C ☐
	D ☐
	E ☐
	F ☐
	G ☐
Check which day is used to calculate Mayoscore	
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐

For annotations, refer to CRF page 184

	O ■
	P ☐
	Q ☐
	R ☐
	S ☐
	T ☐
Day	Day 1 ☐
	Day 2 ☐
	Day 3 ■
	Day 4 ☐
	Day 5 ☐
	Day 6 ☐
	Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐
	B ☐
	C ■
	D ☐
	E ☐
	F ☐
	G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐
	O ☐
	P ■
	Q ☐

For annotations, refer to CRF page 184

	R ☐
	S ☐
	T ☐
Day	Day 1 ☐
	Day 2 ☐
	Day 3 ☐
	Day 4 ■
	Day 5 ☐
	Day 6 ☐
	Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐
	B ☐
	C ☐
	D ■
	E ☐
	F ☐
	G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐
	O ☐
	P ☐
	Q ■
	R ☐
	S ☐
	T ☐

For annotations, refer to CRF page 184

Day	Day 1 ☐ Day 2 ☐ Day 3 ☐ Day 4 ☐ Day 5 ■ Day 6 ☐ Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐ B ☐ C ☐ D ☐ E ■ F ☐ G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐ 1 = Streaks of blood with stool less than half the time ☐ 2 = Obvious blood with stool most of the time ☐ 3 = Blood alone passed ☐
Code	N ☐ O ☐ P ☐ Q ☐ R ■ S ☐ T ☐
Day	Day 1 ☐ Day 2 ☐ Day 3 ☐

 For annotations, refer to CRF page 184

	Day 4 ☐
	Day 5 ☐
	Day 6 ■
	Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐
	B ☐
	C ☐
	D ☐
	E ☐
	F ■
	G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐
	O ☐
	P ☐
	Q ☐
	R ☐
	S ■
	T ☐
Day	Day 1 ☐
	Day 2 ☐
	Day 3 ☐
	Day 4 ☐
	Day 5 ☐
	Day 6 ☐

 For annotations, refer to CRF page 184

	Day 7
Date	
Number of Stools	
Code	A ☐ B ☐ C ☐ D ☐ E ☐ F ☐ G
Check which day is used to calculate Mayoscore	
Rectal Bleeding Score	0 = No blood seen ☐ 1 = Streaks of blood with stool less than half the time ☐ 2 = Obvious blood with stool most of the time ☐ 3 = Blood alone passed ☐
Code	N ☐ O ☐ P ☐ Q ☐ R ☐ S ☐ T

PR = Procedures

MO = Morphology

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Endoscopy Information - Screening ENDO_INFO_001S
Generated On: 13 Jul 2020 10:14:40 PRCAT, MOCAT = ENDOSCOPY INFORMATION

Was Endoscopy performed during Screening?	PRSTAT PRTRT = ENDOSCOPY INFORMATION	Yes ☐ No ☐	ENYN; Coded; Char
If No, specify Reason	PRREASND [NOT SUBMITTED]	Screening Failure ☐ 1 Other ☐ 2	EOTHSPE; Coded; Num
If Other, Specify	PRREASND		ENDOTH; Entered; Char

The screening endoscopy must be performed within 2 weeks (and at least 4 days) before the baseline (Week 0) visit.
Participants with a partial Mayo score ≥3 can proceed with endoscopy.

Date of Endoscopy(dd MMM yyyy)	PRSTDTC, MODTC		ENDT; Time; Date
Findings on Endoscopy (By local reader) MOTEST = Score MOEVAL = LOCAL READER MOORRES when MOTESTCD = SCORE Link between related PR and MO records via RELREC		0 = Normal or inactive disease ☐ 1 = Mild disease (erythema, decreased vascular pattern, mild friability) ☐ 2 = Moderate disease (marked erythema, absent vascular pattern, friability, erosion) ☐ 3 = Severe disease (spontaneous bleeding, ulceration) ☐	ENMAYO; Coded; Num
Extent of Disease MOTEST	MOORRES when MOTESTCD = EXTENT	Extensive ☐ 1 Limited to left side of colon ☐ 2	ENDISEXT; Coded; Num
Type of Endoscopy Performed	PRTRT MOMETHOD	Sigmoidoscopy ☐ 1 Colonoscopy ☐ 2	ENTP; Coded; Num

Use the most recent consecutive 3-day period within the 1 week before the visit, excluding the following: the day medications for constipation, diarrhea, or irregularity were taken (For subjects maintained on a stable dose of bulking or stool softening agents throughout the study, the days on which these agents are taken should not be excluded from consideration in calculating the Mayo score.); the day of a procedure or preparation for a procedure (eg, enemas, other laxatives, a clear liquid diet) that would affect stool frequency and/or blood content of the stool; the 48 hours after the use of antimotility agents (ie, diphenoxylate hydrochloride with atropine sulfate, loperamide or other opioids); the 48 hours immediately following a colonoscopy.

Please indicate the number of stools and rectal bleeding scores recorded by the patient for each day. Check the boxes next to the 3 days which will be used to calculate the Mayo score.

STOOL FREQUENCY

Sum the number of Stools for 3 days checked above = H		*MYSSUM; Entered; Num*
Divide the sum by three and round to one decimal place [H/3] = I	Fixed Unit: Average number stools for three days	*MYSAVG; Entered; Num*
What is the number of stools in 24 hours when in remission or prior to UC **(from Screening Mayo Diary Card)** = J		*MYREM1; Entered; Num*
Subtract number of stools when in remission or prior to UC from average number of stools for three days [I-J] = K		*MYSUBT1; Entered; Nu*
Round number in 'K' to the nearest whole number. Negative numbers should be zero (up if greater than or equal to 0.5 and down if less than 0.5) L	Fixed Unit: No.of stools more than normal	*MYRND; Entered; Num*
Stool Frequency Score = M		*MYSSC; Entered; Num*

RECTAL BLEEDING

Sum the rectal bleeding scores for three days checked above U		*MYRBSUM; Entered; Nu*
Divide the number by three and round to one decimal place [U/3] = V		*MYRBAVG; Entered; Nu*
Rectal Bleeding Score		
Round number in 'V' to the nearest whole number (up if greater than or equal to 0.5 and down if less than 0.5) W		*MYRRND; Entered; Num*

PHYSICIAN'S GLOBAL ASSESSMENT	Score of 0 (normal) ☐	*MYPGA; Coded; Num*
	Score of 1 (mild disease) ☐	
	Score of 2 (moderate disease) ☐	
	Score of 3 (severe disease) ☐	
Physician's Global Assessment Score X		*MYPGAS; Entered; Num*

*Physician's Global Assessment acknowledges the three other criteria, the patient's recall of abdominal discomfort and general sense of well being, and other observations, such as physical findings and the patient's performance status.

PARTIAL MAYO SCORE

Stool Frequency Score + Rectal Bleeding Score + Physician's Global Assessment Score [M+W+X] = Y _____ *MYPMS; Entered; Num*

Day *MYDAY; Coded; Num*
- Day 1
- Day 2
- Day 3
- Day 4
- Day 5
- Day 6
- Day 7

Date _____ *MYDT; Time; Date*

Number of Stools _____ *MYSFSCR; Entered; Nu*

Code *MYSCDE; Coded; Char*
- A
- B
- C
- D
- E
- F
- G

Check which day is used to calculate Mayoscore _____ *MYSLCT; Checked; Nur*

Rectal Bleeding Score *MYRBSCR; Coded; Num*
- 0 = No blood seen
- 1 = Streaks of blood with stool less than half the time
- 2 = Obvious blood with stool most of the time
- 3 = Blood alone passed

Code *MYRCDE; Coded; Char*
- N
- O
- P
- Q
- R
- S

For annotations, refer to CRF page 183

	T ☐
Day	Day 1 ☐ Day 2 ■ Day 3 ☐ Day 4 ☐ Day 5 ☐ Day 6 ☐ Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐ B ■ C ☐ D ☐ E ☐ F ☐ G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐ 1 = Streaks of blood with stool less than half the time ☐ 2 = Obvious blood with stool most of the time ☐ 3 = Blood alone passed ☐
Code	N ☐ O ■ P ☐ Q ☐ R ☐ S ☐ T ☐
Day	Day 1 ☐ Day 2 ☐

For annotations, refer to CRF page 183

	Day 3 ■
	Day 4 ☐
	Day 5 ☐
	Day 6 ☐
	Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐
	B ☐
	C ■
	D ☐
	E ☐
	F ☐
	G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐
	O ☐
	P ■
	Q ☐
	R ☐
	S ☐
	T ☐
Day	Day 1 ☐
	Day 2 ☐
	Day 3 ☐
	Day 4 ■
	Day 5 ☐

For annotations, refer to CRF page 183

	Day 6 ☐
	Day 7 ☐
Date	
Number of Stools	
Code	A ☐
	B ☐
	C ☐
	D ■
	E ☐
	F ☐
	G ☐
Check which day is used to calculate Mayoscore	
Rectal Bleeding Score	0 = No blood seen ☐
	1 = Streaks of blood with stool less than half the time ☐
	2 = Obvious blood with stool most of the time ☐
	3 = Blood alone passed ☐
Code	N ☐
	O ☐
	P ☐
	Q ■
	R ☐
	S ☐
	T ☐
Day	Day 1 ☐
	Day 2 ☐
	Day 3 ☐
	Day 4 ☐
	Day 5 ■
	Day 6 ☐
	Day 7 ☐
Date	
Number of Stools	

For annotations, refer to CRF page 183

Code	A ☐ B ☐ C ☐ D ☐ E ■ F ☐ G ☐
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐ 1 = Streaks of blood with stool less than half the time ☐ 2 = Obvious blood with stool most of the time ☐ 3 = Blood alone passed ☐
Code	N ☐ O ☐ P ☐ Q ☐ R ■ S ☐ T ☐
Day	Day 1 ☐ Day 2 ☐ Day 3 ☐ Day 4 ☐ Day 5 ☐ Day 6 ■ Day 7 ☐
Date	_____
Number of Stools	_____
Code	A ☐ B ☐ C ☐

For annotations, refer to CRF page 183

D ☐
E ☐
F ■
G ☐

Check which day is used to calculate Mayoscore _____

Rectal Bleeding Score

0 = No blood seen ☐
1 = Streaks of blood with stool less than half the time ☐
2 = Obvious blood with stool most of the time ☐
3 = Blood alone passed ☐

Code

N ☐
O ☐
P ☐
Q ☐
R ☐
S ■
T ☐

Day

Day 1 ☐
Day 2 ☐
Day 3 ☐
Day 4 ☐
Day 5 ☐
Day 6 ☐
Day 7 ■

Date _____

Number of Stools _____

Code

A ☐
B ☐
C ☐
D ☐
E ☐
F ☐

For annotations, refer to CRF page 183

	G
Check which day is used to calculate Mayoscore	_____
Rectal Bleeding Score	0 = No blood seen ☐ 1 = Streaks of blood with stool less than half the time ☐ 2 = Obvious blood with stool most of the time ☐ 3 = Blood alone passed ☐
Code	N ☐ O ☐ P ☐ Q ☐ R ☐ S ☐ T

 For annotations, refer to CRF page 191

Date of Endoscopy (dd MMM yyyy)	_____	ENDT; Time; Date
Findings on Endoscopy (By local reader)	0 = Normal or inactive disease ☐ 1 = Mild disease (erythema, decreased vascular pattern, mild friability) ☐ 2 = Moderate disease (marked erythema, absent vascular pattern, friability, erosion) ☐ 3 = Severe disease (spontaneous bleeding, ulceration) ☐	ENMAYO; Coded; Num
Type of Endoscopy Performed	Sigmoidoscopy ☐ 1 Colonoscopy ☐ 2	ENTP; Coded; Num

DD = Death Details

DM = Demographics

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: Death Information *DD_GL_900*

Generated On: 13 Jul 2020 10:14:40

Collection Date	*DDDTC*	*DDDAT; Time; Date*
Death Date	*DTHDTC*	*DTHDAT; Time; Date*
What was the primary cause of death? *DDORRES when DDTESTCD = PRCDTH*	Adverse Event ☐ Other ☐ *[NOT SUBMITTED]*	*PRCDTH; Coded; Char*
If 'Adverse Event', choose the corresponding AE log line, start date, and term	*Linked to related AE record via RELREC*	*AEDSL1; Entered; Char*
AE log line, start date, and term		*AEDSL2; Entered; Char*
AE log line, start date, and term		*AEDSL3; Entered; Char*
If Other, Specify	*DDORRES when DDTESTCD = PRCDTH and DDSTRESC = OTHER*	*PRCDTH_OTH; Entered*
DDTEST Was an autopsy performed? *DDORRES when DDTESTCD = AUTOPIND*	Yes ☐ No ☐ Unknown ☐	*AUTOPSYN; Coded; Ch*

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Updated Study Informed Consent ***DS_GL_910***
Generated On: 13 Jul 2020 10:14:40

Category	***DSCAT*** PROTOCOL MILESTONE ***DSCAT; Coded; Char***
What was the date of informed consent obtained?	***DSSTDTC when DSDECOD = INFORMED CONSE*** ***DSSTDAT; Time; Date***
What was the issue/report date of the protocol the subject consented to?	***SPRTDTC in SUPPDS when IDVAR = DSSEQ*** ***SPRTDAT; Time; Date***

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Treatment Unblinding DS_GL_903
Generated On: 13 Jul 2020 10:14:40

Category	DSCAT OTHER EVENT	DSCAT; Coded; Char
Was treatment unblinded by the site?	Yes / No [NOT SUBMITTED]	DSUNBLND; Coded; Ch
What was the date of unblinding?	DSSTDTC	DSSTDAT; Time; Date
What was the time of unblinding?		DSSTTIM; Time; Date
What was the reason fo	DSTERM when DSDECOD = TREATMENT UNBLINDED Adverse Event / Other [NOT SUBMITTED]	DSTERM_UNRS; Coded
If Adverse Event, choose corresponding AE log line, start date, and term	Linked to related AE record via RELREC	AEDSL1; Entered; Char
AE log line, start date, and term		AEDSL2; Entered; Char
AE log line, start date, and term		AEDSL3; Entered; Char
If Other, Specify	DSTERM when DSDECOD = TREATMENT UNBLINDED	DSTERM_OTH; Entered

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Treatment Disposition DS_GL_908
Generated On: 13 Jul 2020 10:14:40

Category	DSCAT DISPOSITION EVENT	DSCAT; Coded; Char
Subcategory	DSSCAT TREATMENT	DSSCAT_TRT; Entered;
What was the subject's status?	DSTERM Completed [NOT SUBMITTED] Discontinued	DSDECOD; Coded; Cha
Treatment Disposition Date	DSSTDTC	DSSTDAT_TDD; Time; D
What was the subject's primary reason for treatment discontinuation?	DSTERM Adverse Event - Other Adverse Event - Worsening of Ulcerative Colitis Death Initiated Prohibited Medication Lack of Efficacy Protocol Deviation Pregnancy Study Terminated by Sponsor [NOT SUBMITTED] Participant Discontinuation/Withdrawal from study [NOT SUBMITTED] Other	DSDECOD_REAS; Code
If Adverse Event, choose corresponding AE log line, start date, and term	Linked to related AE record via RELREC	AEDSL1; Entered; Char
AE log line, start date, and term		AEDSL2; Entered; Char
AE log line, start date, and term		AEDSL3; Entered; Char
AE log line, start date, and term		AEDSL4; Entered; Char
AE log line, start date, and term		AEDSL5; Entered; Char
If Other, Specify	DSTERM when DSDECOD = OTHER	DSTERM_OTH; Entered
If Withdrawal by Subject, Specify	DSTERM when DSDECOD = WITHDRAWAL BY SUBJECT Enrolling in another study Lack of Improvement Lack of Transportation Loss of Informant Study Tests Too Hard [NOT SUBMITTED] Other	DSTERM_WITH; Coded;
If Other, Specify	DSTERM when DSDECOD = WITHDRAWAL BY SUBJECT	DSTERM_WITHOTH; En
If Pregnancy, Pregnancy Due Date	DSPREDTC in SUPPDS when IDVAR = DSSEQ	DSPREDAT; Time; Date

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Trial Disposition DS_GL_900
Generated On: 13 Jul 2020 10:14:40

Category	DSCAT DISPOSITION EVENT	DSCAT; Coded; Char
Subcategory	DSSCAT TRIAL	DSSCAT; Entered; Char
What was the subject's status?	DSTERM Completed [NOT SUBMITTED] Discontinued	DSDECOD; Coded; Cha
Disposition Date	DSSTDTC	DSSTDAT; Time; Date
What was the subject's primary reason for trial discontinuation?	Adverse Event - Other Adverse Event-Worsening of Ulcerative Colitis Screen Failure (Failure to Meet Eligibility Criteria) Death Pregnancy Lost to Follow-Up Withdrawal by Subject Study Terminated by Sponsor [NOT SUBMITTED] Other	DSDECOD_REAS; Code
	DSTERM	
If Adverse Event, choose corresponding AE log line, start date, and term	Linked to related AE record via RELREC	AEDSL1; Entered; Char
AE log line, start date, and term		AEDSL2; Entered; Char
AE log line, start date, and term		AEDSL3; Entered; Char
AE log line, start date, and term		AEDSL4; Entered; Char
AE log line, start date, and term		AEDSL5; Entered; Char
If Other, Specify	DSTERM when DSDECOD = OTHER	DSTERM; Entered; Char
If Withdrawal by Subject, Specify	Enrolling in another study Lack of Improvement Lack of Transportation Loss of Informant Study Tests Too Hard [NOT SUBMITTED] Other	DSTERM_WITH; Coded;
	DSTERM when DSDECOD = WITHDRAWAL BY SUBJECT	
If Other, Specify	DSTERM when DSDECOD = WITHDRAWAL BY SUBJECT	DSTERM_OTH_GL2; En
If Pregnancy, Pregnancy Due Date	DSPREDTC in SUPPDS when IDVAR = DSSEQ	PREGDAT; Time; Date
Following date fields are used for CTMS		
Completed Date (CTMS field)	[NOT SUBMITTED]	DSCOMDT; Time; Date
Discontinued Date (CTMS field)	[NOT SUBMITTED]	DSDISCDT; Time; Date
Screen Failed Date (CTMS field)	[NOT SUBMITTED]	DSSCRFDT; Time; Date

When data entry and source document verification has been completed up to the interim cut off point for this subject, sign-off for the interim closure can take place.

Interim lock	***INTLK; Entered; Char***
I have verified that all data in this case book were completed up to the cutoff and I am ready to proceed with sign-off for interim closure.	***SIGNCHEK; Checked; N***

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Subject Site Switch DS_GL_904
Generated On: 13 Jul 2020 10:14:40

Category	DSCAT OTHER EVENT	DSCAT; Coded; Char
Subject Site Switch	DSTERM, DSDECOD = SUBJECT SITE SWITCH SUBJECT SITE SWITCH	DSDECOD; Entered; Ch
Provide new Site ID	DSDMSITE in SUPPDS when IDVAR = DSSEQ	DSDMSITE; Entered; Ch
Date subject started at new site	DSSTDTC	SITE2DAT; Time; Date

XZ = Sample Handling

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Limitation of Retention of Samples for Future Research XZ_GL_901_1
Generated On: 13 Jul 2020 10:14:40

Category	XZCAT	LIMITATION OF RETENTION OF SAMPLES FOR FUTURE RESEARCH ☐ OPTIONAL SAMPLE(S) FOR RESEARCH COLLECTION ☐ LIMITATION OF RETENTION OF SAMPLES FOR FUTURE RESEARCH FOR OPTIONAL SAMPLE FOR GENETIC RESEARCH ☐ LIMITATION ON RETENTION OF SAMPLES FOR FUTURE RESEARCH ☐	XZCAT; Coded; Char
XZTEST Did local or country regulations limit the length of storage of samples at this site (to a duration less than that specified in the protocol)? XZORRES when XZTESTCD = STORLIMT		Yes ☐ No ☐	XZSLYN; Coded; Char

For annotations, refer to CRF page 208

Category	LIMITATION OF RETENTION OF SAMPLES FOR FUTURE RESEARCH ☐ OPTIONAL SAMPLE(S) FOR RESEARCH COLLECTION ☐ LIMITATION OF RETENTION OF SAMPLES FOR FUTURE RESEARCH FOR OPTIONAL SAMPLE FOR GENETIC RESEARCH ■ LIMITATION ON RETENTION OF SAMPLES FOR FUTURE RESEARCH ☐	XZCAT; Coded; Char
Did local or country regulations limit the length of storage of samples at this site (to a duration less than that specified in the protocol)?	Yes ☐ No ☐	XZSLYN; Coded; Char

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Withdrawal of Consent for Optional Samples for Genetic Only Research ***BE_GL_903***
Generated On: 13 Jul 2020 10:14:40

Category	*[NOT SUBMITTED]*	OPTIONAL SAMPLES FOR GENETIC ONLY RESEARCH ☐ OPTIONAL SAMPLES FOR SCIENTIFIC RESEARCH ☐ STORAGE OF SAMPLES FOR FUTURE RESEARCH ☐	*BECAT; Coded; Char*
Was consent withdrawn for providing optional sample(s) for genetic research?	*[NOT SUBMITTED]*	Yes ☐ No ☐	*BEWDNAYN; Coded; Cl*
If Yes, date consent withdrawn		*DSSTDTC*	*BESTDAT; Time; Date*

DSCAT = DISPOSITION EVENT
DSTERM, DSDECOD = INFORMED CONSENT WITHDRAWAL FOR OPTIONAL SAMPLES FOR GENETIC ONLY RESEARCH

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Withdrawal of Consent for Storage of Samples for Future Research ***BE_GL_905***
Generated On: 13 Jul 2020 10:14:40

Category		OPTIONAL SAMPLES FOR GENETIC ONLY RESEARCH ☐ OPTIONAL SAMPLES FOR SCIENTIFIC RESEARCH ☐ STORAGE OF SAMPLES FOR FUTURE RESEARCH ■	*BECAT; Coded; Char*
	[NOT SUBMITTED]		
Was consent withdrawn for storage of sample(s) for future research?		Yes ☐	*BEWFRYN; Coded; Cha*
	[NOT SUBMITTED]	No ☐	
If Yes, date consent withdrawn		*DSSTDTC*	*BESTDAT; Time; Date*

DSCAT = DISPOSITION EVENT
DSTERM, DSDECOD = INFORMED CONSENT WITHDRAWAL FOR OPTIONAL SAMPLES FOR FUTURE RESEARCH

DS = Disposition

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique
Project Name: CNTO1959UCO2002
Form: Withdrawal of Consent for Participation in Substudy ***DS_GL_906***
Generated On: 13 Jul 2020 10:14:40

Category	*[NOT SUBMITTED]* OTHER EVENT	*DSCAT; Entered; Char*
Substudy Name *DSCAT = DISPOSITION EVENT*	Week 4 Endoscopy and Biopsy Substudy *DSSCAT*	*DSSCAT; Coded; Char*
Consent Withdrawal Date	*DSSTDTC*	*DSSTDAT; Time; Date*

DSTERM, DSDECOD = INFORMED CONSENT WITHDRAWAL FOR SUB-STUDY PARTICIPATION

FA=Findings About Events or Interventions

CNTO1959UCO2002 Version 5.0 20200713 DS: Unique

Project Name: CNTO1959UCO2002

Form: [COVID-19 Related] Remote/Home Visit Information ***FA_GL_900***

Generated On: 13 Jul 2020 10:14:40

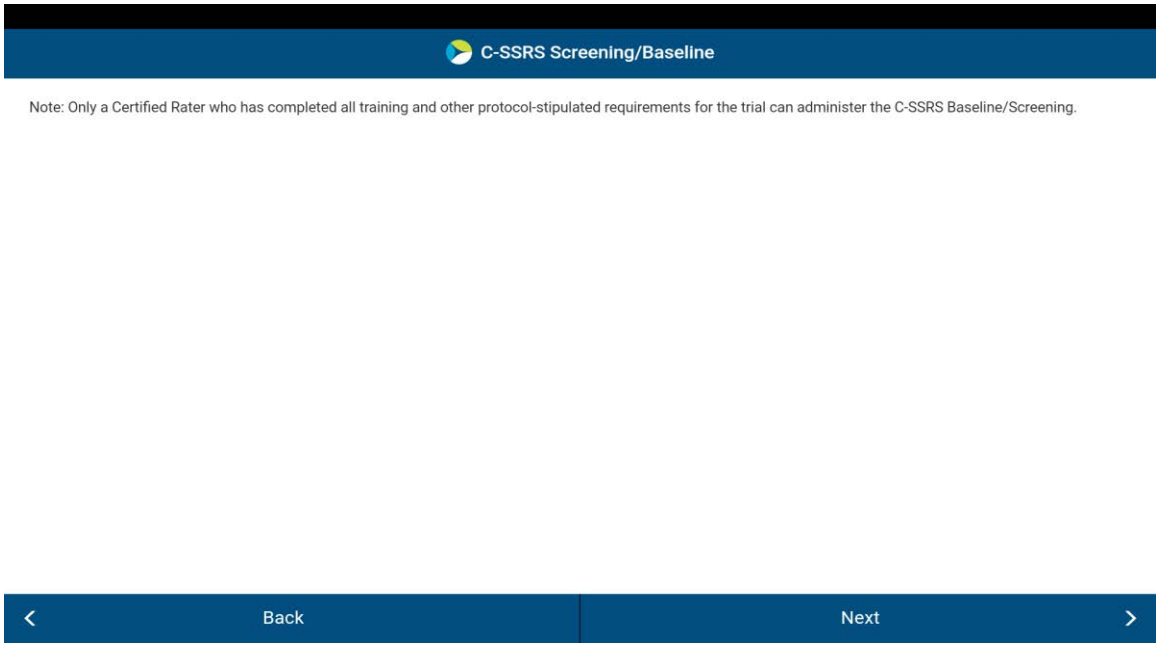
Were any visits [or any part of the visit] performed remotely/at home [due to COVID-19 related reasons]? If 'Yes', add a log line for each remote/home visit	***[NOT SUBMITTED]***	Yes ☐ No ☐	***FAPERF; Coded; Char***
Remote/Home Visit Name	***FAOBJ***		***FAOBJ_DSL; Entered; C***
FATEST Type of Contact	***FAORRES when FATESTCD=TYPVCON***	Home Visit ☐ Remote, Telephone Call ☐ Remote, Video Call ☐ ***[NOT SUBMITTED]*** Other ☐	***TYPVCON; Coded; Char***
If 'Other,' Specify	***FAORRES, FASTRESC=OTHER when FATESTCD=TYPVCON***		***FATPCONO; Entered; C***
Now	***[NOT SUBMITTED]***		***FAGL900NOW; Time; D***

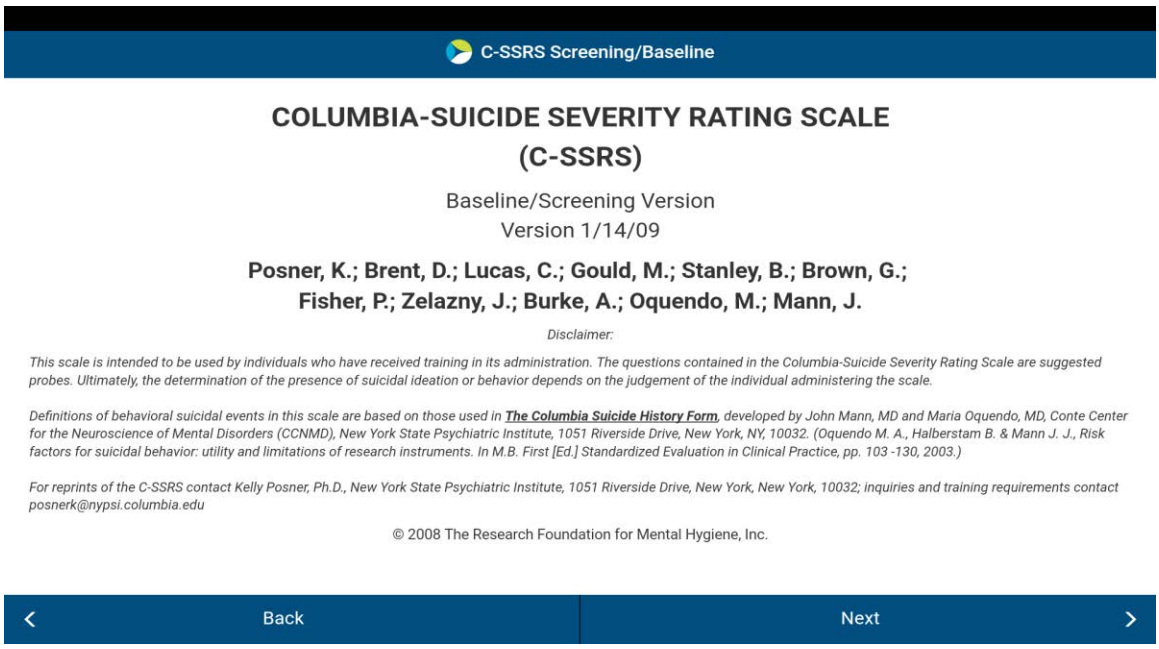
Note:
1. FADTC is populated using the date on the date of visit form. This data should be linked based on visit date of Date of visit CRF.
2. FAOBJ must match with the visit name as specified in the SV domain. FAOBJ value is a link to the SV.VISIT.

Janssen CNTO1959UCO2002

C-SSRS Baseline Screening eCOA Tablet Screenshots

Project Information	
Assessment Name:	C-SSRS Baseline Screening
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1


C-SSRS Screening/Baseline
Note: Only a Certified Rater who has completed all training and other protocol-stipulated requirements for the trial can administer the C-SSRS Baseline/Screening.
Back
Next

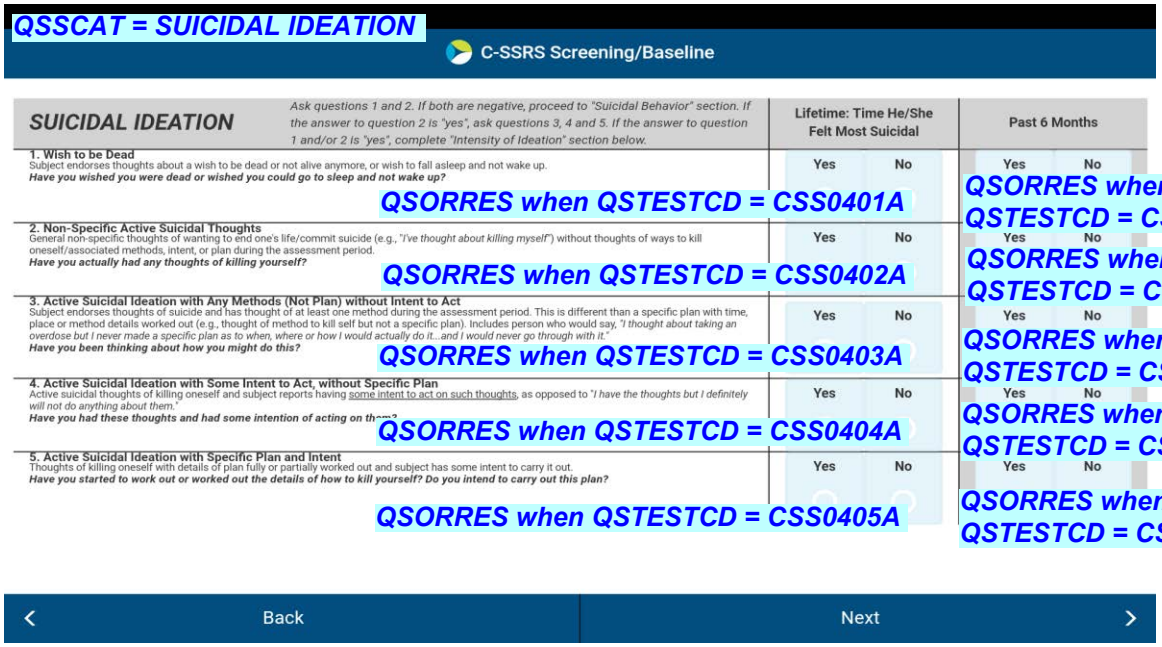

C-SSRS Screening/Baseline
COLUMBIA-SUICIDE SEVERITY RATING SCALE
(C-SSRS)
Baseline/Screening Version
Version 1/14/09
Posner, K.; Brent, D.; Lucas, C.; Gould, M.; Stanley, B.; Brown, G.;
Fisher, P.; Zelazny, J.; Burke, A.; Oquendo, M.; Mann, J.
Disclaimer:
This scale is intended to be used by individuals who have received training in its administration. The questions contained in the Columbia-Suicide Severity Rating Scale are suggested probes. Ultimately, the determination of the presence of suicidal ideation or behavior depends on the judgement of the individual administering the scale.
Definitions of behavioral suicidal events in this scale are based on those used in The Columbia Suicide History Form, developed by John Mann, MD and Maria Oquendo, MD, Conte Center for the Neuroscience of Mental Disorders (CCNMD), New York State Psychiatric Institute, 1051 Riverside Drive, New York, NY, 10032. (Oquendo M. A., Halberstam B. & Mann J. J., Risk factors for suicidal behavior: utility and limitations of research instruments. In M.B. First [Ed.] Standardized Evaluation in Clinical Practice, pp. 103 -130, 2003.)
For reprints of the C-SSRS contact Kelly Posner, Ph.D., New York State Psychiatric Institute, 1051 Riverside Drive, New York, New York, 10032; inquiries and training requirements contact posnerk@nypsi.columbia.edu
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QS = Questionnaires

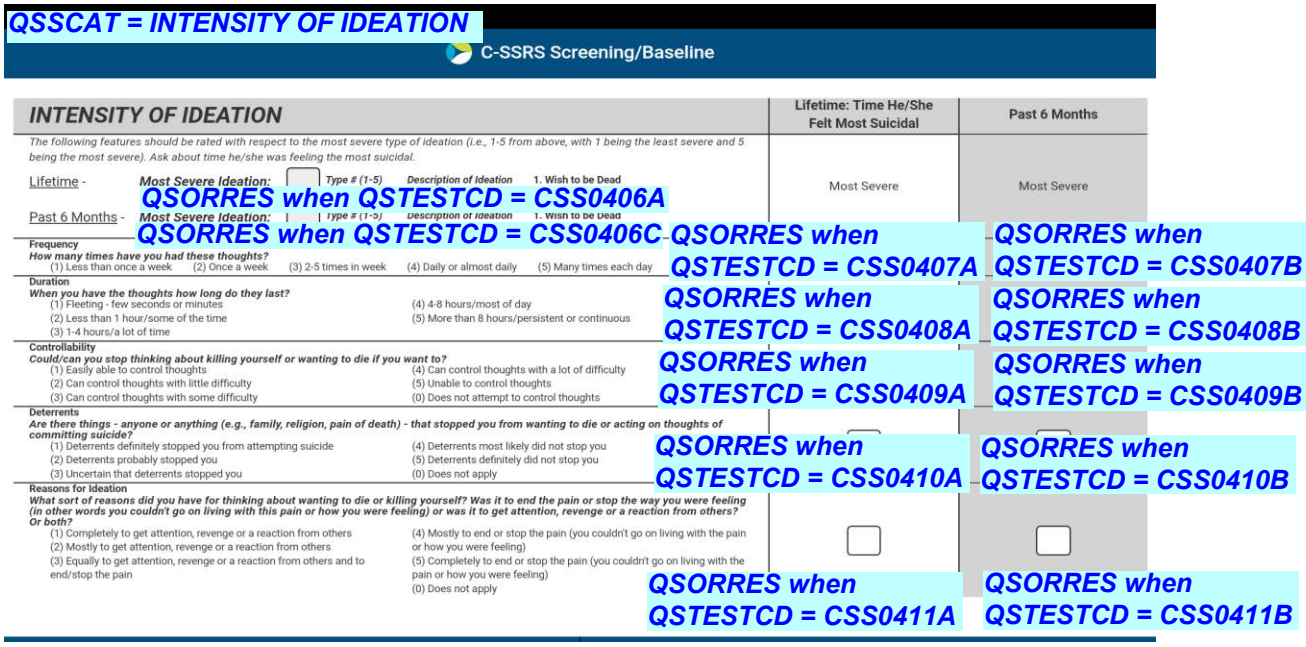
QSEVAL = STUDY SUBJECT

QSCAT = C-SSRS BASELINE/SCREENING VERSION

QSSCAT = SUICIDAL IDEATION



QSSCAT = INTENSITY OF IDEATION



For annotations, refer to CRF page 3

C-SSRS Screening/Baseline

INTENSITY OF IDEATION	Lifetime: Time He/She Felt Most Suicidal	Past 6 Months
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe). Ask about time he/she was feeling the most suicidal. Lifetime - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **2. Non-Specific Active Suicidal Thoughts** Past 6 Months - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **2. Non-Specific Active Suicidal Thoughts**	Most Severe	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐	☐
Duration ***When you have the thoughts how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐	☐

C-SSRS Screening/Baseline

INTENSITY OF IDEATION	Lifetime: Time He/She Felt Most Suicidal	Past 6 Months
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe). Ask about time he/she was feeling the most suicidal. Lifetime - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act** Past 6 Months - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act**	Most Severe	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐	☐
Duration ***When you have the thoughts how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐	☐

For annotations, refer to CRF page 3

C-SSRS Screening/Baseline

INTENSITY OF IDEATION	Lifetime: Time He/She Felt Most Suicidal	Past 6 Months
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe). Ask about time he/she was feeling the most suicidal. Lifetime - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan** Past 6 Months - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan**	Most Severe	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐	☐
Duration ***When you have the thoughts how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐	☐

C-SSRS Screening/Baseline

INTENSITY OF IDEATION	Lifetime: Time He/She Felt Most Suicidal	Past 6 Months
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe). Ask about time he/she was feeling the most suicidal. Lifetime - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **5. Active Suicidal Ideation with Specific Plan and Intent** Past 6 Months - ***Most Severe Ideation:*** ☐ ***Type # (1-5)*** ***Description of Ideation*** **5. Active Suicidal Ideation with Specific Plan and Intent**	Most Severe	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐	☐
Duration ***When you have the thoughts how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐	☐

QS = Questionnaires

QSEVAL = STUDY SUBJECT

QSCAT = C-SSRS BASELINE/SCREENING VERSION

QSSCAT = SUICIDAL BEHAVIOR

C-SSRS Screening/Baseline

SUICIDAL BEHAVIOR (Check all that apply, so long as these are separate events; must ask about all types)	Lifetime	Past 5 Years
Actual Attempt: A potentially self-injurious act committed with at least some wish to die, *as a result of the act*. Behavior was in part thought of as method to kill oneself. Intent does not have to be 100%. If there is ***any*** intent/desire to die associated with the act, then it can be considered an actual suicide attempt. ***There does not have to be any injury or harm***, just the potential for injury or harm. If person pulls trigger while gun is in mouth but gun is broken so no injury results, this is considered an attempt. Inferring Intent: Even if an individual denies intent/wish to die, it may be inferred clinically from the behavior or circumstances. For example, a highly lethal act that is clearly not an accident so no other intent but suicide can be inferred (e.g., gunshot to head, jumping from window of a high floor/story). Also, if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred.	Yes / No	Yes / No
Have you made a suicide attempt?	QSORRES when QSTESTCD = CSS0412A	QSORRES when QSTESTCD = CSS0412B
Have you done anything to harm yourself?		
Have you done anything dangerous where you could have died?		
What did you do?		
Did you _____ as a way to end your life?		
Did you want to die (even a little) when you _____?		
Were you trying to end your life when you _____?		
Or did you think it was possible you could have died from _____?	Total # of Attempts	Total # of Attempts
Or did you do it purely for other reasons / without ANY intention of killing yourself (like to relieve stress, feel better, get sympathy, or get something else to happen)? *(Self-Injurious Behavior without suicidal intent)*	QSORRES when QSTESTCD = CSS0413A	QSORRES when QSTESTCD = CSS0413B
Has subject engaged in Non-Suicidal Self-Injurious Behavior?	Yes / No	Yes / No
	QSORRES when QSTESTCD = CSS0414A	QSORRES when QSTESTCD = CSS0414B

QSSCAT = SUICIDAL BEHAVIOR

SUICIDAL BEHAVIOR (Check all that apply, so long as these are separate events; must ask about all types)	Lifetime	Past 5 Years
Interrupted Attempt: When the person is interrupted (by an outside circumstance) from starting the potentially self-injurious act *(if not for that, actual attempt would have occurred)*. Overdose: Person has pills in hand but is stopped from ingesting. Once they ingest any pills, this becomes an attempt rather than an interrupted attempt. Shooting: Person has gun pointed toward self, gun is taken away by someone else, or is somehow prevented from pulling trigger. Once they pull the trig even if the gun fails to fire, it is an attempt. Jumping: Person is poised to jump, is grabbed and taken down from ledge. Hanging: Person has noose arou neck but has not yet started to hang - is stopped from doing so.	Yes / No	Yes / No
Has there been a time when you started to do something to end your life but someone or something stopped you before you actually did anything?	QSORRES when QSTESTCD = CSS0415A	QSORRES when QSTESTCD = CSS0415B
	Total # of Interrupted	Total # of Interrupted
	QSORRES when QSTESTCD = CSS0416A	QSORRES when QSTESTCD = CSS0416B
Aborted Attempt: When person begins to take steps toward making a suicide attempt, but stops themselves before they actually have engaged in any self-destructive behavior. Examples are similar to interrupted attempts, except that the individual stops him/herself, instead of being stopped by something else.	Yes / No	Yes / No
Has there been a time when you started to do something to try to end your life but you stopped yourself before you actually did anything?	QSORRES when QSTESTCD = CSS0417A	QSORRES when QSTESTCD = CSS0417B
	Total # of Aborted	Total # of Aborted
	QSORRES when QSTESTCD = CSS0418A	QSORRES when QSTESTCD = CSS0418B
Preparatory Acts or Behavior: Acts or preparation towards imminently making a suicide attempt. This can include anything beyond a verbalization or thought, such as assembling a specific method (e.g., buying pills, purchasing a gun) or preparing for one's death by suicide (e.g., giving things away, writing a suicide note).	Yes / No	Yes / No
Have you taken any steps towards making a suicide attempt or preparing to kill yourself (such as collecting pills, getting a gun, giving valuables away or writing a suicide note)?	QSORRES when QSTESTCD = CSS0419A	QSORRES when QSTESTCD = CSS0419B
Suicidal Behavior: Suicidal behavior was present during the assessment period?	Yes / No	Yes / No
	QSORRES when QSTESTCD = CSS0420A	QSORRES when QSTESTCD = CSS0420B

QS = Questionnaires

QSEVAL = STUDY SUBJECT
QSCAT = C-SSRS BASELINE/SCREENING VERSION

QSSCAT = ACTUAL ATTEMPTS

Answer for Actual Attempts Only	DAY MON YEAR	DAY MON YEAR	DAY MON YEAR
Actual Lethality/Medical Damage: 0. No physical damage or very minor physical damage (e.g., surface scratches). 1. Minor physical damage (e.g., lethargic speech; first-degree burns; mild bleeding; sprains). 2. Moderate physical damage; medical attention needed (e.g., conscious but sleepy, somewhat responsive; second-degree burns; bleeding of major vessel). 3. Moderately severe physical damage; *medical* hospitalization and likely intensive care required (e.g., comatose with reflexes intact; third-degree burns less than 20% of body; extensive blood loss but can recover; major fractures). 4. Severe physical damage; *medical* hospitalization with intensive care required (e.g., comatose without reflexes; third-degree burns over 20% of body; extensive blood loss with unstable vital signs; major damage to a vital area). 5. Death	*Enter Code* ☐	*Enter Code* ☐	*Enter Code* ☐
Potential Lethality: Only Answer if Actual Lethality = 0 Likely lethality of actual attempt if no medical damage (the following examples, while having no actual medical damage, had potential for very serious lethality: put gun in mouth and pulled the trigger but gun fails to fire so no medical damage; laying on train tracks with oncoming train but pulled away before run over). 0 = Behavior not likely to result in injury 1 = Behavior likely to result in injury but not likely to cause death 2 = Behavior likely to result in death despite available medical care	*Enter Code* ☐	*Enter Code* ☐	*Enter Code* ☐

QSORRES when QSTESTCD = CSS0421A

QSORRES when QSTESTCD = CSS0421B

QSORRES when QSTESTCD = CSS0421C

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C-SSRS Screening/Baseline **[NOT SUBMITTED]**

Subject has reported suicidal ideation or suicidal behavior based on the Columbia Suicide Severity Rating Scale (C-SSRS). The Principal Investigator must evaluate positive responses on the C-SSRS and take appropriate action in consideration of medical judgement and the training and certification process for administering the test. It is the responsibility of the PI to review the results in all subjects and determine if any result requires evaluation by a mental health professional.

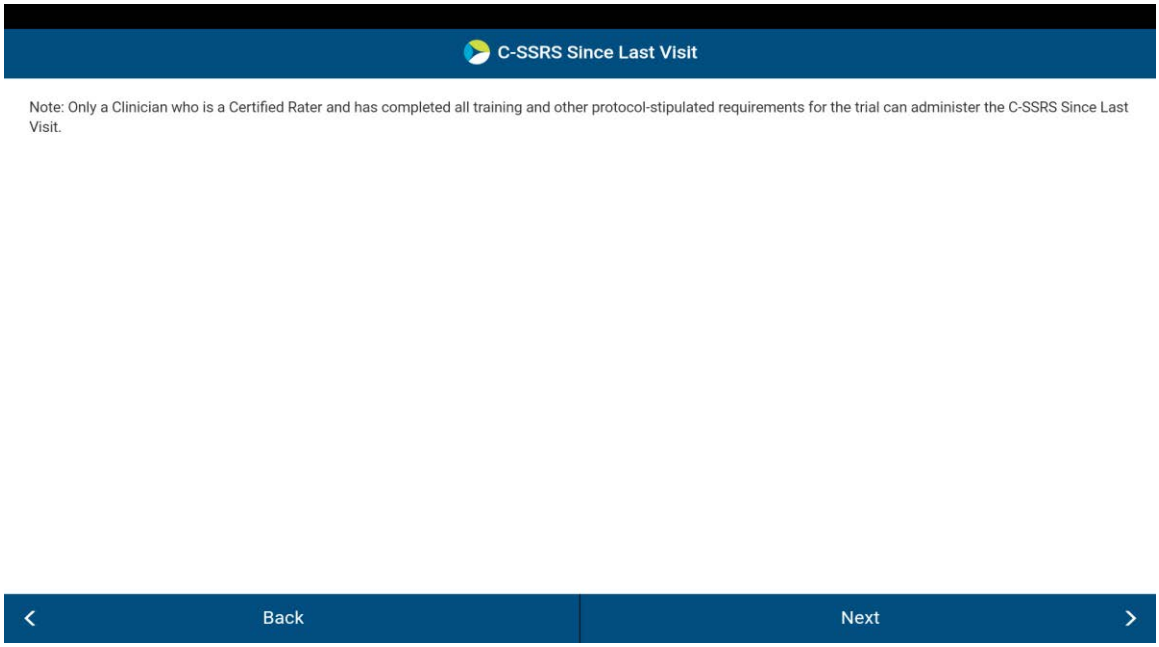
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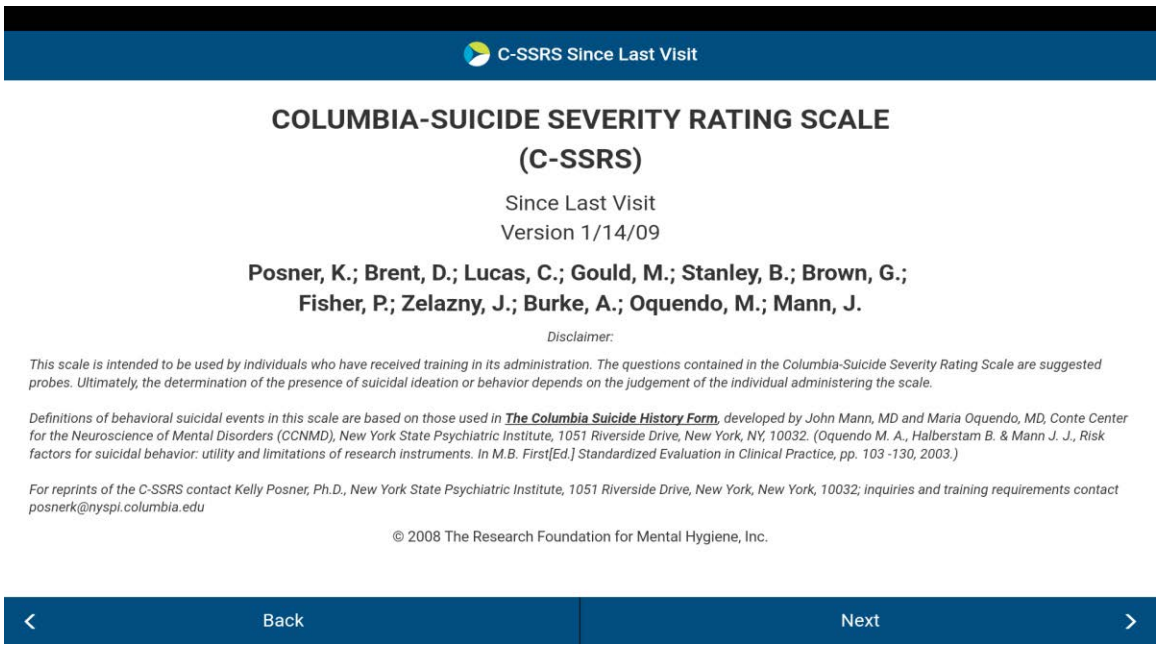
[NOT SUBMITTED]

Janssen CNTO1959UCO2002

C-SSRS Since Last Visit eCOA Tablet Screenshots

Project Information	
Assessment Name:	C-SSRS Since Last Visit
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1


C-SSRS Since Last Visit
Note: Only a Clinician who is a Certified Rater and has completed all training and other protocol-stipulated requirements for the trial can administer the C-SSRS Since Last Visit.
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C-SSRS Since Last Visit
COLUMBIA-SUICIDE SEVERITY RATING SCALE
(C-SSRS)
Since Last Visit
Version 1/14/09
Posner, K.; Brent, D.; Lucas, C.; Gould, M.; Stanley, B.; Brown, G.;
Fisher, P.; Zelazny, J.; Burke, A.; Oquendo, M.; Mann, J.
Disclaimer:
This scale is intended to be used by individuals who have received training in its administration. The questions contained in the Columbia-Suicide Severity Rating Scale are suggested probes. Ultimately, the determination of the presence of suicidal ideation or behavior depends on the judgement of the individual administering the scale.
Definitions of behavioral suicidal events in this scale are based on those used in The Columbia Suicide History Form, developed by John Mann, MD and Maria Oquendo, MD, Conte Center for the Neuroscience of Mental Disorders (CCNMD), New York State Psychiatric Institute, 1051 Riverside Drive, New York, NY, 10032. (Oquendo M. A., Halberstam B. & Mann J. J., Risk factors for suicidal behavior: utility and limitations of research instruments. In M.B. First[Ed.] Standardized Evaluation in Clinical Practice, pp. 103 -130, 2003.)
For reprints of the C-SSRS contact Kelly Posner, Ph.D., New York State Psychiatric Institute, 1051 Riverside Drive, New York, New York, 10032; inquiries and training requirements contact posnerk@nyspi.columbia.edu
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QS = Questionnaires **QSEVAL = STUDY SUBJECT**
QSCAT = C-SSRS SINCE LAST VISIT

QSSCAT = SUICIDAL IDEATION

C-SSRS Since Last Visit

SUICIDAL IDEATION	Ask questions 1 and 2. If both are negative, proceed to "Suicidal Behavior" section. If the answer to question 2 is "yes", ask questions 3, 4 and 5. If the answer to question 1 and/or 2 is "yes", complete "Intensity of Ideation" section below.	Since Last Visit	
		Yes	No
1. Wish to be Dead Subject endorses thoughts about a wish to be dead or not alive anymore, or wish to fall asleep and not wake up. ***Have you wished you were dead or wished you could go to sleep and not wake up?***			
2. Non-Specific Active Suicidal Thoughts General, non-specific thoughts of wanting to end one's life/commit suicide (e.g., *"I've thought about killing myself"*) without thoughts of ways to kill oneself/associated methods, intent, or plan during the assessment period. ***Have you actually had any thoughts of killing yourself?***			
3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act Subject endorses thoughts of suicide and has thought of at least one method during the assessment period. This is different than a specific plan with time, place or method details worked out (e.g., thought of method to kill self but not a specific plan). Includes person who would say, *"I thought about taking an overdose but I never made a specific plan as to when, where or how I would actually do it...and I would never go through with it."* ***Have you been thinking about how you might do this?***			
4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan Active suicidal thoughts of killing oneself and subject reports having some intent to act on such thoughts, as opposed to *"I have the thoughts but I definitely will not do anything about them."* ***Have you had these thoughts and had some intention of acting on them?***			
5. Active Suicidal Ideation with Specific Plan and Intent Thoughts of killing oneself with details of plan fully or partially worked out and subject has some intent to carry it out. ***Have you started to work out or worked out the details of how to kill yourself? Do you intend to carry out this plan?***			

QSORRES when QSTESTCD = CSS0201
QSORRES when QSTESTCD = CSS0202
QSORRES when QSTESTCD = CSS0203
QSORRES when QSTESTCD = CSS0204
QSORRES when QSTESTCD = CSS0205

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QSSCAT = INTENSITY OF IDEATION

C-SSRS Since Last Visit

INTENSITY OF IDEATION		Since Last Visit
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe).		
Most Severe Ideation: ☐ *Type #(1-5)* *Description of Ideation* **1. Wish to be Dead**		Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day		☐
Duration ***When you have the thoughts, how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time	(4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty	(4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you	(4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain	(4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐

QSORRES when QSTESTCD = CSS0206
QSORRES when QSTESTCD = CSS0207
QSORRES when QSTESTCD = CSS0208
QSORRES when QSTESTCD = CSS0209
QSORRES when QSTESTCD = CSS0210
QSORRES when QSTESTCD = CSS0211

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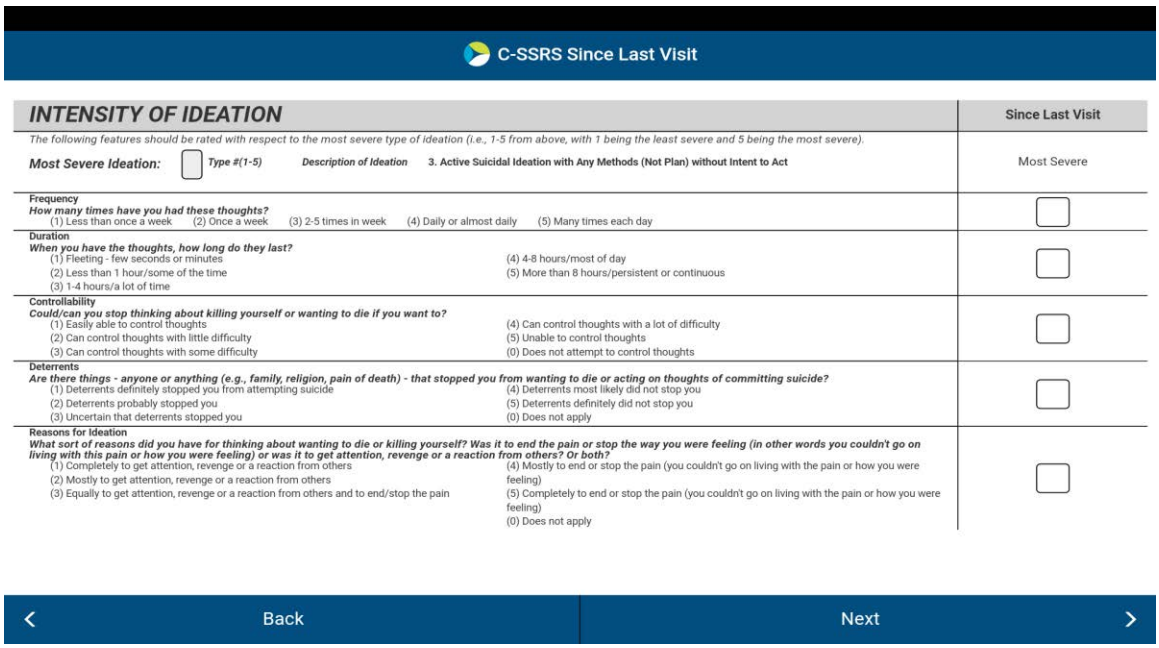
For annotations, refer to CRF page 10



C-SSRS Since Last Visit

INTENSITY OF IDEATION	Since Last Visit
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe).	
Most Severe Ideation: ☐ *Type #(1-5)* *Description of Ideation* **2. Non-Specific Active Suicidal Thoughts**	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐
Duration ***When you have the thoughts, how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐

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C-SSRS Since Last Visit

INTENSITY OF IDEATION	Since Last Visit
The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe).	
Most Severe Ideation: ☐ *Type #(1-5)* *Description of Ideation* **3. Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act**	Most Severe
Frequency ***How many times have you had these thoughts?*** (1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day	☐
Duration ***When you have the thoughts, how long do they last?*** (1) Fleeting - few seconds or minutes (2) Less than 1 hour/some of the time (3) 1-4 hours/a lot of time (4) 4-8 hours/most of day (5) More than 8 hours/persistent or continuous	☐
Controllability ***Could/can you stop thinking about killing yourself or wanting to die if you want to?*** (1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts	☐
Deterrents ***Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?*** (1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain that deterrents stopped you (4) Deterrents most likely did not stop you (5) Deterrents definitely did not stop you (0) Does not apply	☐
Reasons for Ideation ***What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?*** (1) Completely to get attention, revenge or a reaction from others (2) Mostly to get attention, revenge or a reaction from others (3) Equally to get attention, revenge or a reaction from others and to end/stop the pain (4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling) (0) Does not apply	☐

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For annotations, refer to CRF page 10



C-SSRS Since Last Visit

INTENSITY OF IDEATION | Since Last Visit

The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe).

Most Severe Ideation: ☐ *Type #(1-5)* *Description of Ideation* **4. Active Suicidal Ideation with Some Intent to Act, without Specific Plan** | Most Severe

Frequency
How many times have you had these thoughts?
(1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day | ☐

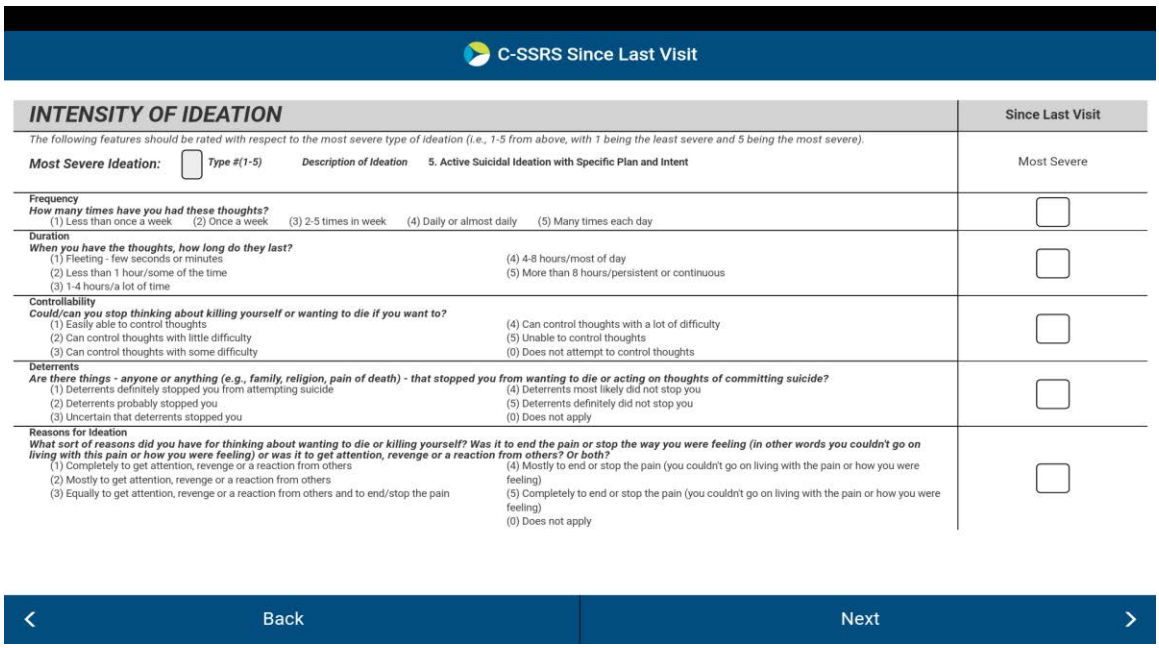
Duration
When you have the thoughts, how long do they last?
(1) Fleeting - few seconds or minutes
(2) Less than 1 hour/some of the time
(3) 1-4 hours/a lot of time
(4) 4-8 hours/most of day
(5) More than 8 hours/persistent or continuous | ☐

Controllability
Could/can you stop thinking about killing yourself or wanting to die if you want to?
(1) Easily able to control thoughts
(2) Can control thoughts with little difficulty
(3) Can control thoughts with some difficulty
(4) Can control thoughts with a lot of difficulty
(5) Unable to control thoughts
(0) Does not attempt to control thoughts | ☐

Deterrents
Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?
(1) Deterrents definitely stopped you from attempting suicide
(2) Deterrents probably stopped you
(3) Uncertain that deterrents stopped you
(4) Deterrents most likely did not stop you
(5) Deterrents definitely did not stop you
(0) Does not apply | ☐

Reasons for Ideation
What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?
(1) Completely to get attention, revenge or a reaction from others
(2) Mostly to get attention, revenge or a reaction from others
(3) Equally to get attention, revenge or a reaction from others and to end/stop the pain
(4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling)
(5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling)
(0) Does not apply | ☐

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C-SSRS Since Last Visit

INTENSITY OF IDEATION | Since Last Visit

The following features should be rated with respect to the most severe type of ideation (i.e., 1-5 from above, with 1 being the least severe and 5 being the most severe).

Most Severe Ideation: ☐ *Type #(1-5)* *Description of Ideation* **5. Active Suicidal Ideation with Specific Plan and Intent** | Most Severe

Frequency
How many times have you had these thoughts?
(1) Less than once a week (2) Once a week (3) 2-5 times in week (4) Daily or almost daily (5) Many times each day | ☐

Duration
When you have the thoughts, how long do they last?
(1) Fleeting - few seconds or minutes
(2) Less than 1 hour/some of the time
(3) 1-4 hours/a lot of time
(4) 4-8 hours/most of day
(5) More than 8 hours/persistent or continuous | ☐

Controllability
Could/can you stop thinking about killing yourself or wanting to die if you want to?
(1) Easily able to control thoughts
(2) Can control thoughts with little difficulty
(3) Can control thoughts with some difficulty
(4) Can control thoughts with a lot of difficulty
(5) Unable to control thoughts
(0) Does not attempt to control thoughts | ☐

Deterrents
Are there things - anyone or anything (e.g., family, religion, pain of death) - that stopped you from wanting to die or acting on thoughts of committing suicide?
(1) Deterrents definitely stopped you from attempting suicide
(2) Deterrents probably stopped you
(3) Uncertain that deterrents stopped you
(4) Deterrents most likely did not stop you
(5) Deterrents definitely did not stop you
(0) Does not apply | ☐

Reasons for Ideation
What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop the way you were feeling (in other words you couldn't go on living with this pain or how you were feeling) or was it to get attention, revenge or a reaction from others? Or both?
(1) Completely to get attention, revenge or a reaction from others
(2) Mostly to get attention, revenge or a reaction from others
(3) Equally to get attention, revenge or a reaction from others and to end/stop the pain
(4) Mostly to end or stop the pain (you couldn't go on living with the pain or how you were feeling)
(5) Completely to end or stop the pain (you couldn't go on living with the pain or how you were feeling)
(0) Does not apply | ☐

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QS = Questionnaires **QSEVAL = STUDY SUBJECT**
QSCAT = C-SSRS SINCE LAST VISIT

QSSCAT = SUICIDAL BEHAVIOR

C-SSRS Since Last Visit

SUICIDAL BEHAVIOR (Check all that apply, so long as these are separate events; must ask about all types)	Since Last Visit
Actual Attempt: A potentially self-injurious act committed with at least some wish to die, *as a result of the act*. Behavior was in part thought of as method to kill oneself. Intent does not have to be 100%. If there is ***any*** intent/desire to die associated with the act, then it can be considered an actual suicide attempt. ***There does not have to be any injury or harm***, just the potential for injury or harm. If person pulls trigger while gun is in mouth but gun is broken so no injury results, this is considered an attempt. Inferring Intent: Even if an individual denies intent/wish to die, it may be inferred clinically from the behavior or circumstances. For example, a highly lethal act that is clearly not an accident so no other intent but suicide can be inferred (e.g., gunshot to head, jumping from window of a high floor/story). Also, if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred. ***Have you made a suicide attempt?*** ***Have you done anything to harm yourself?*** ***Have you done anything dangerous where you could have died?*** ***What did you do?*** ***Did you _____ as a way to end your life?*** ***Did you want to die (even a little) when you _____?*** ***Were you trying to end your life when you _____?*** ***Or did you think it was possible you could have died from _____?*** ***Or did you do it purely for other reasons / without ANY intention of killing yourself (like to relieve stress, feel better, get sympathy, or get something else to happen)?*** *(Self-Injurious Behavior without suicidal intent)*	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0212** Total # of Attempts **QSORRES when QSTESTCD = CSS0213**
Has subject engaged in Non-Suicidal Self-Injurious Behavior?	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0214**

QSSCAT = SUICIDAL BEHAVIOR

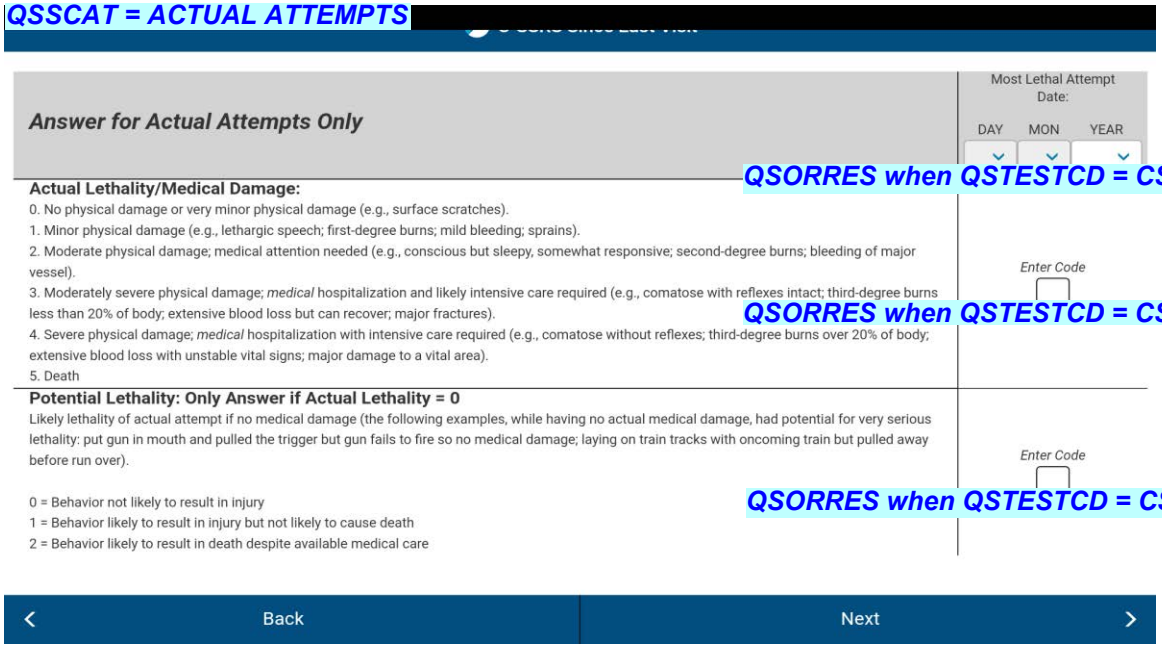
SUICIDAL BEHAVIOR (Check all that apply, so long as these are separate events; must ask about all types)	Since Last Visit
Interrupted Attempt: When the person is interrupted (by an outside circumstance) from starting the potentially self-injurious act *(if not for that, actual attempt would have occurred)*. Overdose: Person has pills in hand but is stopped from ingesting. Once they ingest any pills, this becomes an attempt rather than an interrupted attempt. Shooting: Person has gun pointed toward self, gun is taken away by someone else, or is somehow prevented from pulling trigger. Once they pull the trigger, even if the gun fails to fire, it is an attempt. Jumping: Person is poised to jump, is grabbed and taken down from ledge. Hanging: Person has noose around neck but has not yet started to hang - is stopped from doing so. ***Has there been a time when you started to do something to end your life but someone or something stopped you before you actually did anything?***	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0215** Total # of Interrupted **QSORRES when QSTESTCD = CSS0216**
Aborted Attempt: When person begins to take steps toward making a suicide attempt, but stops themselves before they actually have engaged in any self-destructive behavior. Examples are similar to interrupted attempts, except that the individual stops him/herself, instead of being stopped by something else. ***Has there been a time when you started to do something to try to end your life but you stopped yourself before you actually did anything?***	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0217** Total # of Aborted **QSORRES when QSTESTCD = CSS0218**
Preparatory Acts or Behavior: Acts or preparation towards imminently making a suicide attempt. This can include anything beyond a verbalization or thought, such as assembling a specific method (e.g., buying pills, purchasing a gun) or preparing for one's death by suicide (e.g., giving things away, writing a suicide note). ***Have you taken any steps towards making a suicide attempt or preparing to kill yourself (such as collecting pills, getting a gun, giving valuables away or writing a suicide note)?***	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0219**
Suicidal Behavior: Suicidal behavior was present during the assessment period?	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0220**
Suicide:	Yes ○ No ○ **QSORRES when QSTESTCD = CSS0221**

QS = Questionnaires

QSEVAL = STUDY SUBJECT

QSCAT = C-SSRS SINCE LAST VISIT

QSSCAT = ACTUAL ATTEMPTS

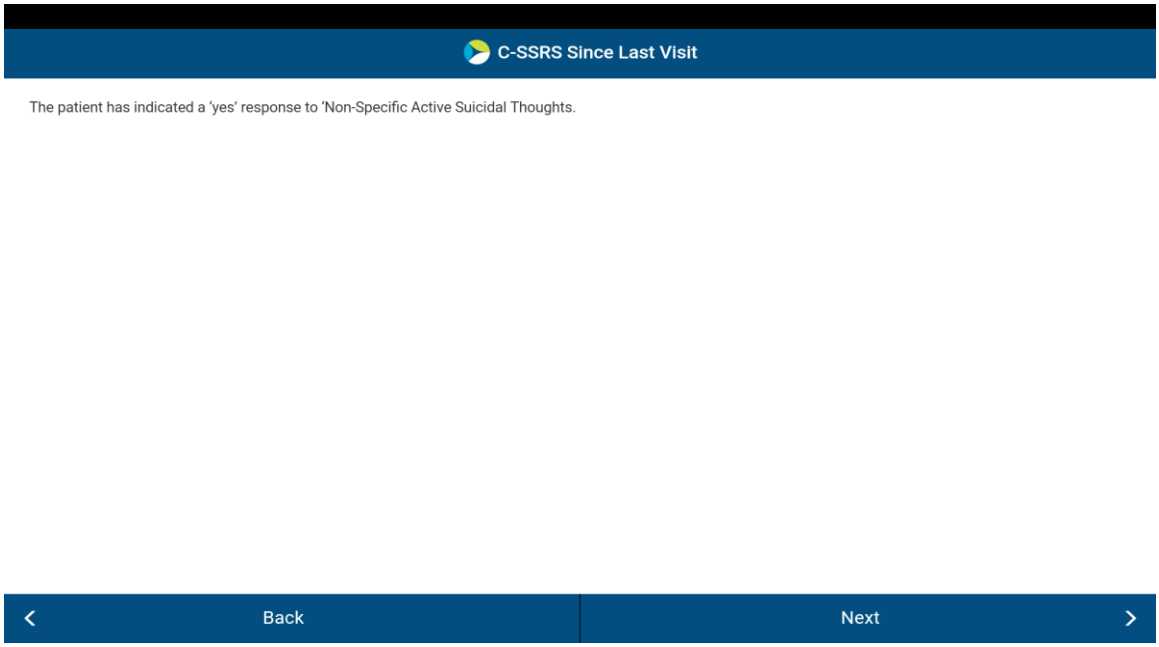


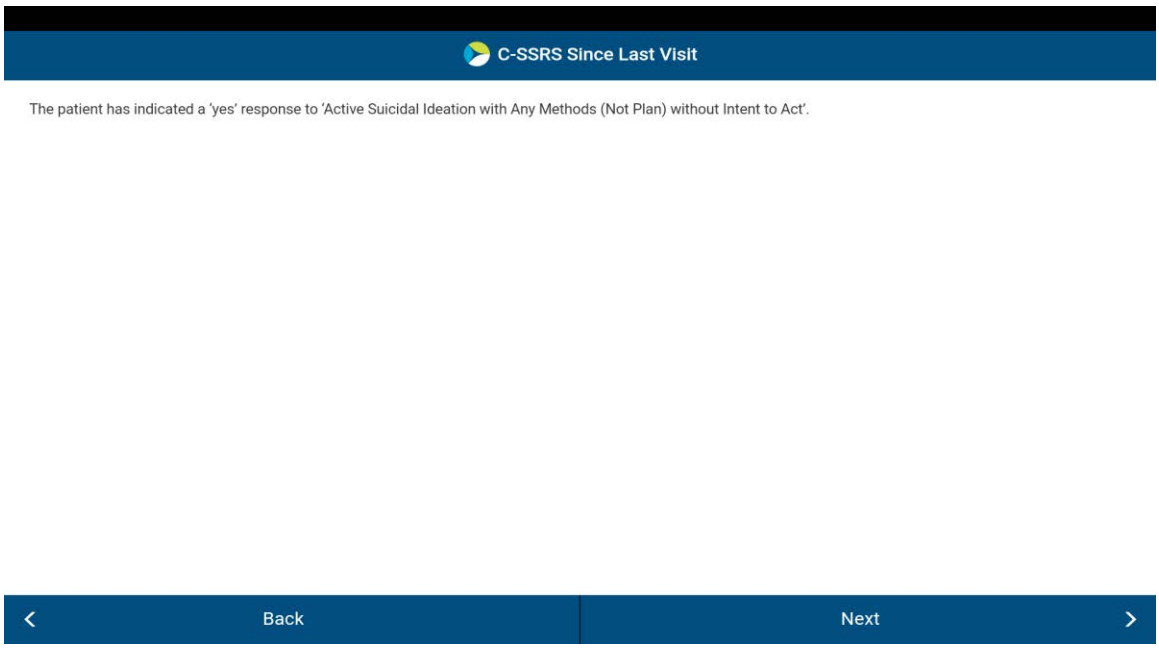
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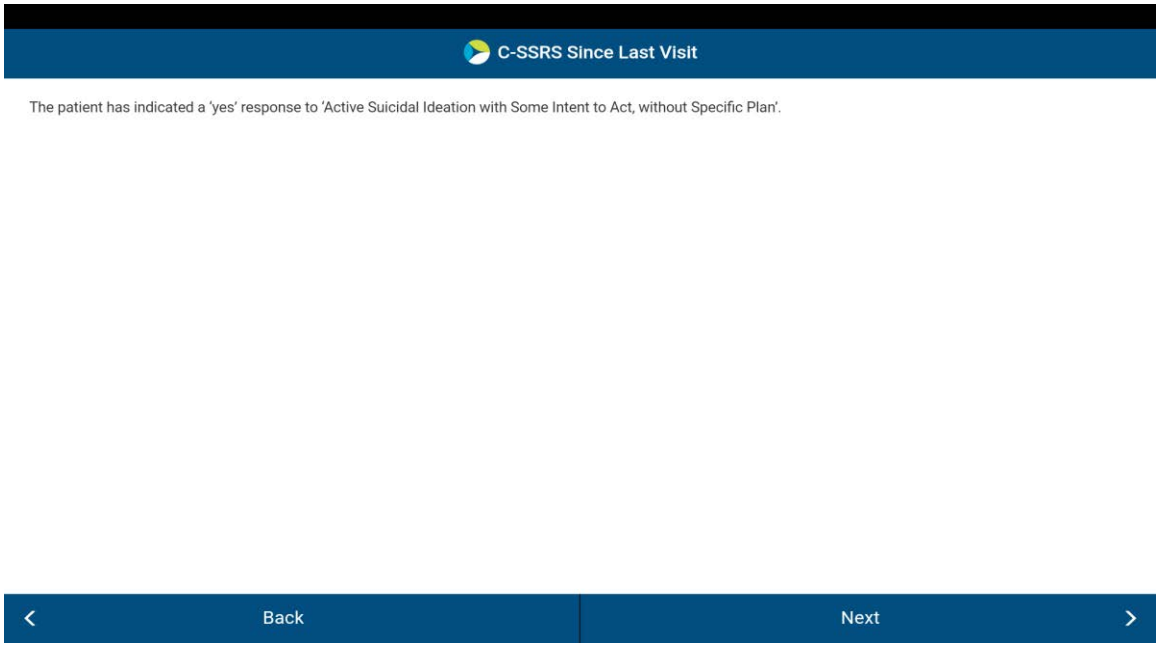
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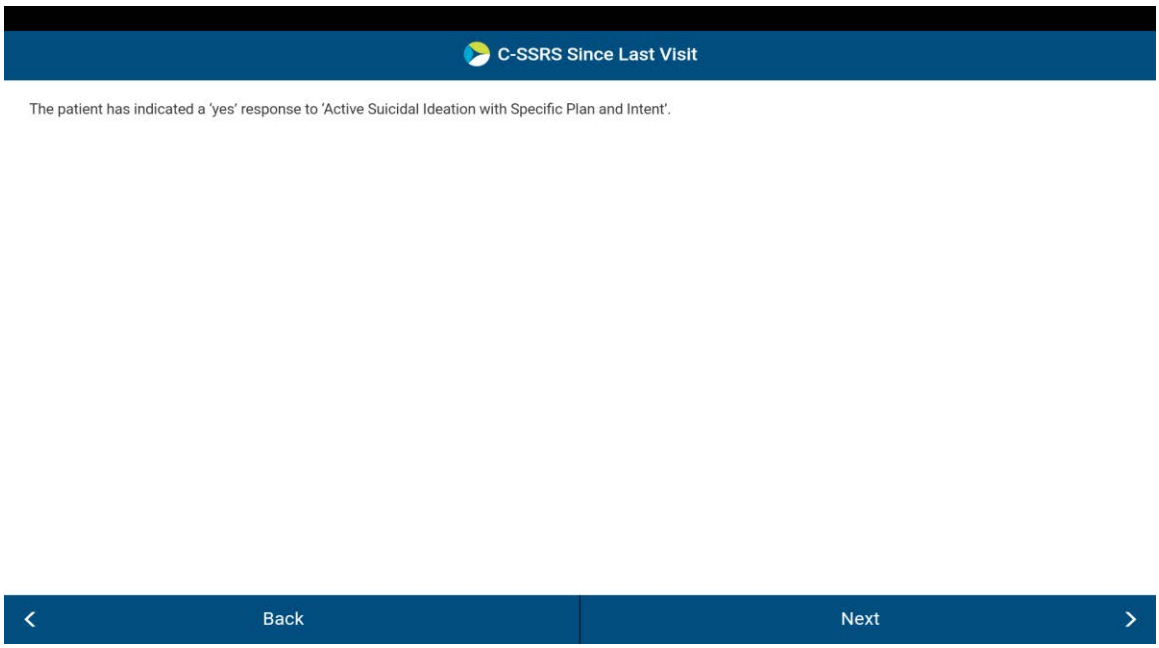
QSORRES when QSTESTCD = CSS0222C



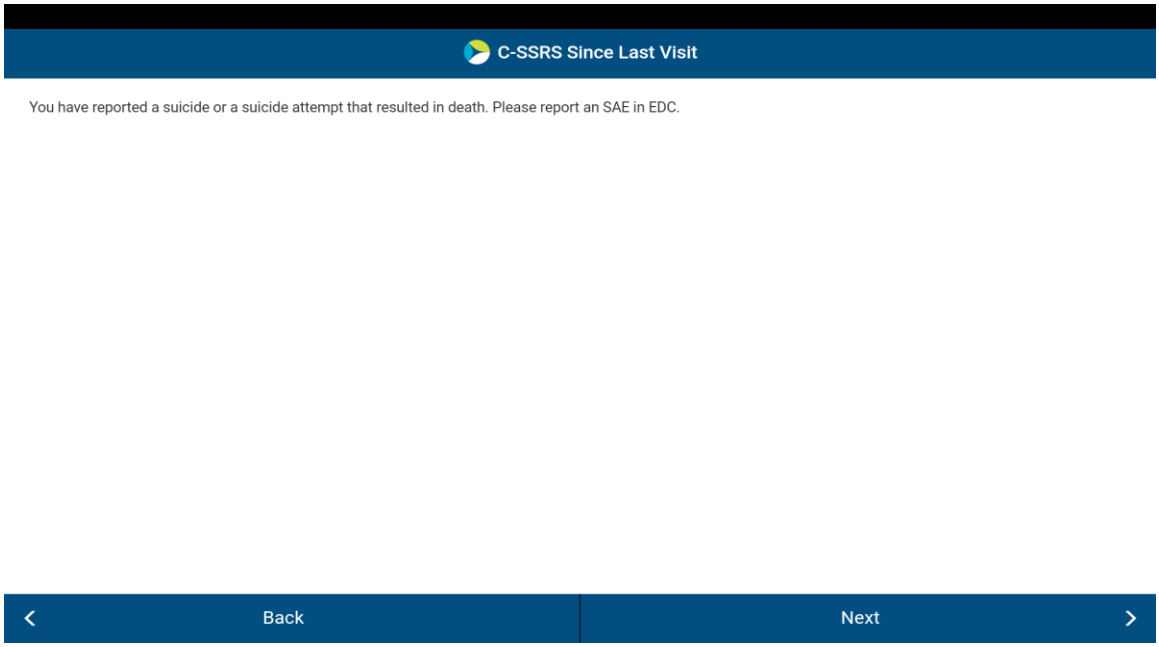

C-SSRS Since Last Visit
The patient has indicated a 'yes' response to 'Non-Specific Active Suicidal Thoughts.
Back
Next


C-SSRS Since Last Visit
The patient has indicated a 'yes' response to 'Active Suicidal Ideation with Any Methods (Not Plan) without Intent to Act'.
Back
Next


C-SSRS Since Last Visit
The patient has indicated a 'yes' response to 'Active Suicidal Ideation with Some Intent to Act, without Specific Plan'.
Back
Next


C-SSRS Since Last Visit
The patient has indicated a 'yes' response to 'Active Suicidal Ideation with Specific Plan and Intent'.
Back
Next

[NOT SUBMITTED]

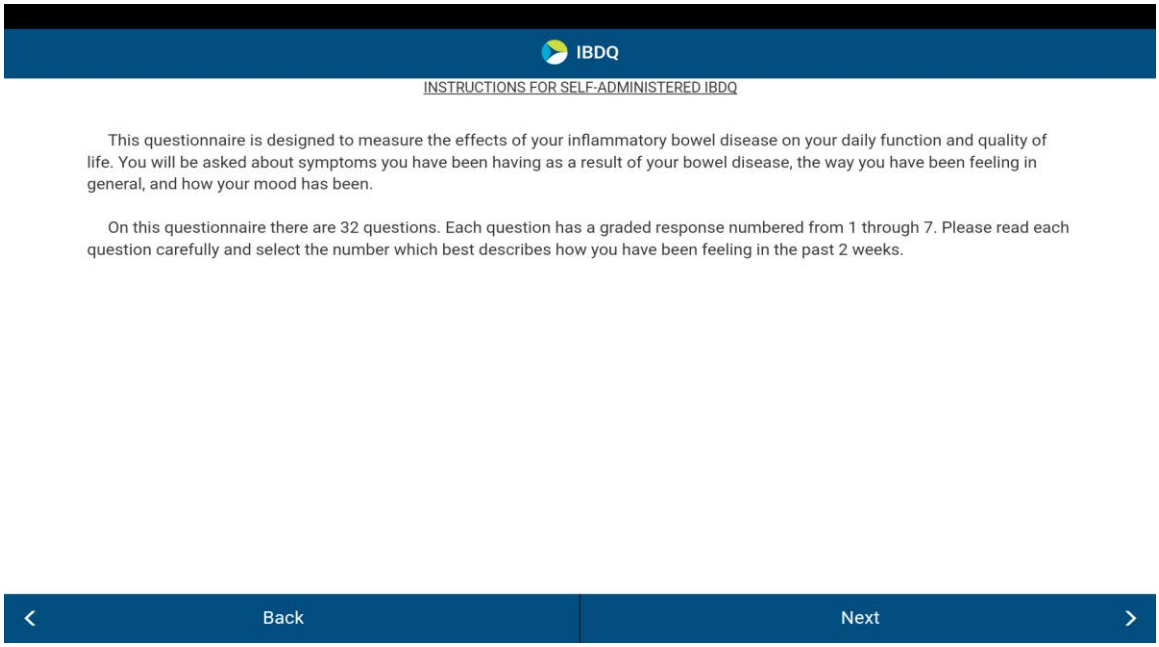


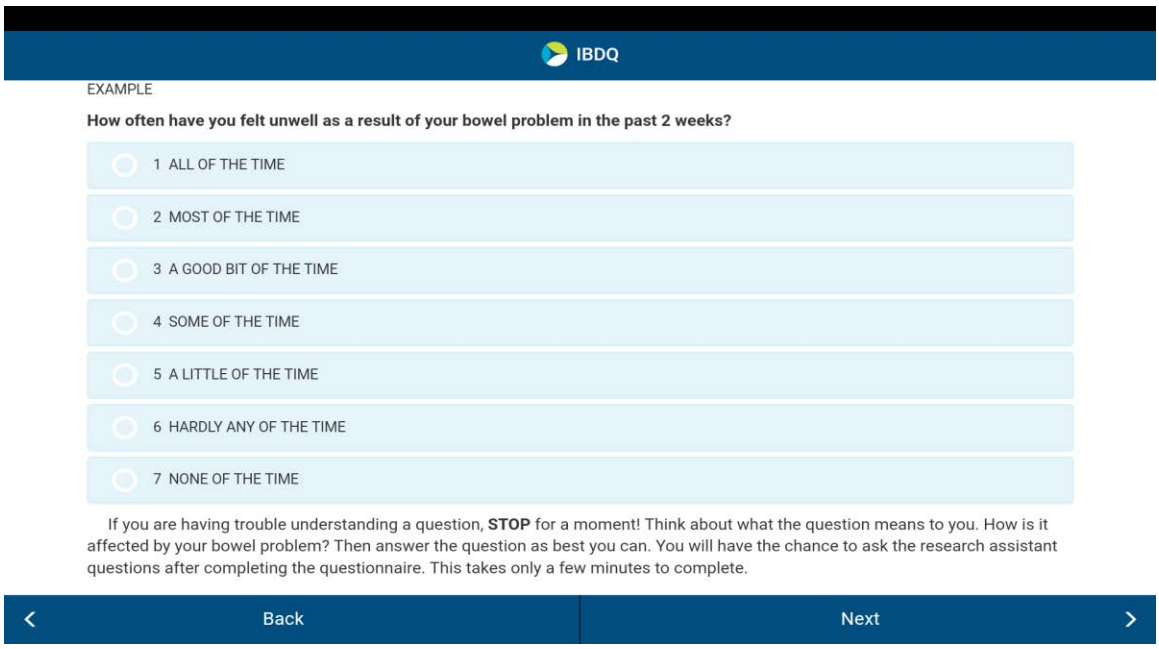
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Janssen CNTO1959UCO2002

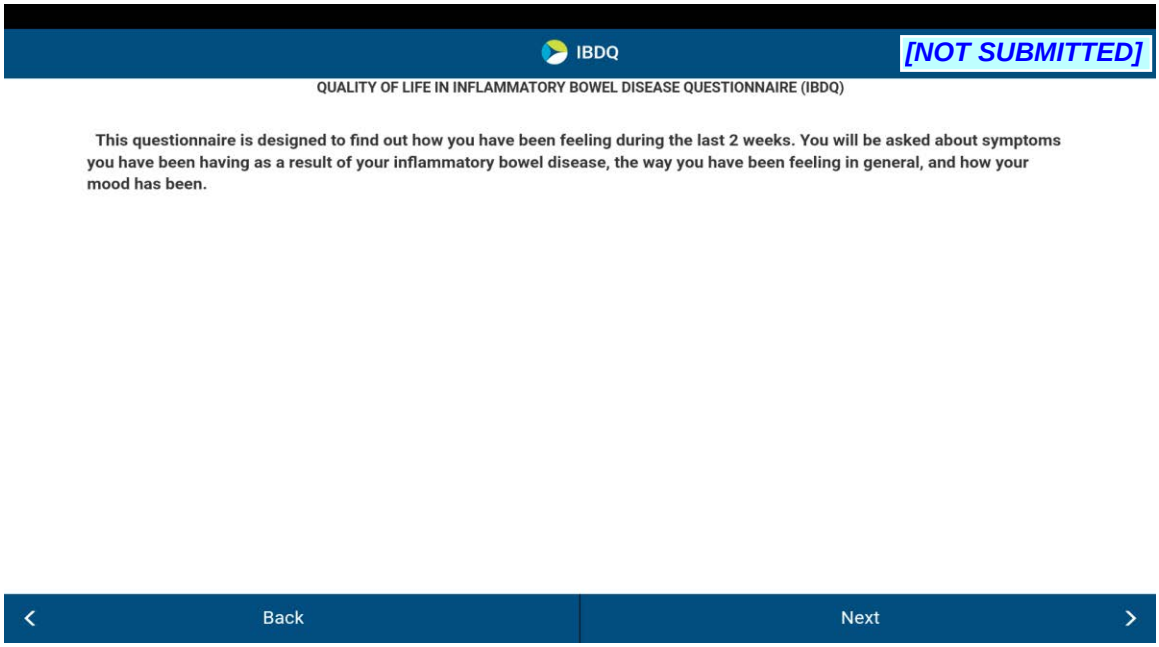
IBDQ eCOA Tablet Screenshots

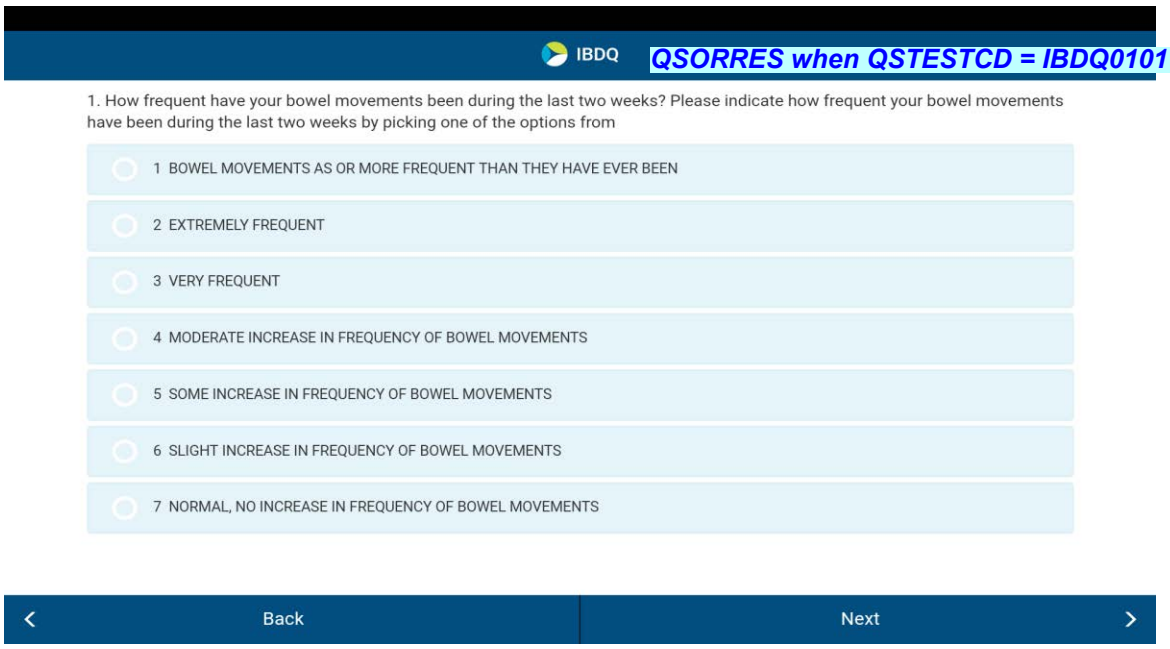
Project Information	
Assessment Name:	IBDQ
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1


IBDQ
INSTRUCTIONS FOR SELF-ADMINISTERED IBDQ
This questionnaire is designed to measure the effects of your inflammatory bowel disease on your daily function and quality of life. You will be asked about symptoms you have been having as a result of your bowel disease, the way you have been feeling in general, and how your mood has been.
On this questionnaire there are 32 questions. Each question has a graded response numbered from 1 through 7. Please read each question carefully and select the number which best describes how you have been feeling in the past 2 weeks.
Back
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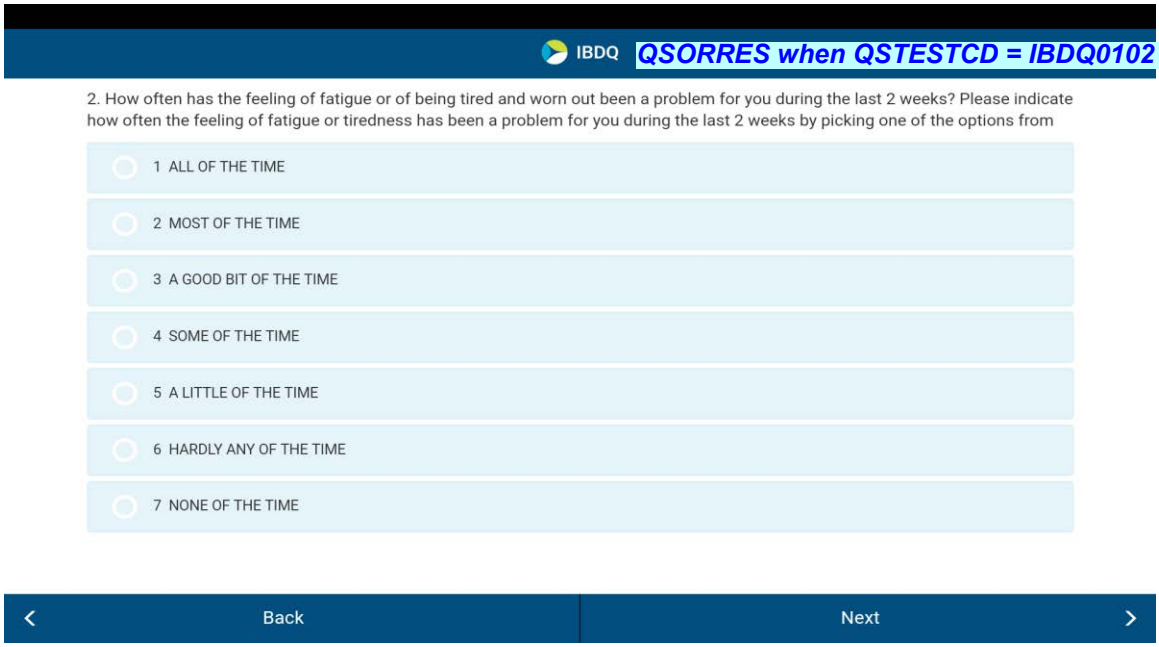

IBDQ
EXAMPLE
How often have you felt unwell as a result of your bowel problem in the past 2 weeks?
1 ALL OF THE TIME
2 MOST OF THE TIME
3 A GOOD BIT OF THE TIME
4 SOME OF THE TIME
5 A LITTLE OF THE TIME
6 HARDLY ANY OF THE TIME
7 NONE OF THE TIME
If you are having trouble understanding a question, STOP for a moment! Think about what the question means to you. How is it affected by your bowel problem? Then answer the question as best you can. You will have the chance to ask the research assistant questions after completing the questionnaire. This takes only a few minutes to complete.
Back
Next

QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**



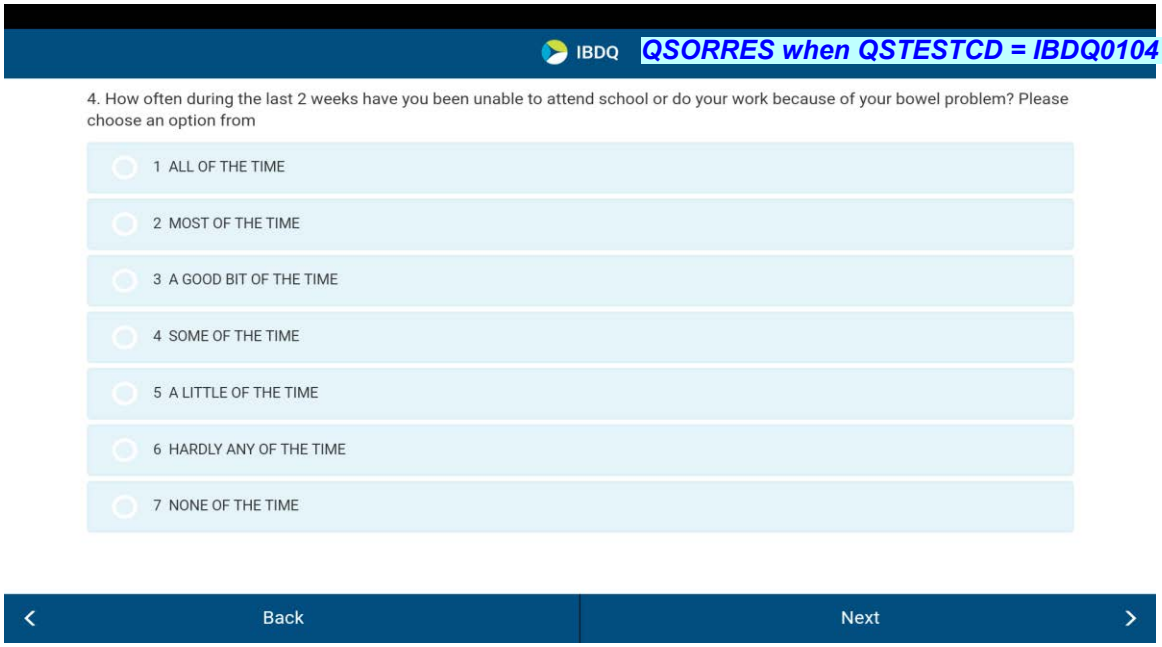


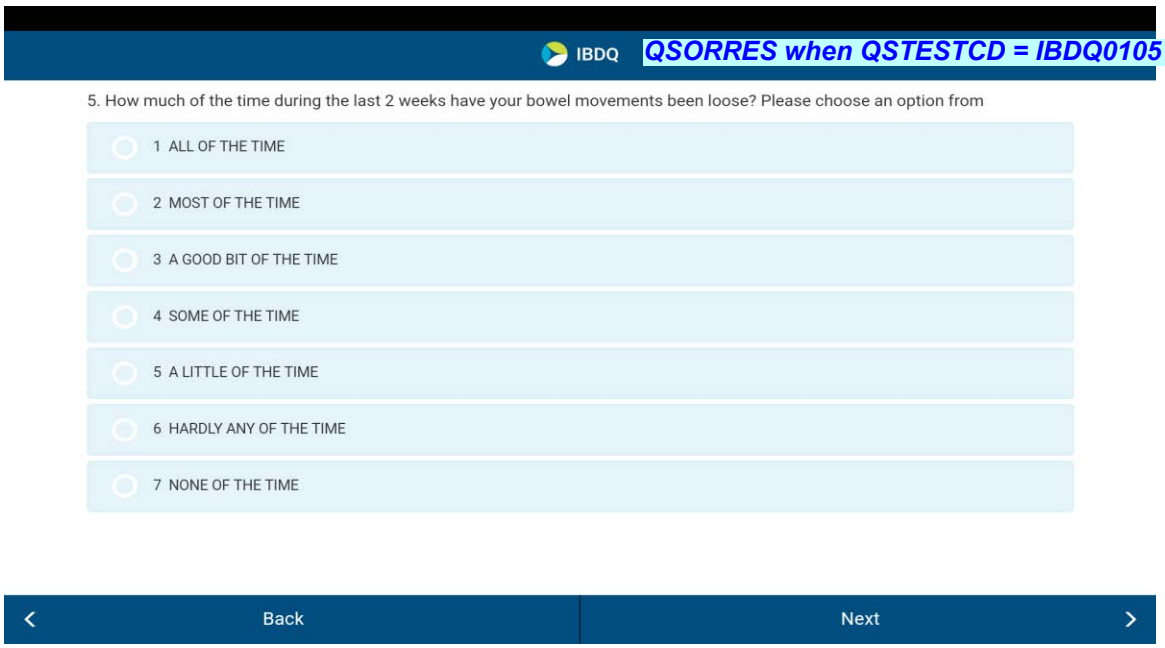
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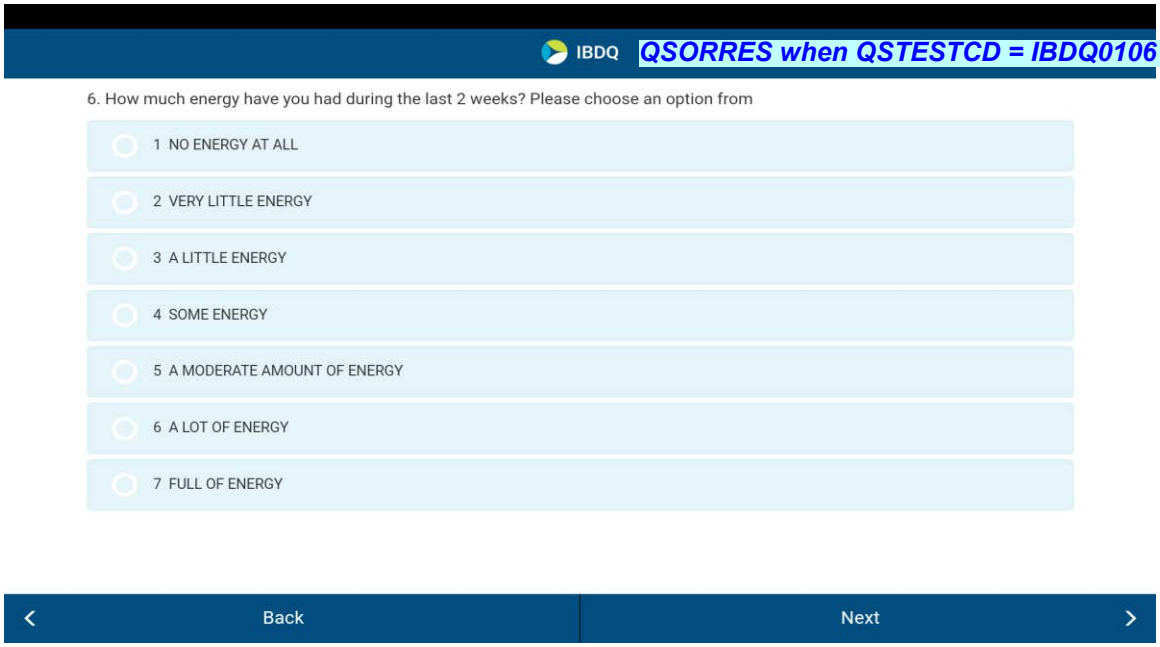


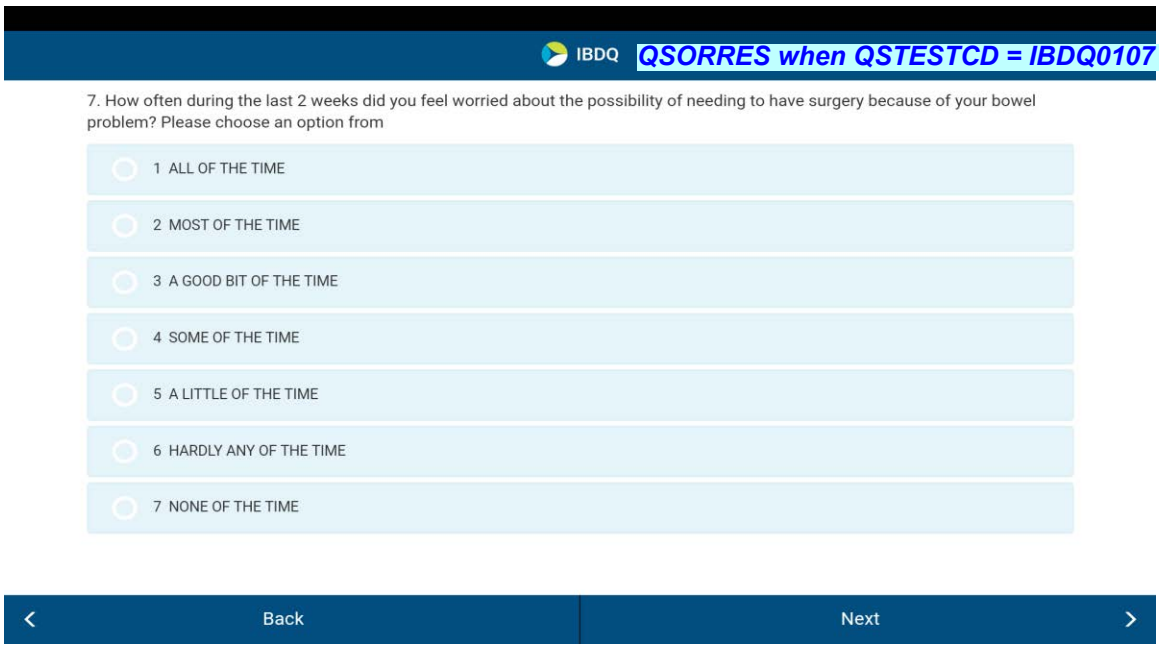
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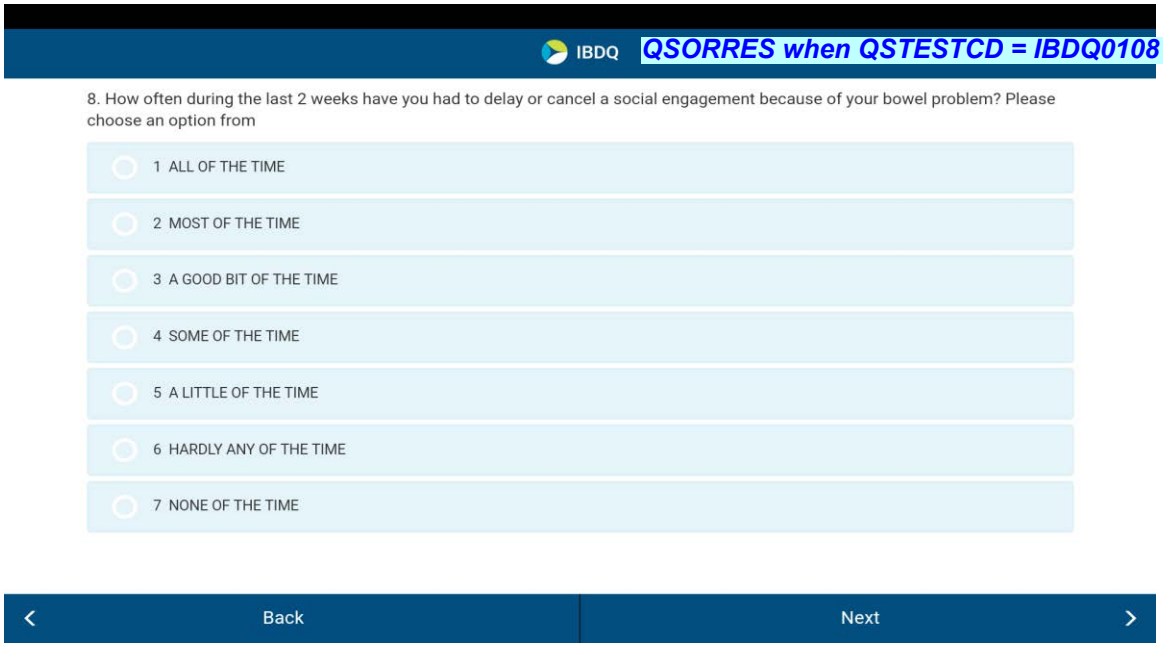


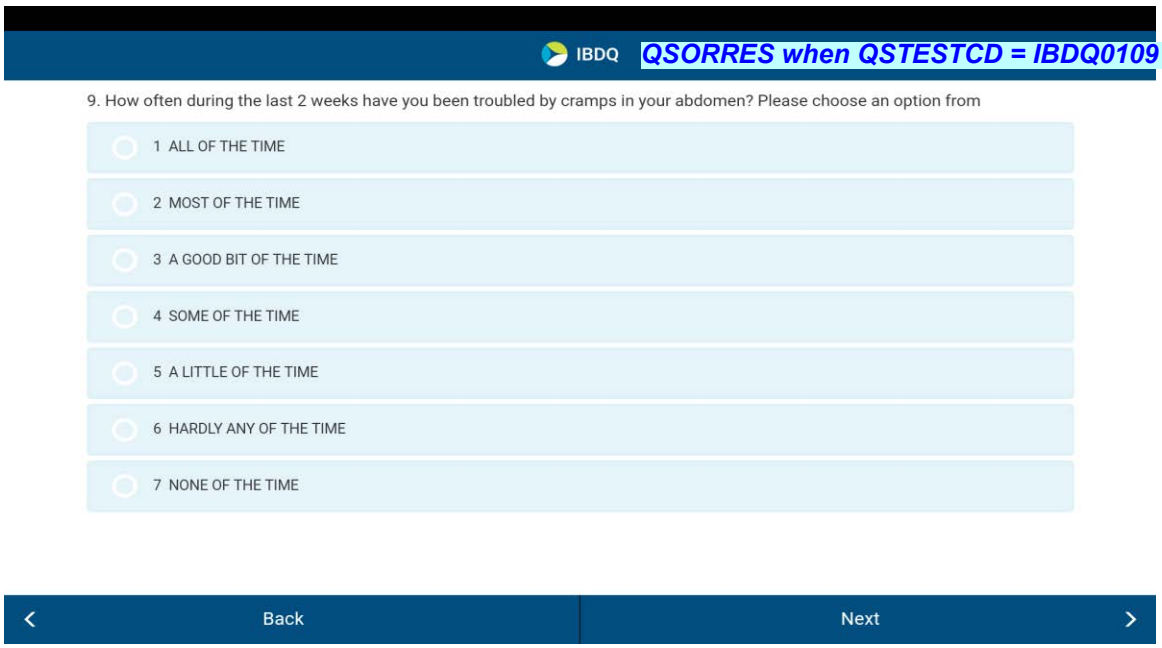
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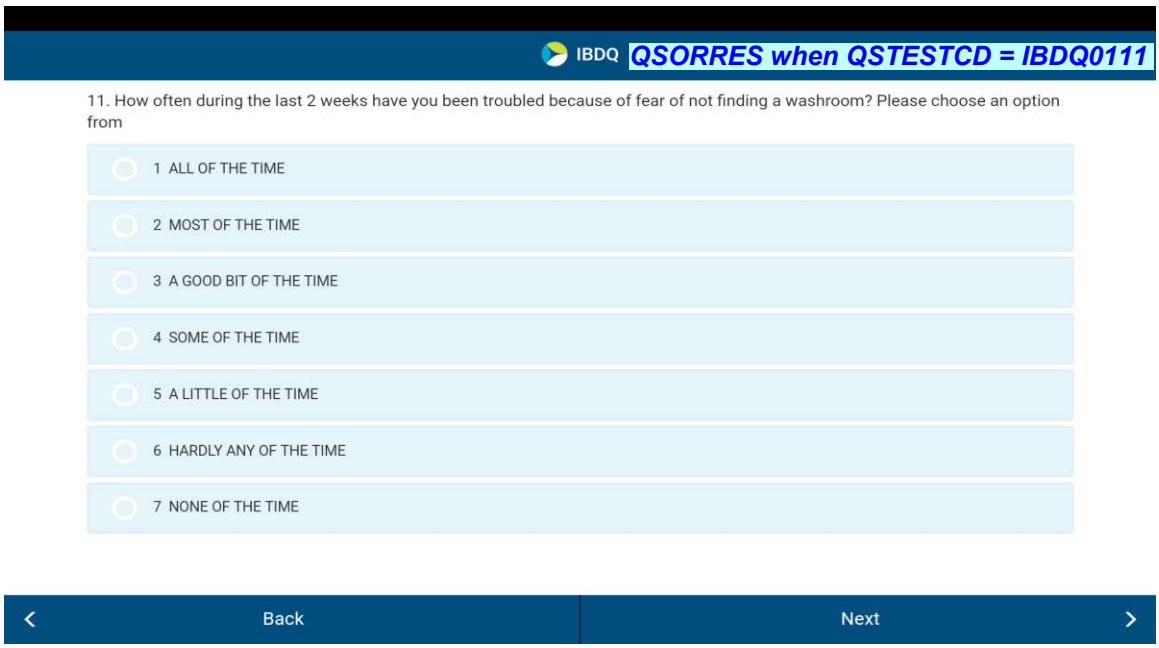
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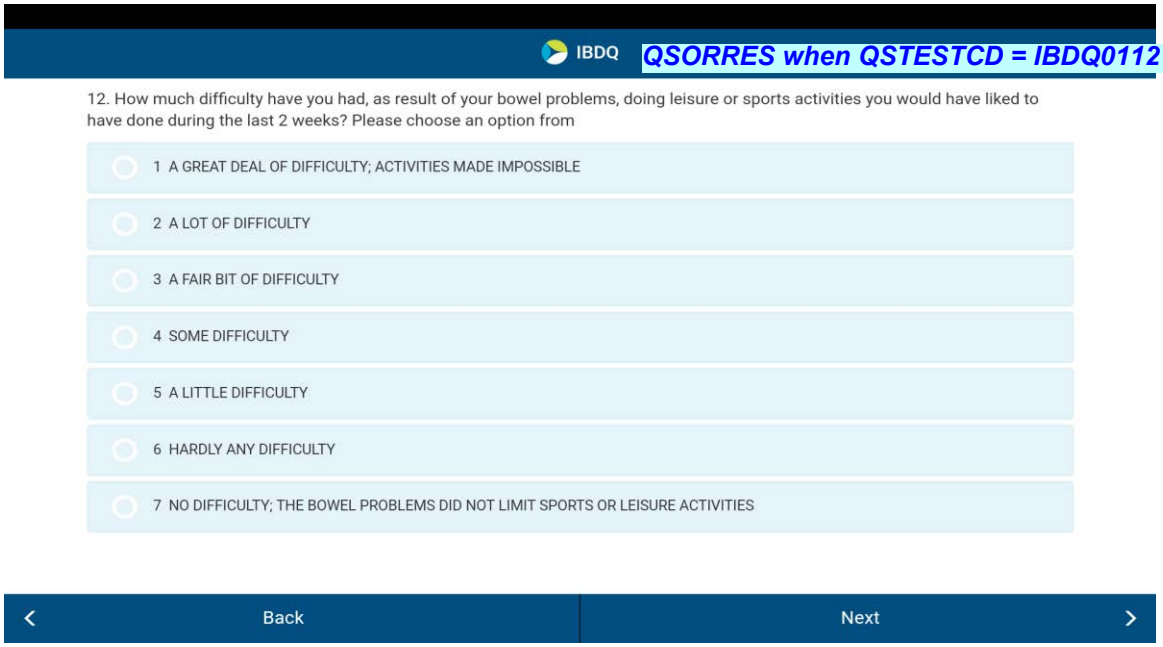


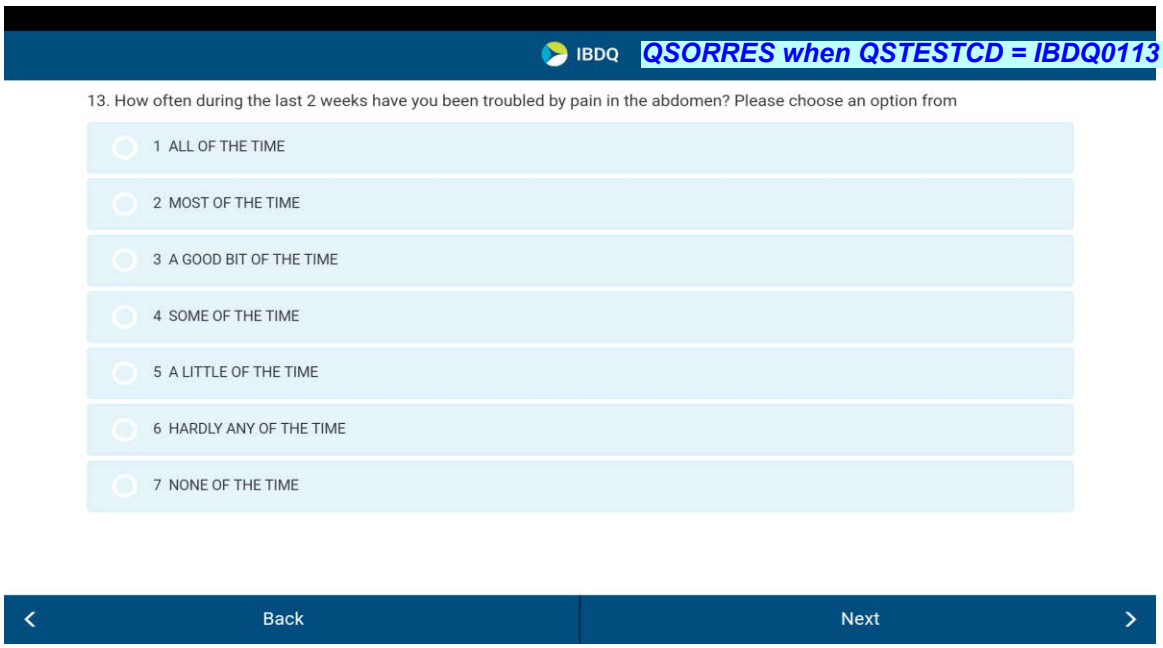
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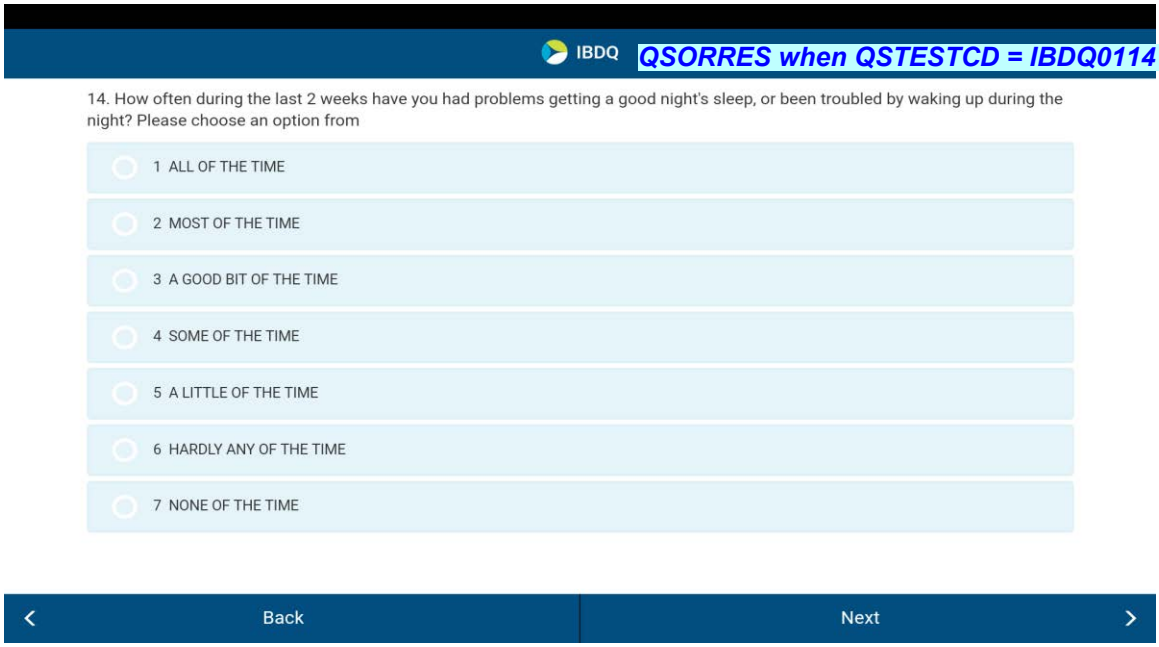


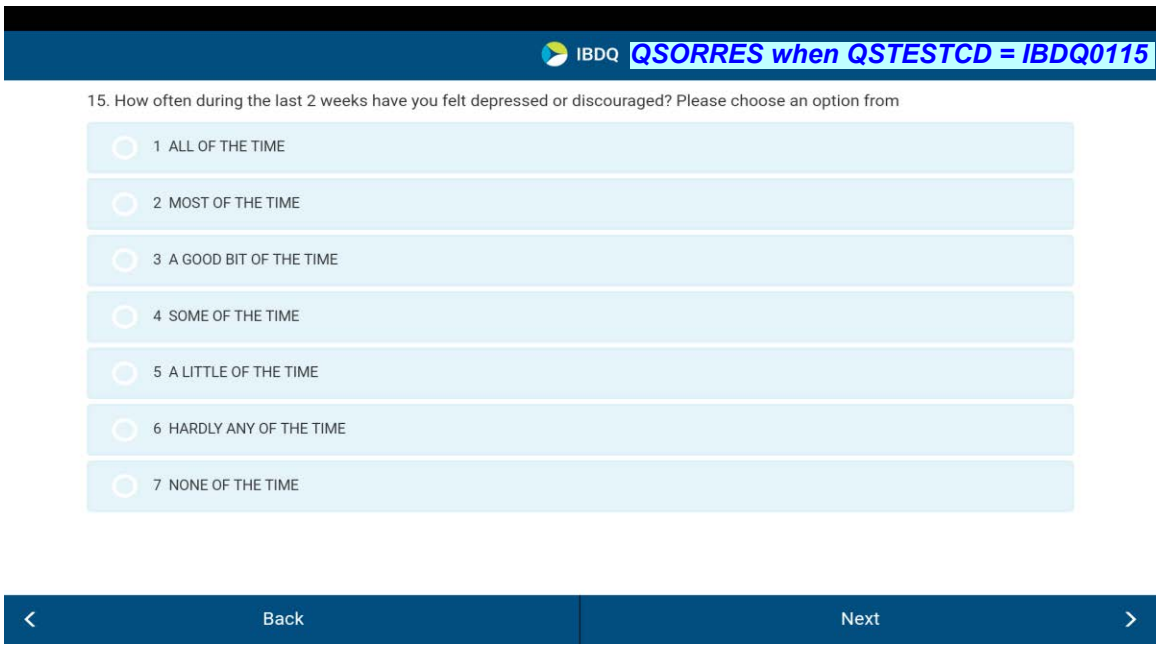
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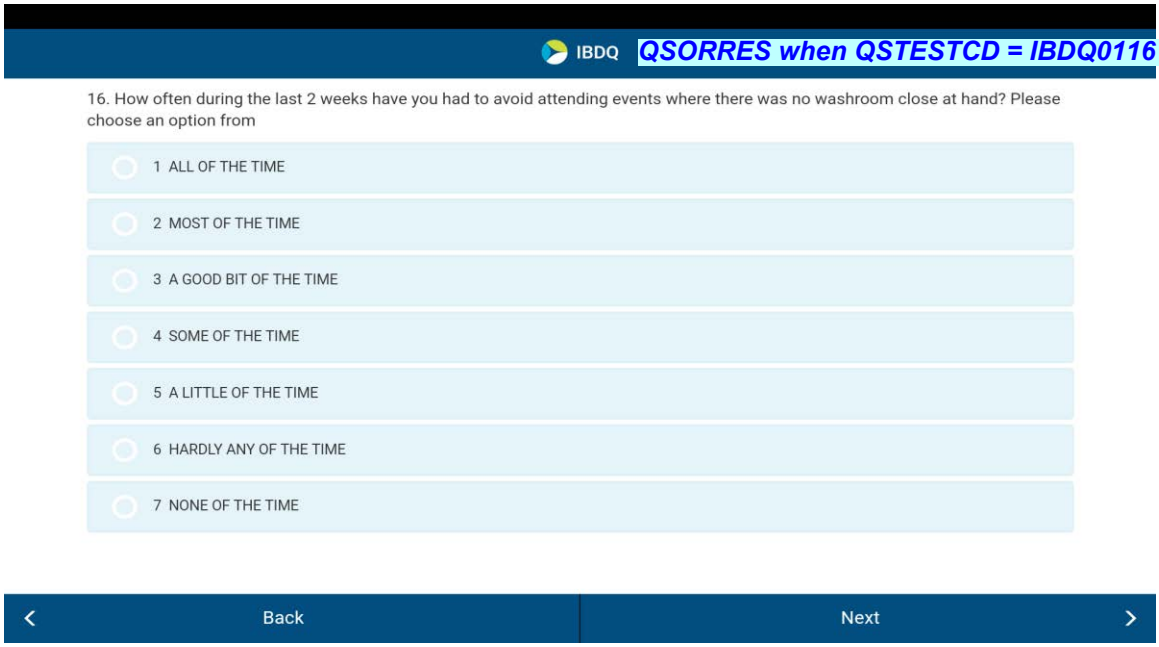


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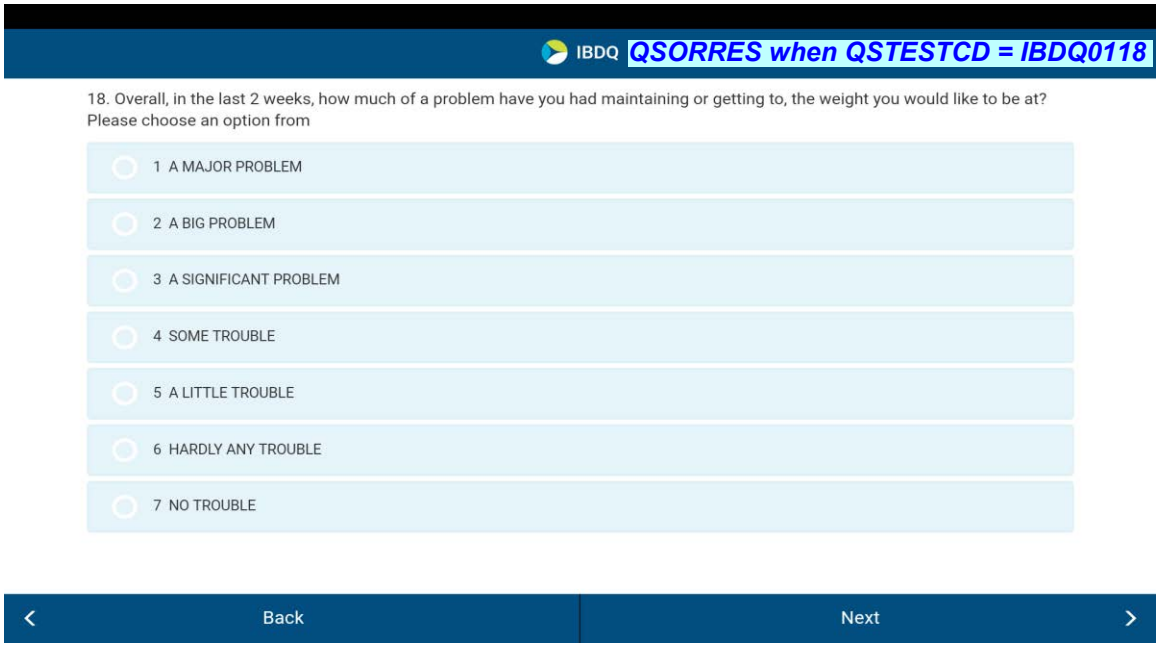


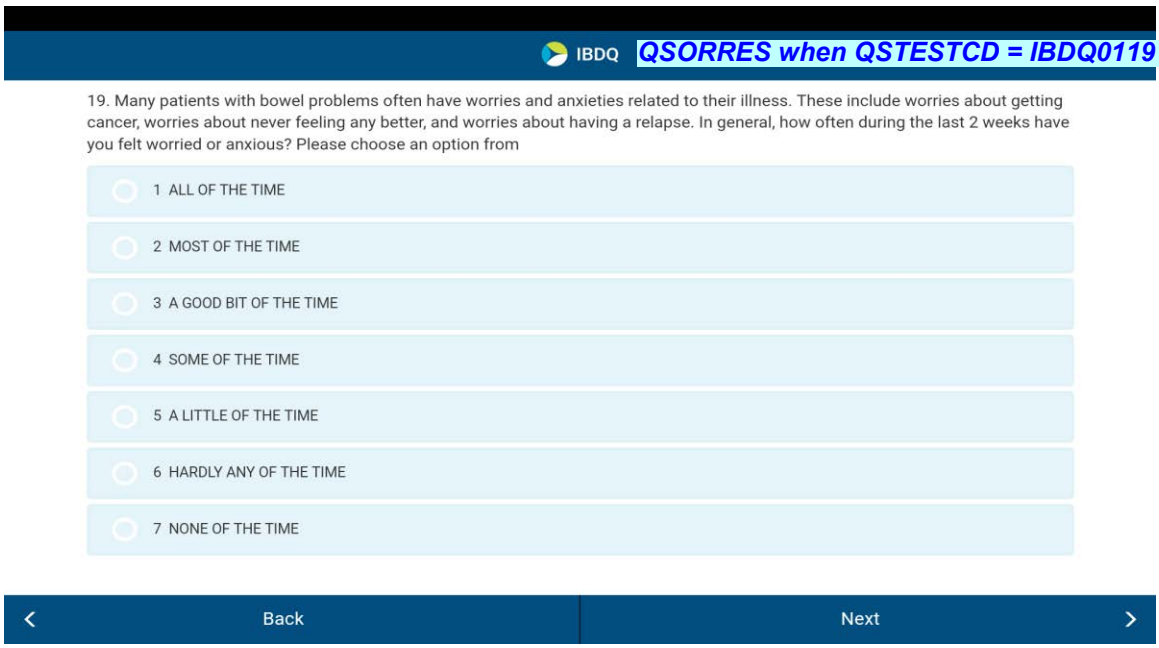
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QSCAT = IBDQ



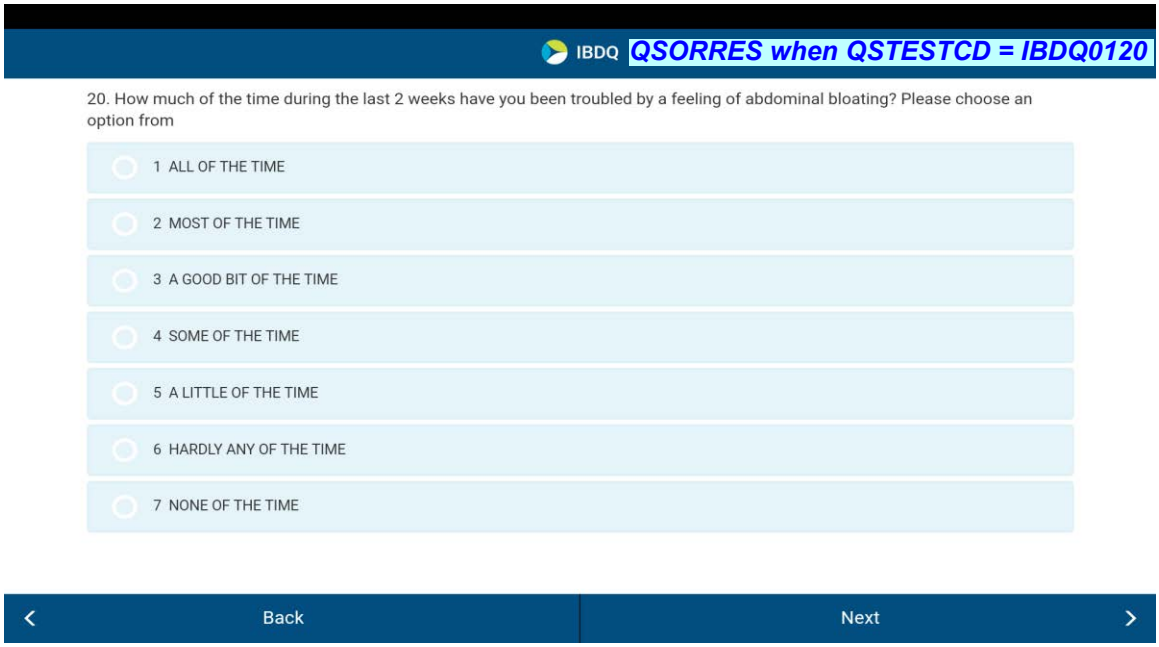


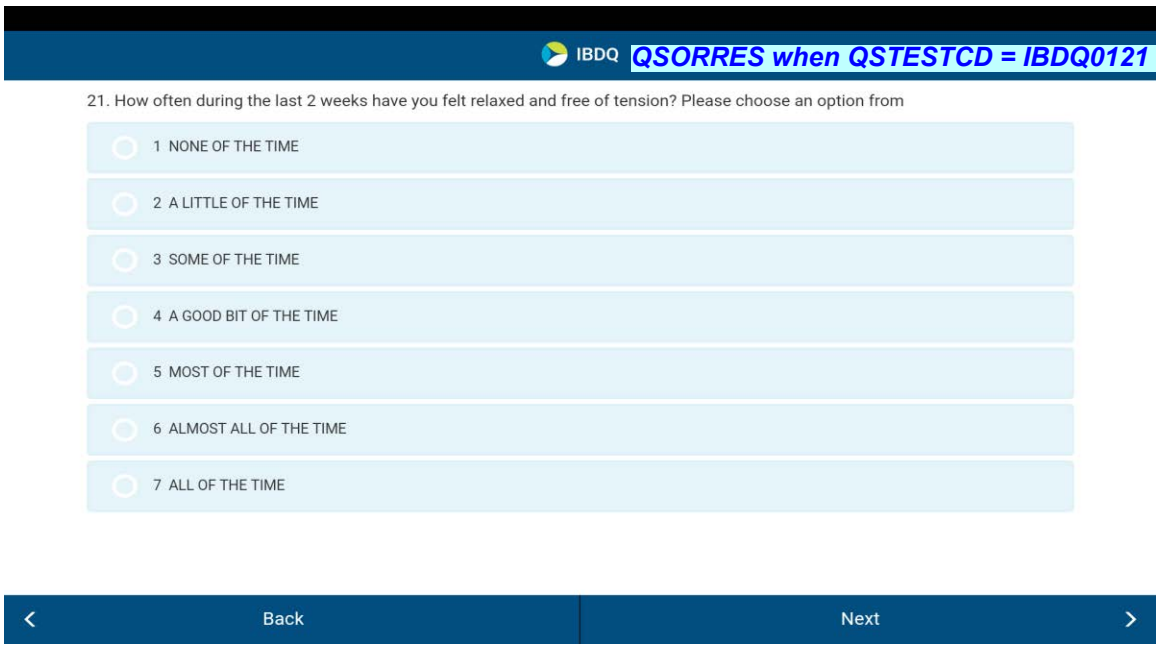
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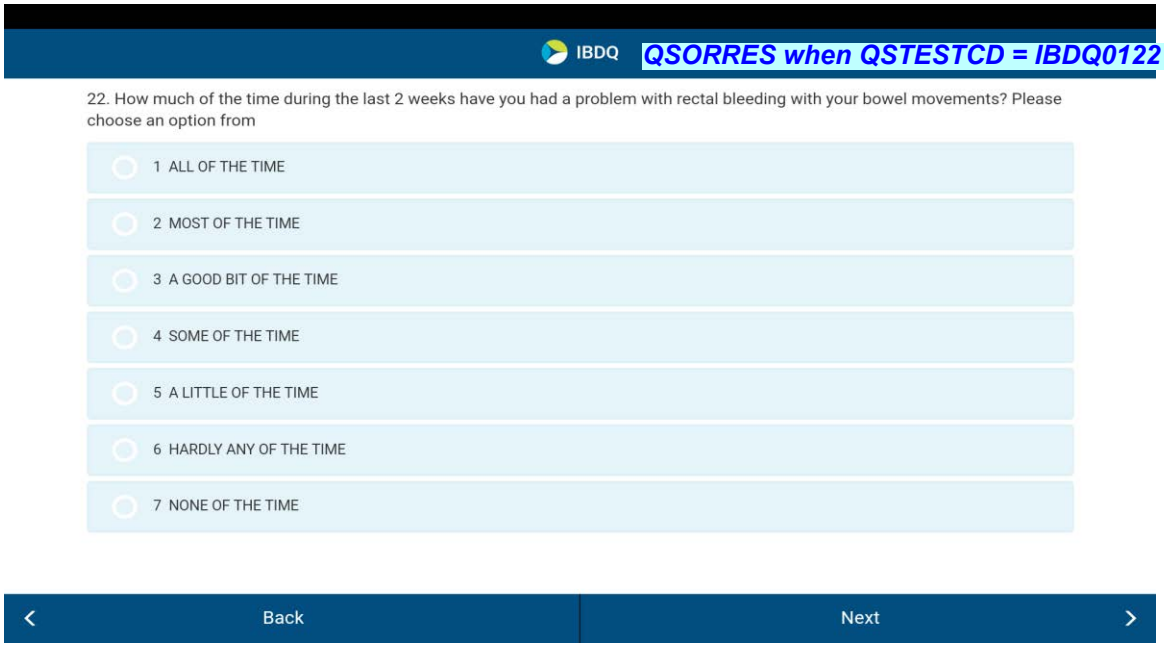


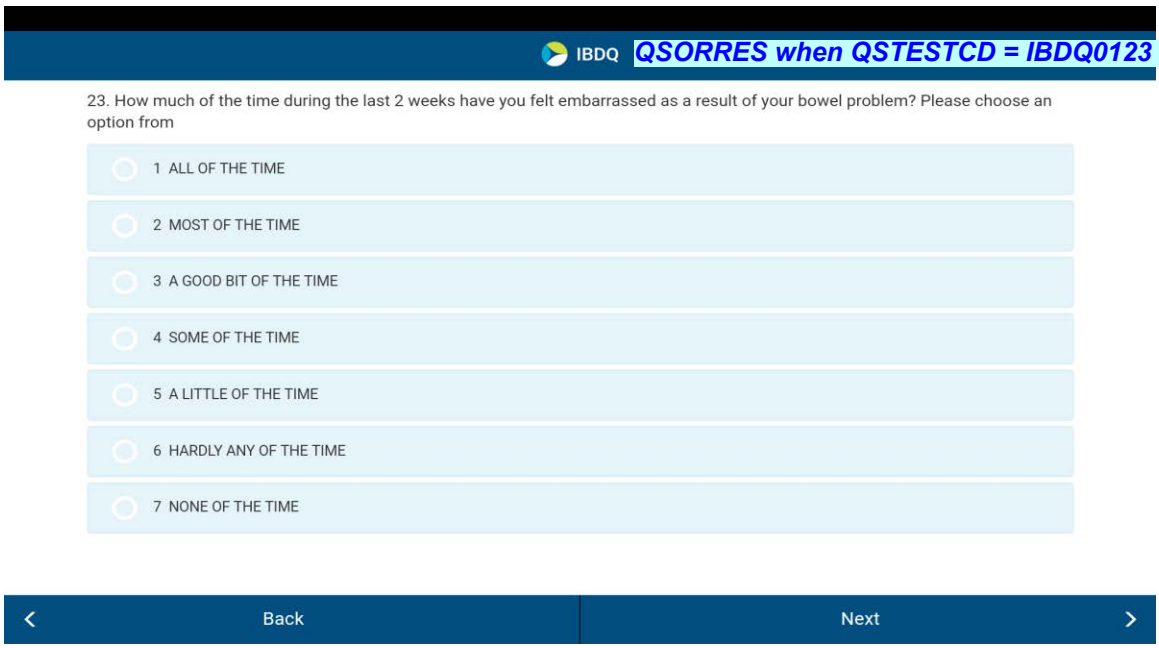
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**



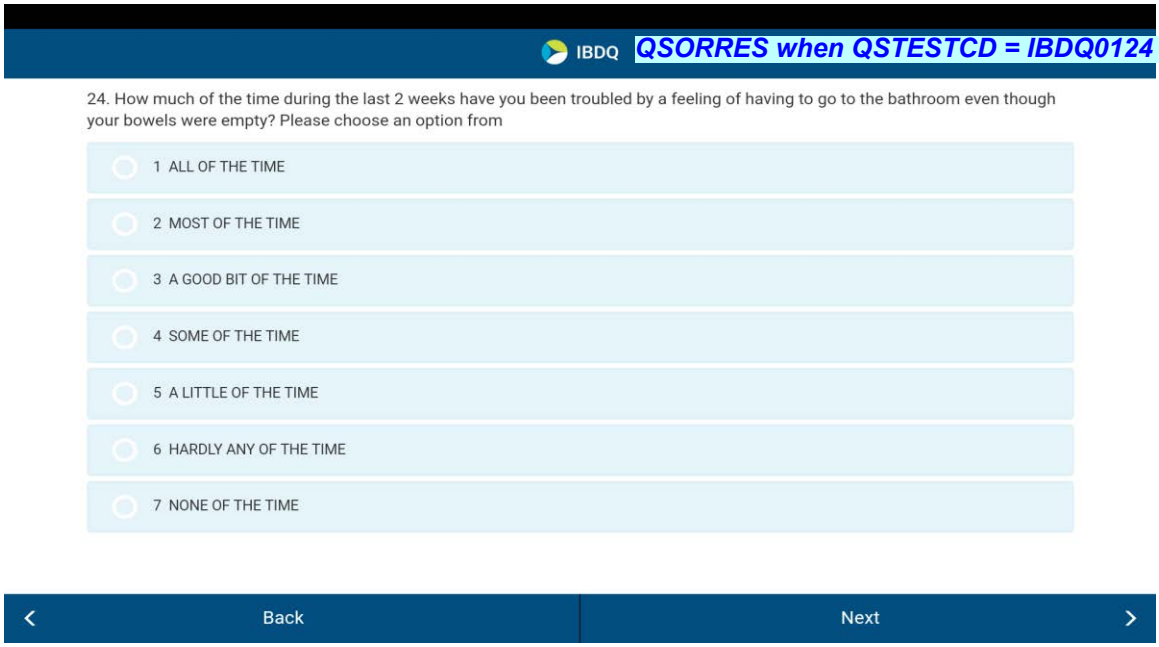


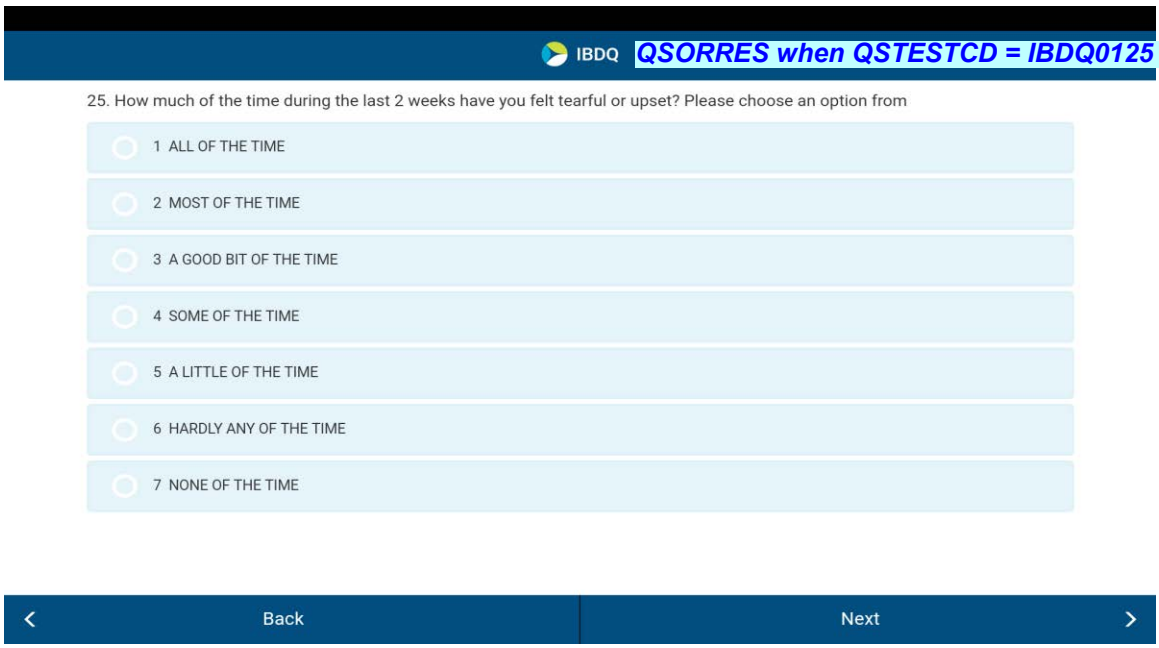
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**



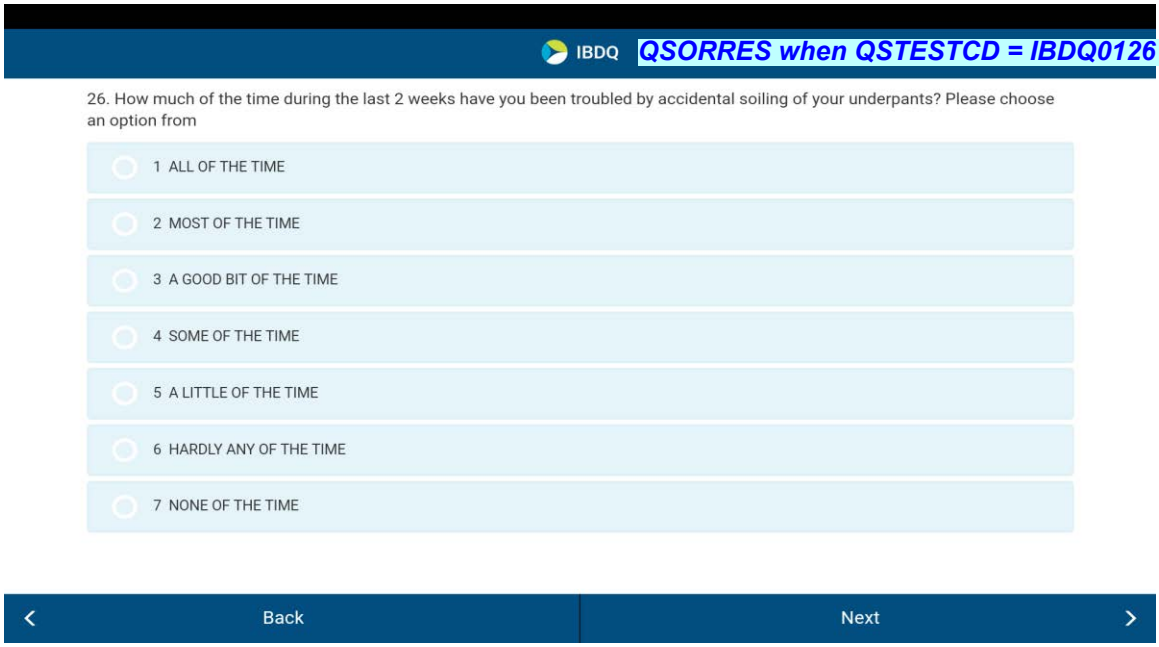


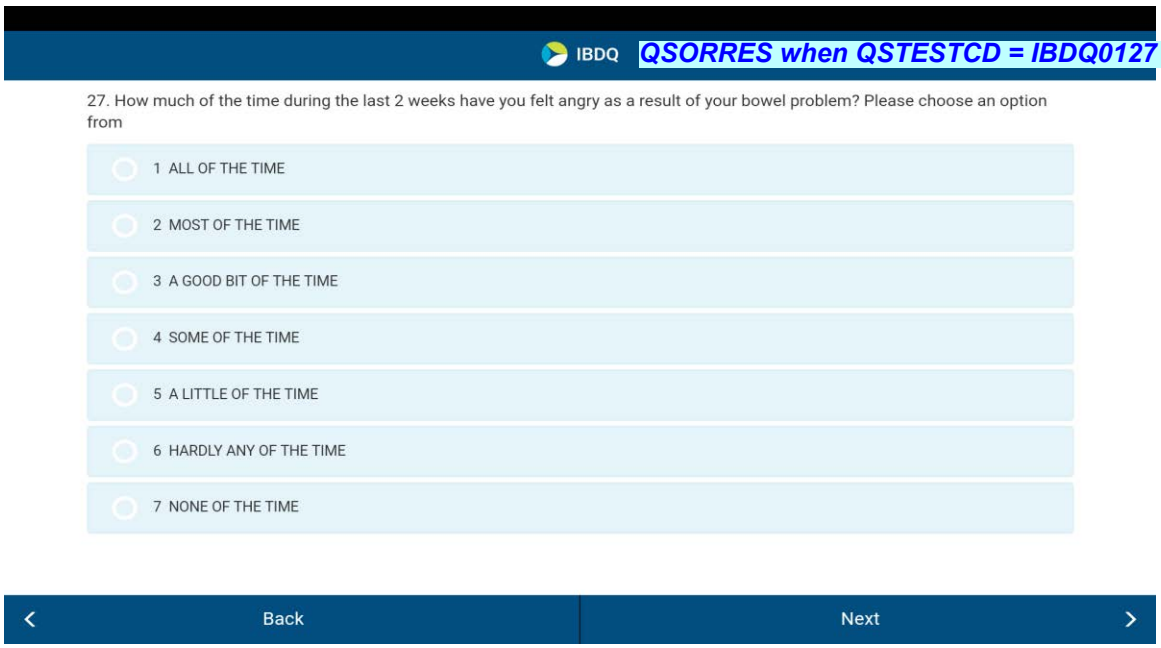
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**



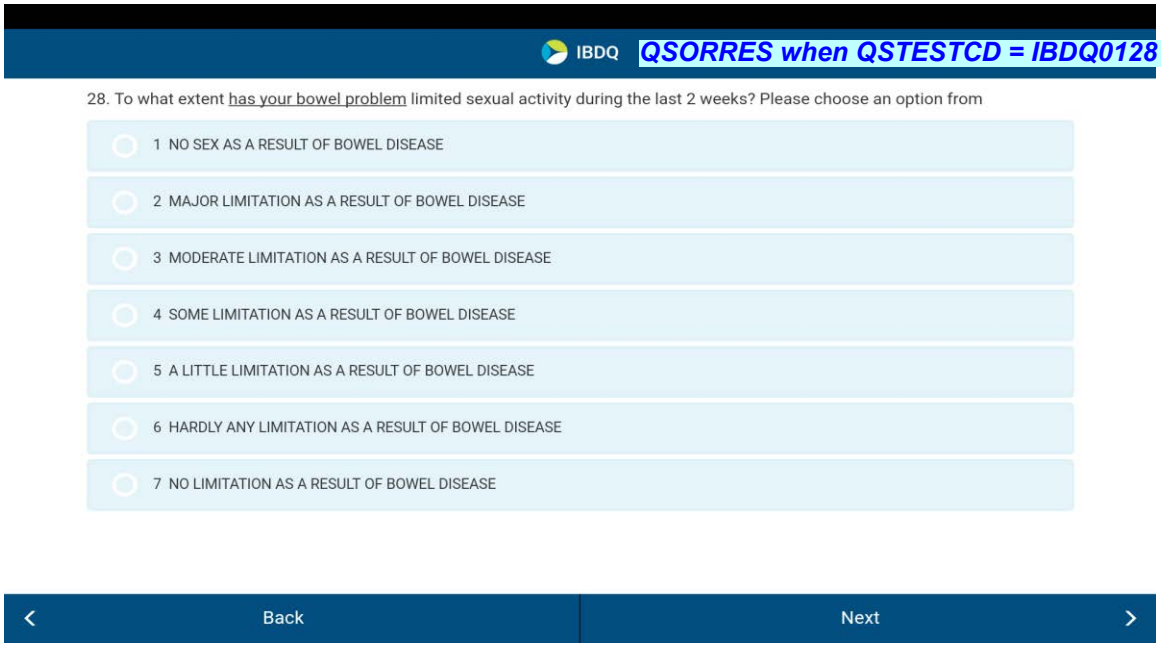


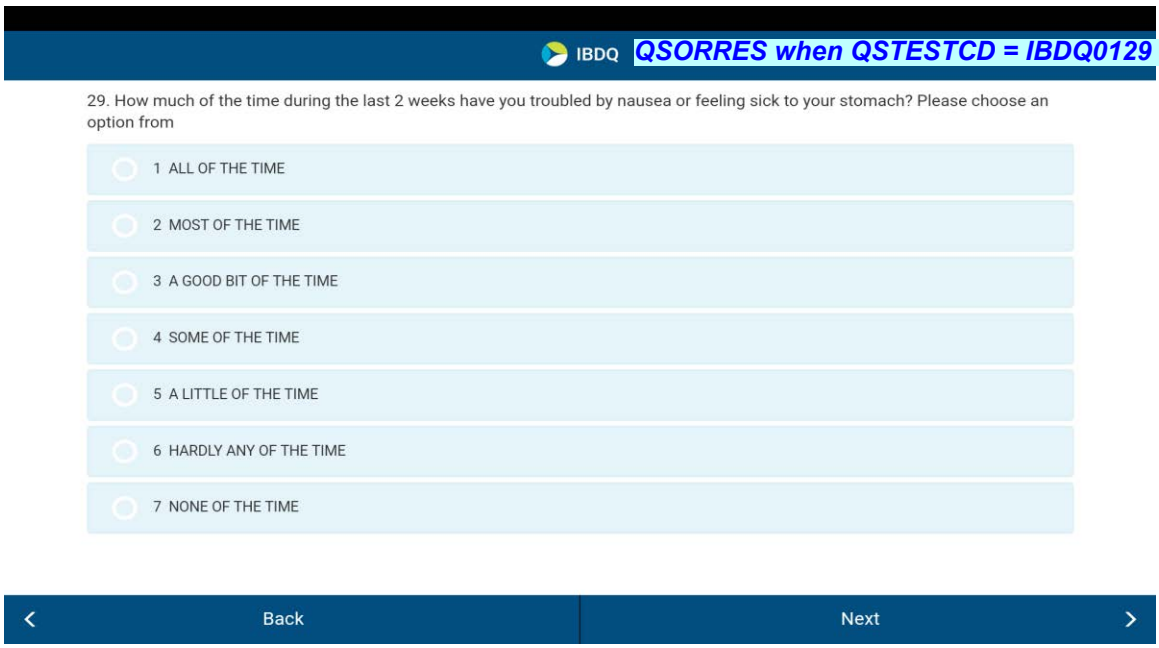
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W**
QSCAT = IBDQ



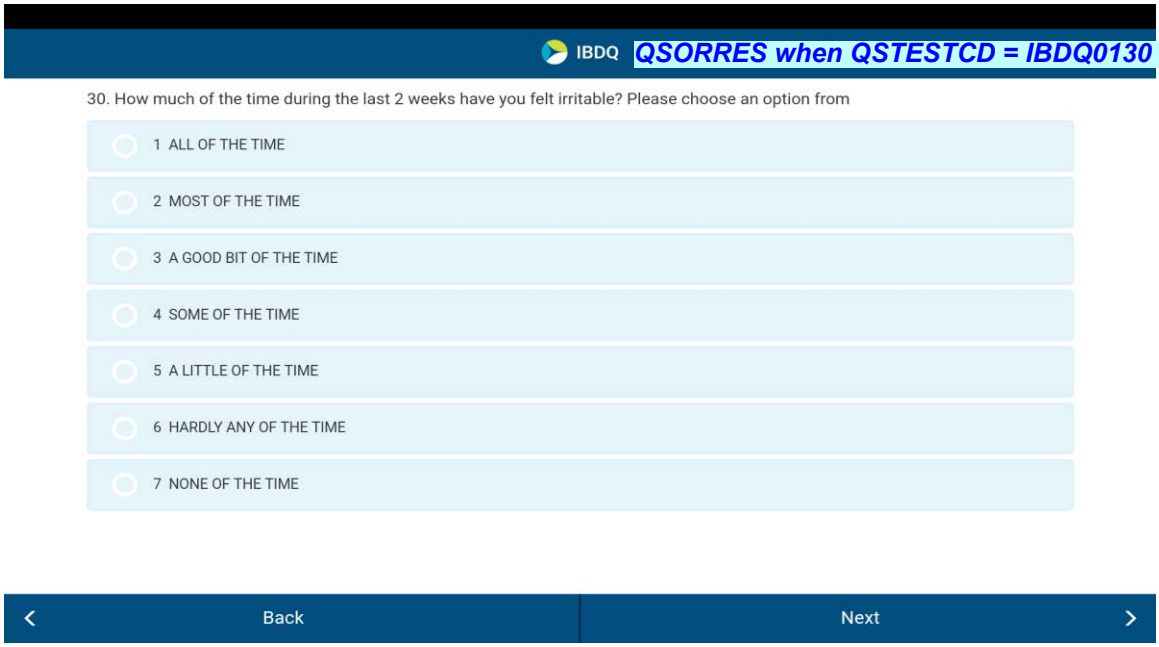


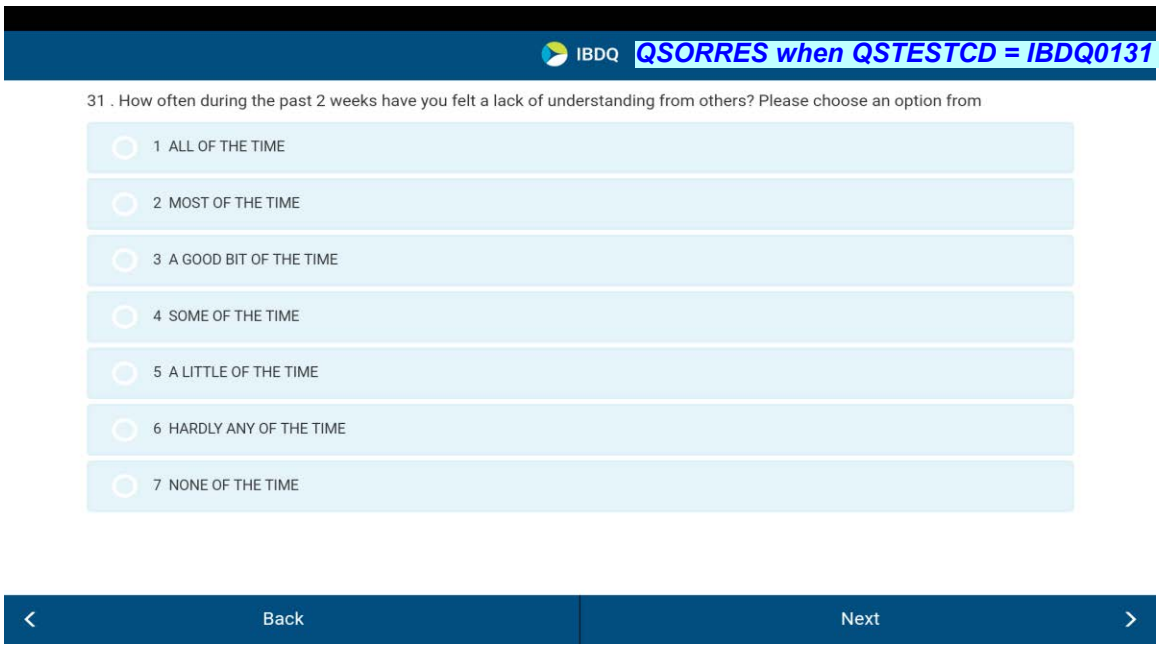
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**



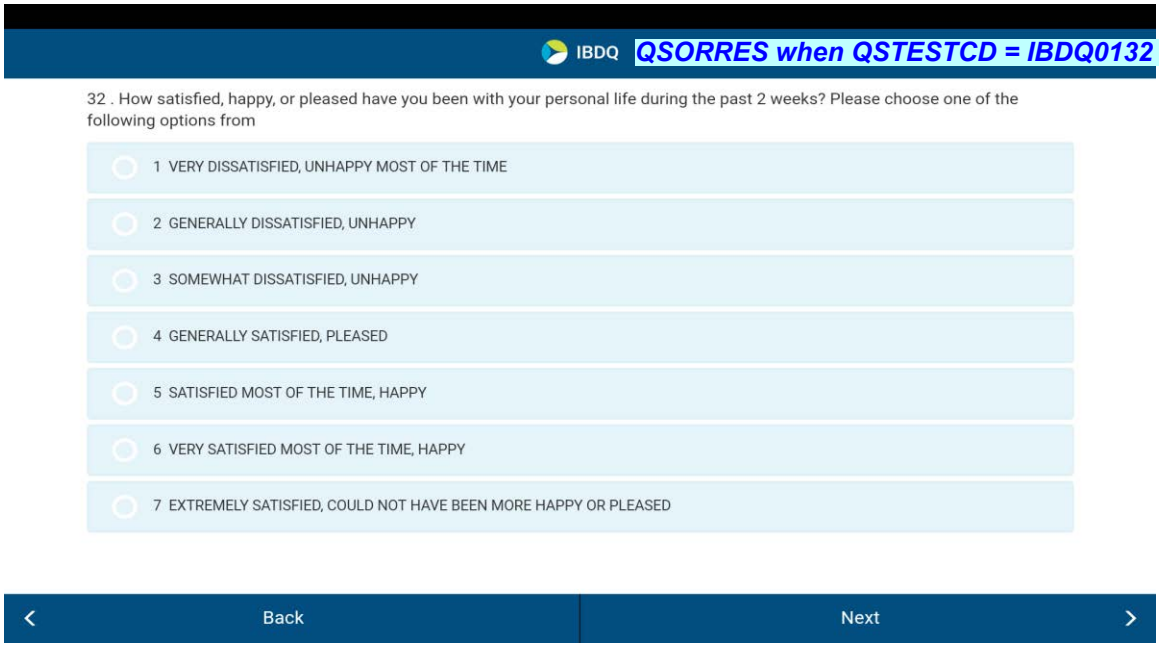


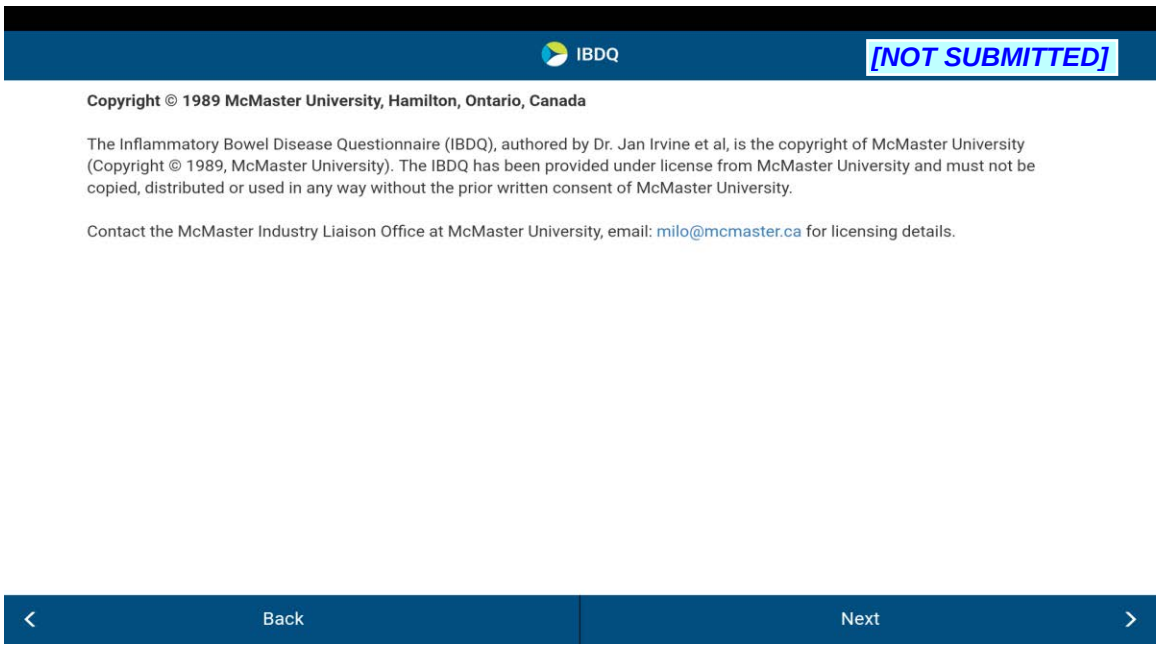
QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W** **QSCAT = IBDQ**





QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P2W**
QSCAT = IBDQ





[NOT SUBMITTED]

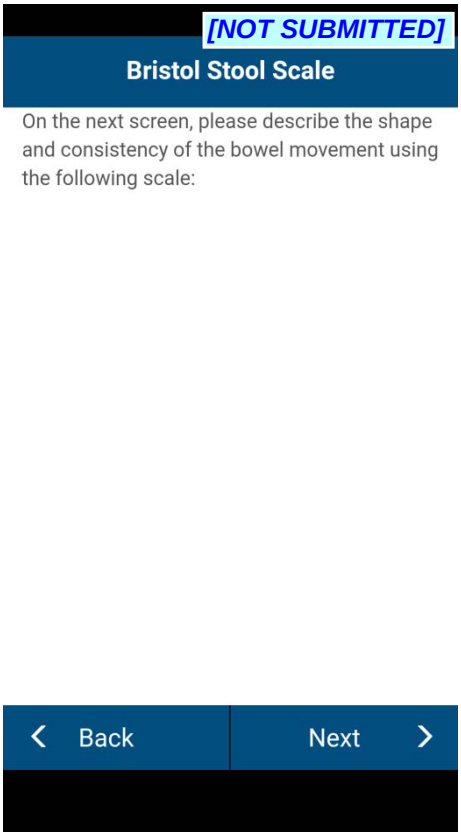
Janssen CNTO1959UCO2002

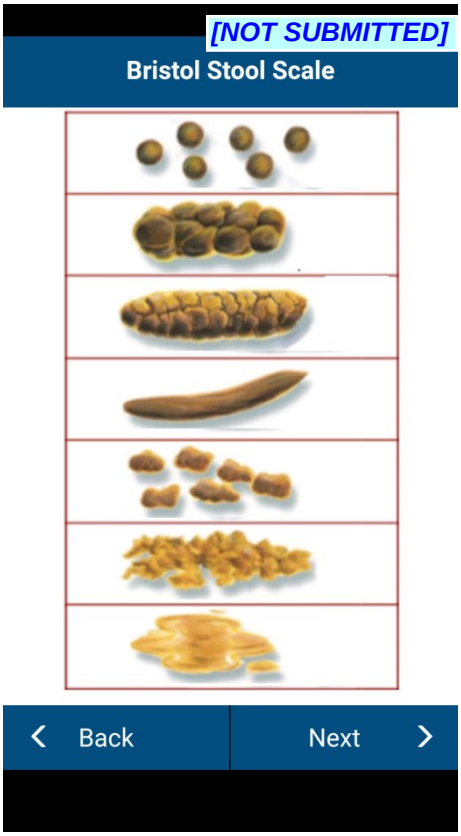
BSS eCOA Handheld Screenshots

Project Information	
Sponsor name and Protocol number:	Janssen CNTO1959UCO2002
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	0.01

QS = Questionnaires

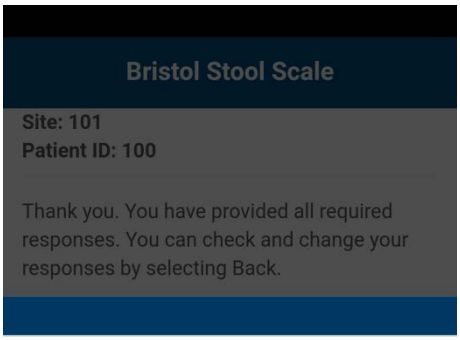
QSEVAL = STUDY SUBJECT
QSCAT = BRISTOL STOOL CHART







Type 1: Separate hard lumps like nuts (hard to pass)

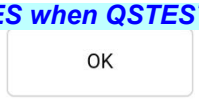


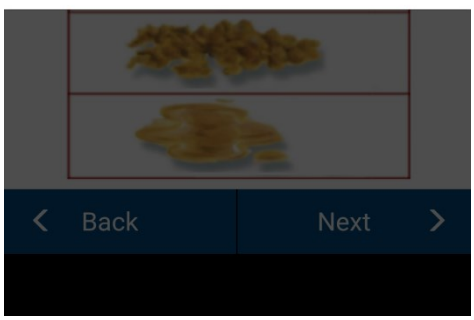
Type 2: Sausage-shaped but lumpy

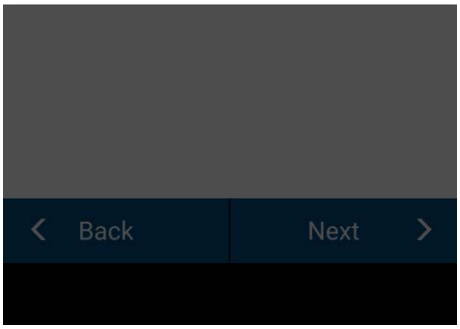
QSORRES when QSTESTCD = BSC0101



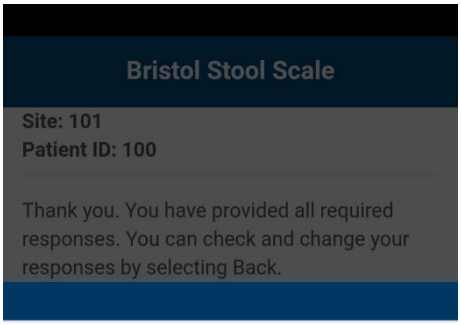
QSORRES when QSTESTCD = BSC0101





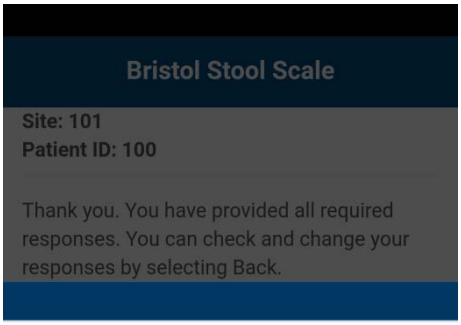


For annotations, refer to CRF page 38



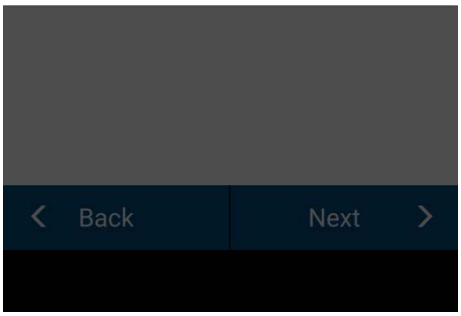
Type 3: Like a sausage but with cracks on its surface

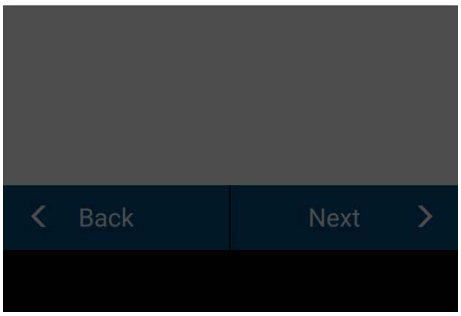
OK

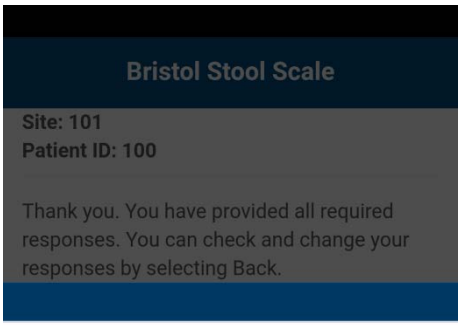


Type 4: Like a sausage or snake, smooth and soft

OK

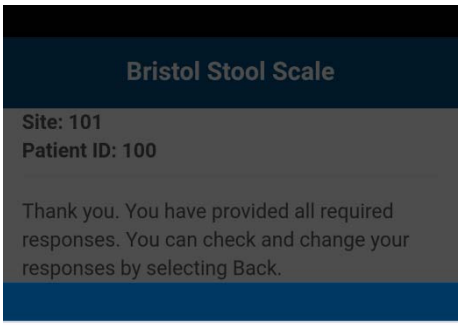






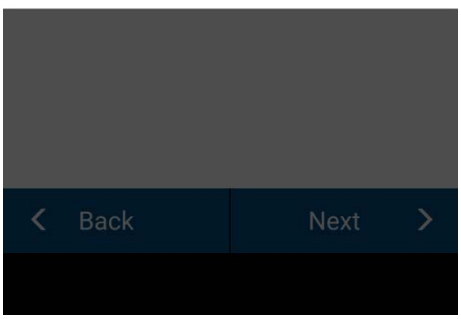
Type 5: Soft blobs with clear-cut edges (passed easily)

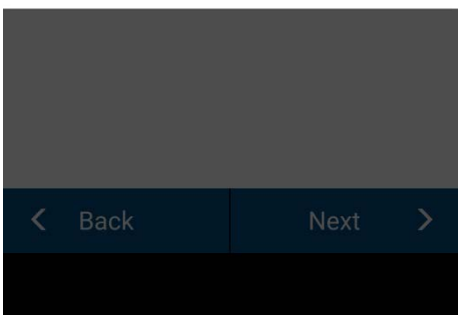
OK



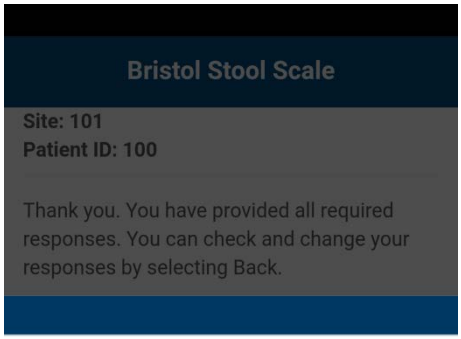
Type 6: Fluffy pieces with ragged edges, a mushy stool

OK



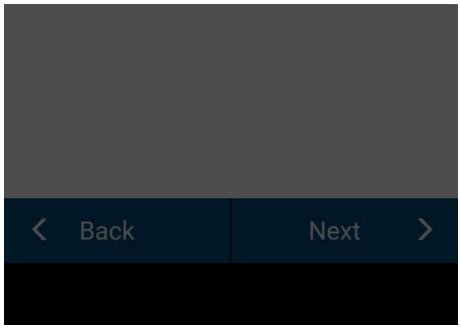


For annotations, refer to CRF page 38



Type 7: Watery, no solid pieces (entirely liquid)

OK



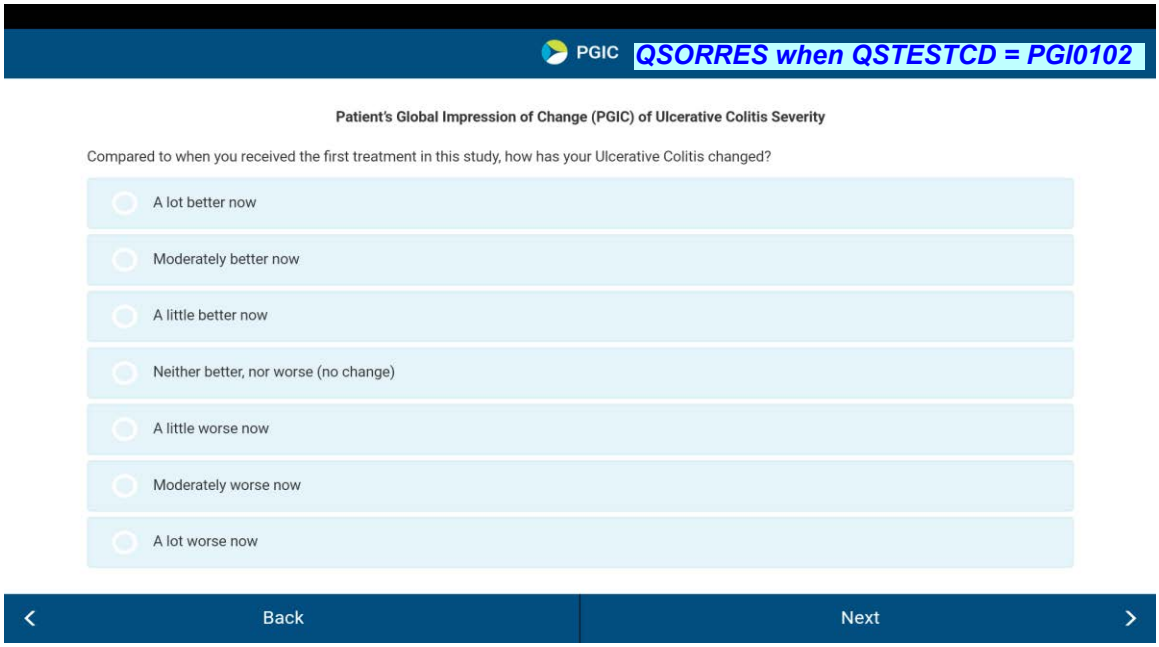
[NOT SUBMITTED]

Janssen CNTO1959UCO2002

PGIC eCOA Tablet Screenshots

Project Information	
Sponsor name and Protocol number:	Janssen CNTO1959UCO2002
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1

QS = Questionnaires ***QSEVAL = STUDY SUBJECT***
QSCAT = PGI



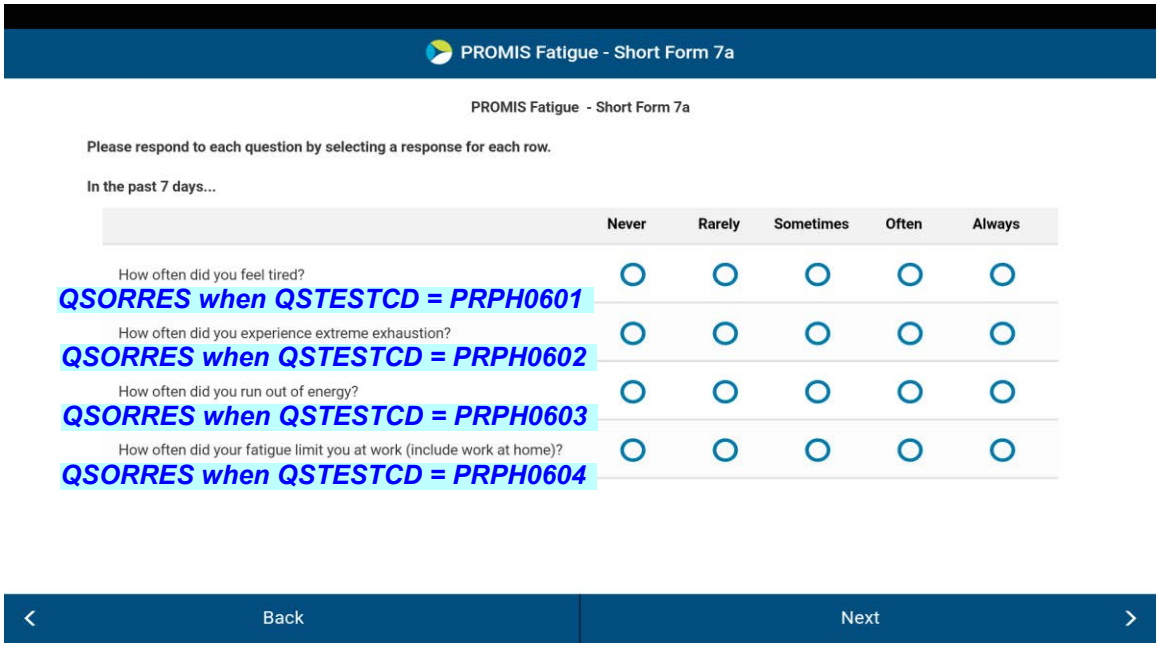
[NOT SUBMITTED]

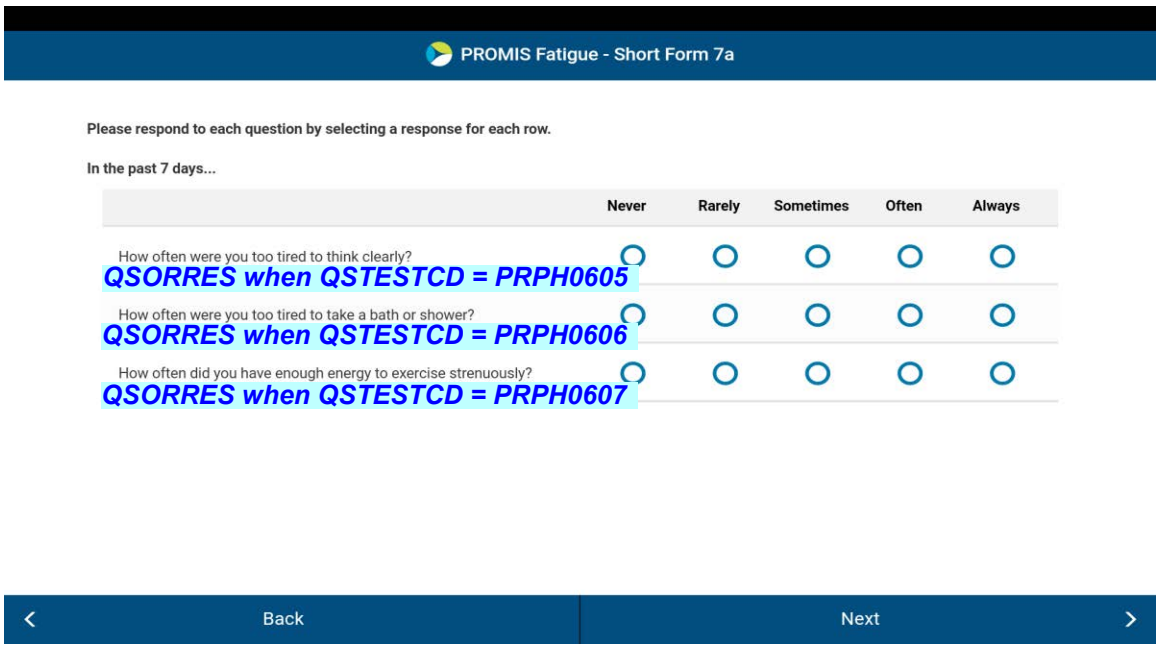
Janssen CNTO1959UCO2002

PROMISFatigue-ShortForm7a eCOA Tablet Screenshots

Project Information	
Sponsor name and Protocol number:	Janssen CNTO1959UCO2002
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1

QS = Questionnaires **QSEVAL = STUDY SUBJECT** **QSEVLINT = -P7D**
QSCAT = PROMIS SF V1 FATIGUE 7A





[NOT SUBMITTED]

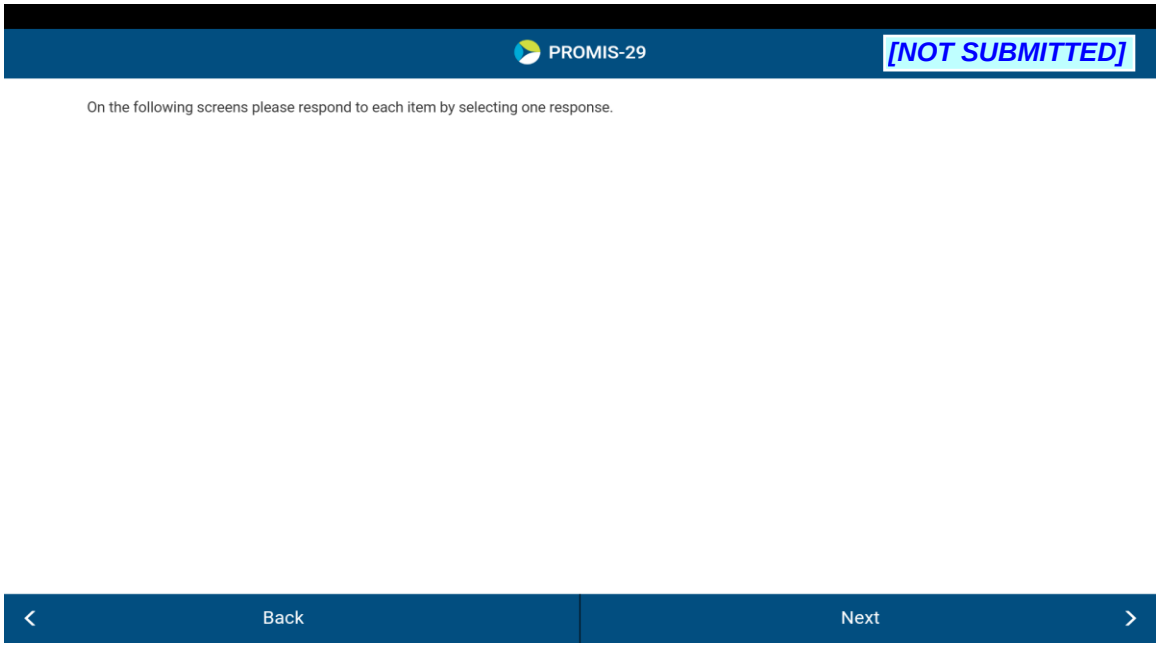
Janssen CNTO1959UCO2002

PROMIS-29 eCOA Tablet Screenshots

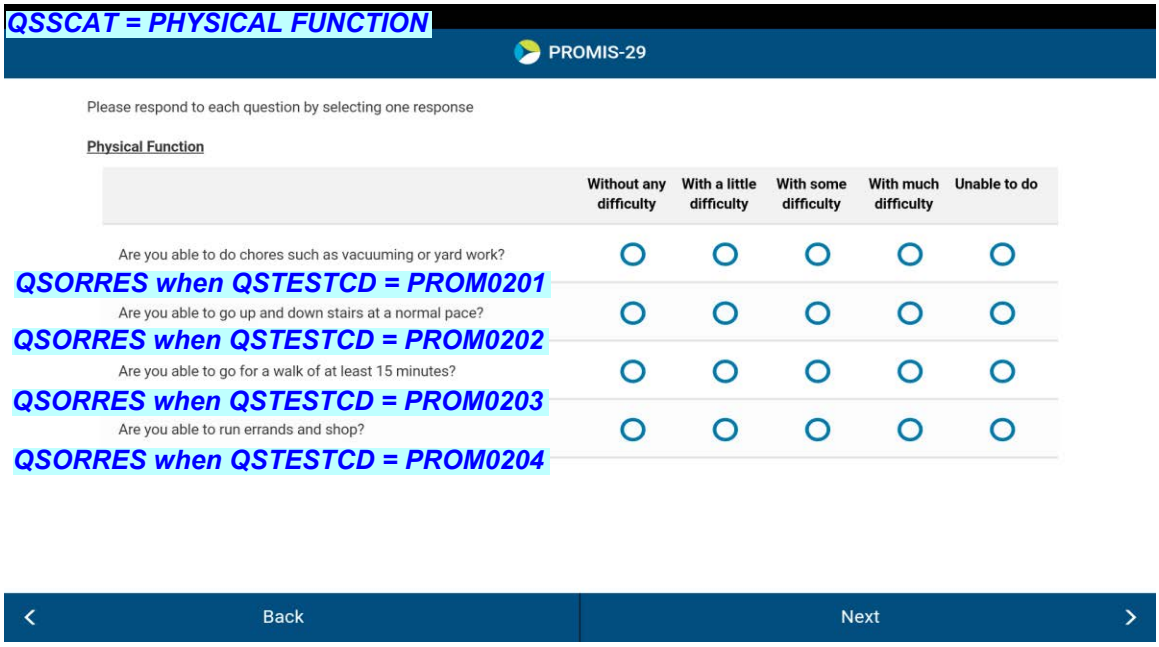
Project Information	
Sponsor name and Protocol number:	Janssen CNTO1959UCO2002
Source Language:	English (US)
Target Language:	N/A
Screenshot Document Date:	26-Sep-18
Screenshot Document Version:	1

QS = Questionnaires

QSEVAL = STUDY SUBJECT
QSCAT = PROMIS-29 PROFILE V2 PARTICIPANT



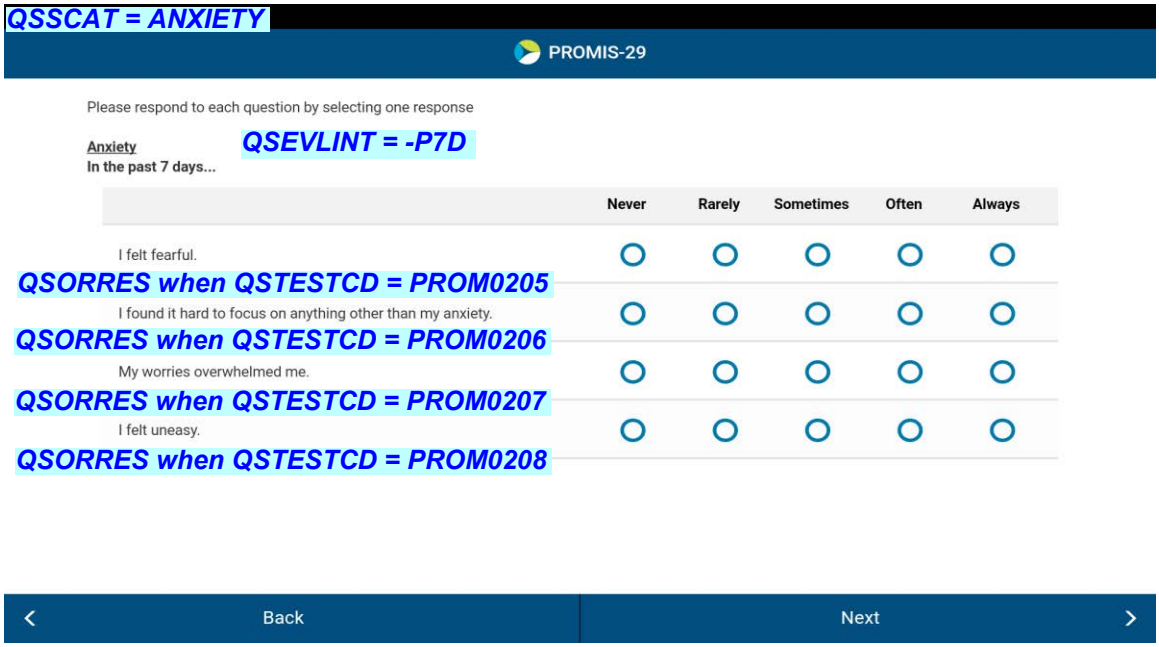
QSSCAT = PHYSICAL FUNCTION



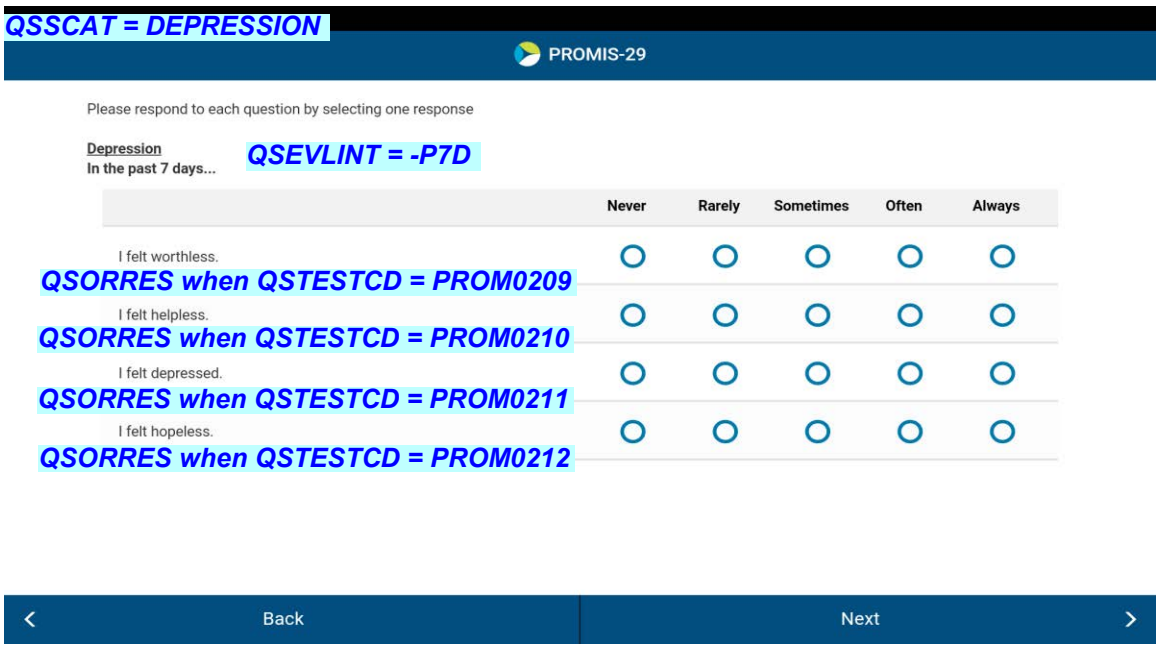
QS = Questionnaires

QSEVAL = STUDY SUBJECT
QSCAT = PROMIS-29 PROFILE V2 PARTICIPANT

QSSCAT = ANXIETY

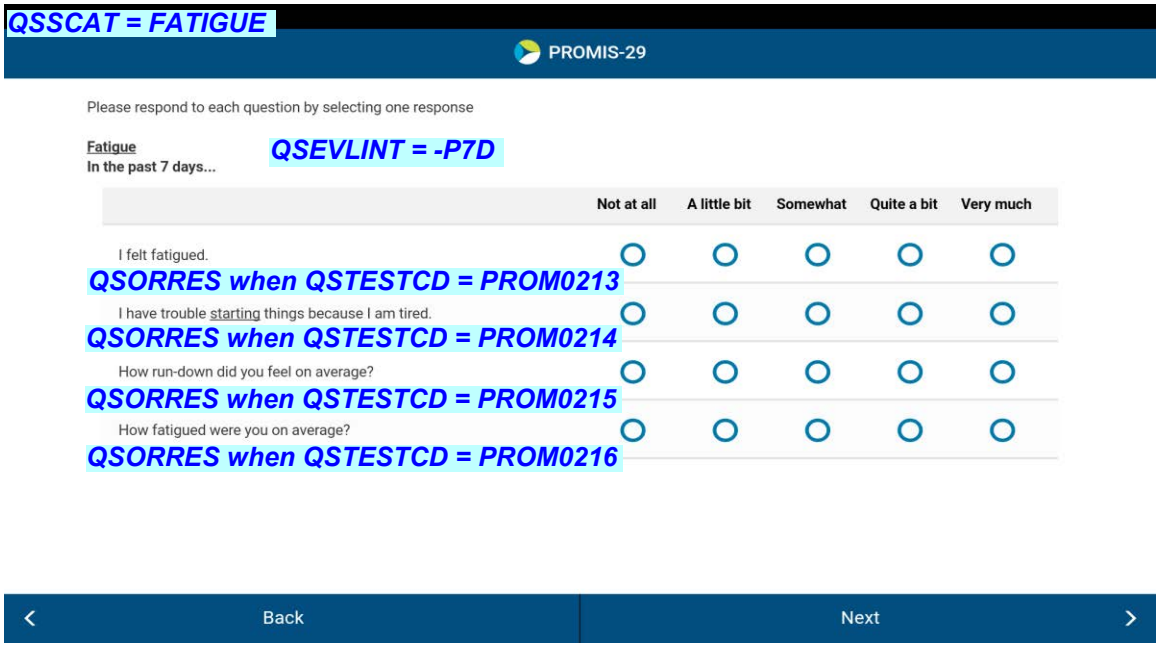


QSSCAT = DEPRESSION

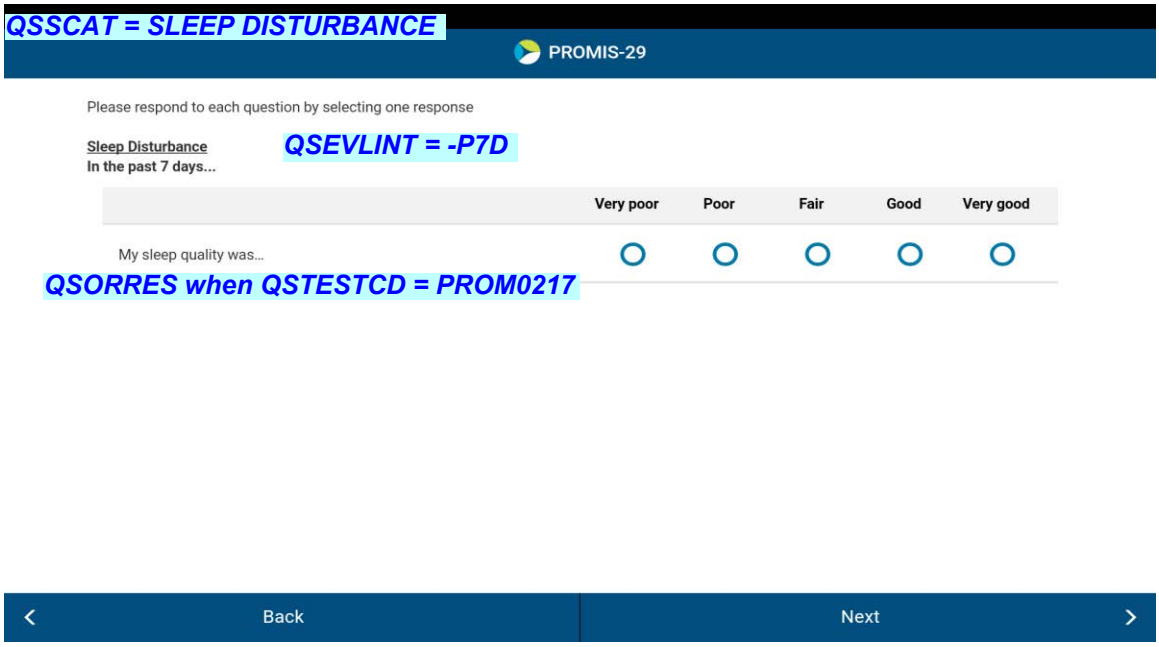


QS = Questionnaires **QSEVAL = STUDY SUBJECT**
QSCAT = PROMIS-29 PROFILE V2 PARTICIPANT

QSSCAT = FATIGUE



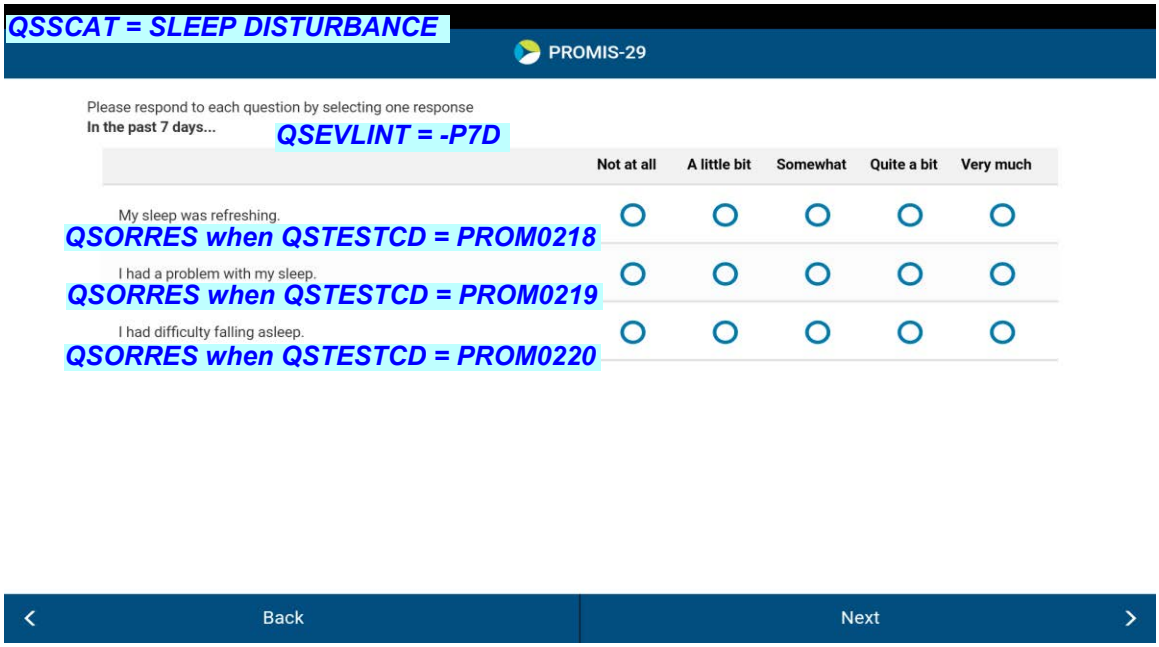
QSSCAT = SLEEP DISTURBANCE



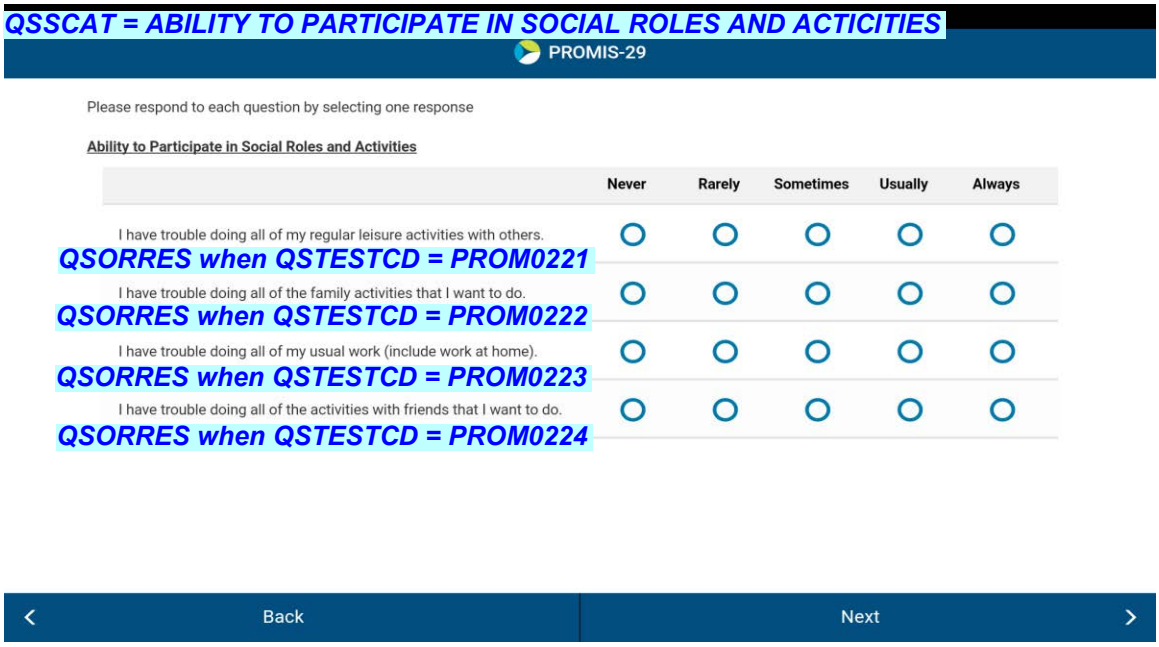
QS = Questionnaires

QSEVAL = STUDY SUBJECT
QSCAT = PROMIS-29 PROFILE V2 PARTICIPANT

QSSCAT = SLEEP DISTURBANCE



QSSCAT = ABILITY TO PARTICIPATE IN SOCIAL ROLES AND ACTICITIES

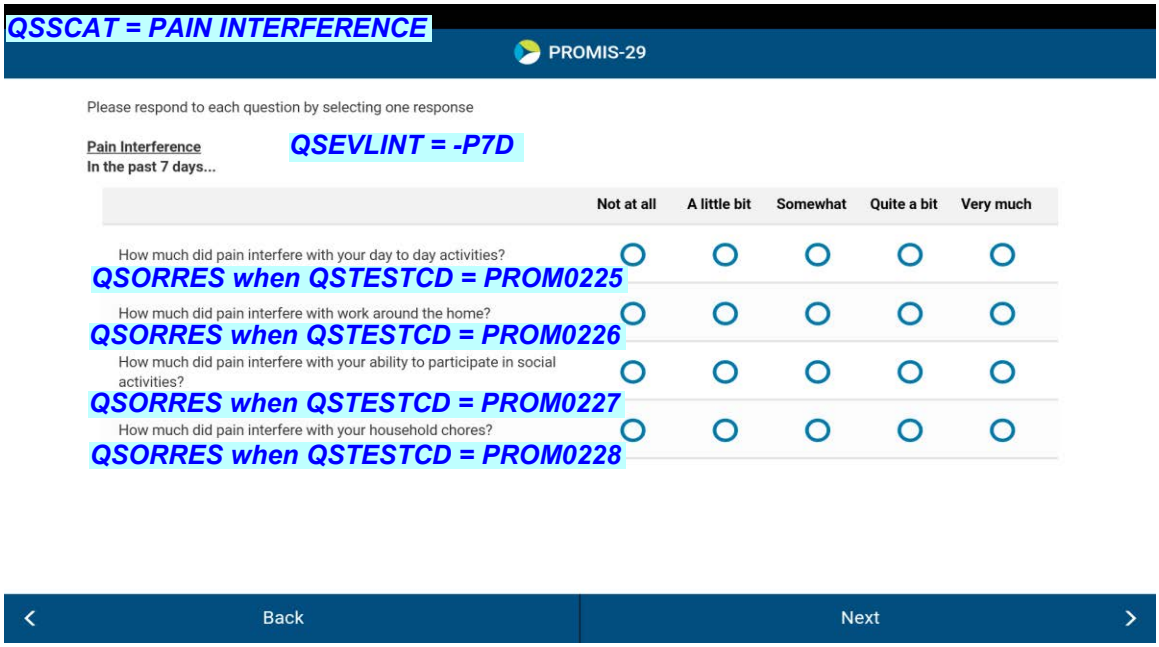


QS = Questionnaires

QSEVAL = STUDY SUBJECT

QSCAT = PROMIS-29 PROFILE V2 PARTICIPANT

QSSCAT = PAIN INTERFERENCE



QSSCAT = PAIN INTENSITY

